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UPSC MAINS 2027

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Social Justice

General Studies Paper - II

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Chapter 1

Welfare Schemes for Vulnerable Sections

Welfare Schemes for Vulnerable Sections

PYQs

[2026] Does India's tribal development policy reflect the aspirations for a socially grounded and equity-based governance? Justify your answer. (10)

[2026] Can the constitutional mandate of rights-based welfare be effectively realised in the context of non-integrated governance and minimal public investment? Examine. (15)

[2025] Women's social capital complements in advancing empowerment and gender equity. Explain. (10)

[2023] "Development and welfare schemes for the vulnerable, by its nature, are discriminatory in approach." Do you agree? Give reasons for your answer. 15

[2022] The Rights of Persons with Disabilities Act, 2016 remains only a legal document without intense sensitisation of government functionaries and citizens regarding disability. Comment. (10)

[2022] Besides the welfare schemes, India needs deft management of inflation and unemployment to serve the poor and the underprivileged sections of the society. Discuss. (15)

[2021] "Though women in post-Independent India have excelled in various fields, the social attitude towards women and feminist movement has been patriarchal." Apart from women education and women empowerment schemes, what interventions can help change this milieu? (15)

[2019] Performance of welfare schemes that are implemented for vulnerable sections is not so effective due to absence of their awareness and active involvement at all stages of policy process – Discuss. (15)

In the Indian socio-legal context, **Vulnerable Sections** refer to groups that face a higher risk of poverty, social exclusion, and structural discrimination compared to the general population.

It arises from an interconnection of **socio-economic, physical, and historical factors**, making it difficult for these individuals to access basic rights, resources, or opportunities without state intervention.

India's commitment to **inclusive development** is enshrined in the **Preamble** (justice & equality), **FRs (Articles 14, 15, 16, 17, 21, 23, 24)**, and **Directive Principles (Articles 38, 39, 41, 46)**.

"Vulnerable sections" include

1. **Scheduled Castes (SCs)** : 16.63% of population
2. **Scheduled Tribes (STs)** : ~8.6% of population
3. **Other Backward Classes (OBCs)** : ~41% (NSSO estimates; Census 2027 will provide actual data)
4. **Women** : ~48.5% of population
5. **Children** : ~37% (under 18)

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6. **Senior Citizens** : ~10.5% (65+ in 2024)
7. **Persons with Disabilities (PwDs)** : ~2.21% Census 2011 (estimated higher)
8. **Minorities** : ~19.3% (Muslims, Christians, Sikhs, Buddhists, Parsis, Jains)
9. **Transgender Persons** : 4.88 lakh (Census 2011)
10. **Migrant Workers** : 45 crore/28.9% of population (Census 2011)
11. **Manual Scavengers, Devadasis, Bonded Labourers, Trafficking Victims**

Rationale behind Welfare Schemes

1. **Constitutional Mandate :** Articles 38, 39, and 41-46 mandate the State to minimize inequalities and promote the welfare of the people,
2. **Historical and Corrective Justice :** Addressing centuries of systemic marginalization, untouchability, and exclusion.
3. **Human Rights :** Ensuring that every citizen enjoys a "Social Minimum" (basic food, health, and dignity) as part of their Right to Life (**Article 21**). Eg. **National Food Security Act (NFSA)**
4. **Economic Inclusive Growth :** Bringing marginalized groups into the formal economy increases the productive workforce and boosts aggregate demand. **Eg. Stand-Up India**
5. **Demographic Dividend Optimization : Eg. POSHAN Abhiyaan** targets stunting and wasting in children to ensure future labor productivity.
6. **Political Stability and Social Cohesion :** Reducing radicalization and civil unrest by ensuring that those at the bottom of the pyramid feel invested in the nation's growth. **Eg. PESA Act**
7. **Climate Justice :** Vulnerable groups (like coastal fishers or forest dwellers) are the first victims of climate change and need "Resilience Support."
8. **Addressing Market Failures :** welfare schemes act as a safety net where private services are absent. **Eg. Ayushman Bharat (PM-JAY)**
9. **Empowerment and Agency :** Moving from "charity" to "rights-based entitlement" to give vulnerable individuals the agency to make life choices. **Eg. MGNREGA** ("Right to Work")
10. **Global Commitments (SDGs) :** As a global leader, India is committed to the UN Sustainable Development Goals, specifically **SDG 1 (No Poverty)** and **SDG 10 (Reduced Inequalities)**.
11. **Promoting balanced regional growth** : Eg. **Aspirational Districts Programme**

Major challenges in welfare schemes in India

Formulation Stage

1. **Fragmented Policy Design:** Schemes are often designed in **"departmental silos,"** leading to duplication of effort and resource wastage. **Eg.** Overlap between **Skill India** and **DAY-NRLM**
2. **Exclusionary Eligibility Criteria** : **Eg.** Using **2011 SECC data** for 2026 welfare targeting misses a decade of demographic shifts.
3. **TopDown Approach:** Policies designed **without local consultation**, ignoring contextual needs ("one-size-fits-all" models)
4. **Political Populism:** Welfare schemes influenced by **short-term electoral motives** rather than long-term sustainability. **Eg.** Farm loan waivers
5. **Weak Evidence Base:** Insufficient **impact assessments or pilot studies** before large-scale rollout. **Eg.** skill schemes with low placement rates.

Implementation Stage

1. **Administrative Bottlenecks:** Delays due to **bureaucratic red-tape** and poor inter-agency coordination.

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2. **Leakages and Corruption:** Diversion of funds and **ghost beneficiaries** reduce actual impact. **Eg**. 10 lakh ghost beneficiaries under Ayushaman Bharat (CAG report).
3. **Digital Divide:** Reliance on online platforms excludes **rural poor, women, and PwDs** with limited **Eg.** Chhattisgarh girls death due to lack of biometric updation with PDS.
4. **Infrastructural Gaps:** Weak connectivity and logistics, especially in **tribal and remote regions**.
5. **Centre-State Fiscal Friction** due to delays in releasing the "Central Share" lead to stalled projects and wage arrears in states. **Eg**. MGNREGA wages delay

Monitoring Stage

1. **Outlay vs. Outcome Gap :** Success is measured by "budget spent" rather than actual improvement in social indicators. **Eg**. "learning poverty" despite high **Education spending**
2. **Technocratic Grievance Redressal :** Automated portals (CPGRAMS) often close tickets based on "procedural replies" rather than "actual resolution."
3. **Weak Social Audits :** Lack of independent, community-led audits (except MGNREGA) makes officials "judge and jury" of their work.
4. **Lack of Real-time Data** prevents "mid-course corrections". **Eg.** Relying on **NFHS-5** (2019-21) data to tackle **2026 stunting trends** in tribal belts.
5. Local officials often under-report failures to meet "Targets." **Eg.** Over-reporting **ODF (Open Defecation Free)** status despite lack of functional water in toilets.

Crisis Management Stage

1. **Poor Preparedness:** Welfare delivery systems often **reactive, not proactive**, during crises **Eg.** COVID-19 migrant crisis.
2. **Coordination Failures:** Weak coordination between **Centre, States, and NGOs** during disaster response. **Eg.**Non payment of bills by govt for Oxygen cylinders led to deaths in private hospitals
3. **Exclusion during Disruptions:** Beneficiaries without digital/ID linkage **miss out on relief Eg.** DBT failures.

Way Forward

Planning stage

1. Adopting a **"Whole-of-Government" approach** : Transitioning from individual scheme-based planning to an overarching social protection strategy to prevent administrative duplication.
2. **Data-Driven Targeting** to minimize inclusion and exclusion errors.
3. **Agro-Climatic and Regional Customization:** Allowing states greater fiscal flexibility to tailor central schemes to local demographic and geographical risk profiles.
4. **Outcome-Linked Budgeting:** Shifting focus from "outlays" (money spent) to "outcomes" (lives improved) through pre-defined, measurable performance indicators.

Implementation

1. **Universalizing Digital Portability** to protect the rights of inter-state migrant workers. **Eg. "One Nation One Ration Card"**
2. **Frontline Administrative Empowerment:** Strengthening local bodies with better tools, talent, and authority to resolve local grievances.
3. **Horizontal Reservation** within schemes for the most marginalized, such as transgender persons and the elderly. **Eg.** Karnataka's **1% horizontal reservation** for transgender
4. Moving toward **"doorstep delivery"** and direct cash transfers to reduce the "compliance burden" on vulnerable sections. **Eg. e-SeHAT initiative**

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Feedback stage

1. **Mandatory Social Audits for** all welfare schemes through Gram Sabhas
2. **Real-Time Grievance Redressal:** Integrating AI-driven chatbots and multi-lingual helplines
3. **Community Monitoring Systems:** Empowering Self-Help Groups (SHGs) and local NGOs to act as "Social Auditors" for schools and health centers.
4. **Independent Impact Assessments:** Partnering with academic institutions to conduct third-party "Randomized Control Trials" (RCTs) on scheme effectiveness.

Women

Women empowerment involves enhancing a woman's sense of self-worth, freedom to make her own choices, and the right to drive social change for herself and others.

B.R. Ambedkar: "I measure the progress of a community by the degree of progress which women have achieved."

"When women progress, the family progresses, the village progresses, and the nation progresses."

Kofi Annan: "There is no tool for development more effective than the empowerment of women."

Simone de Beauvoir: "One is not born, but rather becomes, a woman."

Gender equality can add **USD 700 billion** to India's GDP in 2025. (**McKinsey report**)

Demographic Profile

- **Sex ratio (overall):** 943 (2011) → **952 (2036 projected)**
- **Sex ratio at birth (SRS): 917** (SRS 2021-23) ↑ from 904 (2017-19)
- The median age for marriage is **19.2 years**. (NFHS-5)
- **Female-Headed Households: 17.6%** (NFHS-5, 2019-21).

Health and Fertility

- **TFR (Total Fertility Rate):** 1.9 (2023/26) ↓ from 2.0 (2020). Rural - 2.1 | Urban - 1.5
- **Maternal Mortality Ratio (MMR):** 88 per lakh live births
- **Infant Mortality Rate (IMR):** 25 (2023/26) ↓ from 28 (2020)
- **Under-5 Mortality Rate (U5MR):** 29 (2023/26) ↓ from 32 (2020)
- **Anaemia among women (15-49 yrs):** 57.2% (NFHS-5)
- **Institutional deliveries:** 96.2% (MoSPI/PIB Health Survey, April 2026) ↑ from 88.6% (2019-21)

Literacy and Education

- **Female literacy:** Rose from 9% (1947) to **79.5%** (2025/26 Projected) ↑ from 77% (2023)
- **Primary (GER): 107.2** (Girls) vs 97.4 (Boys) (Economic Survey 2025-26)
- **Higher Secondary (GER): 66.8** (Girls) ↑ from 58.2 in 2021-22
- Women account for **43% of STEM enrolment and over 53% of UGC-NET research fellowships in 2024-25.**
- **Higher Education (GER): 30.2%**(Female) vs **28.9%** (Male). (surged from 1.57 crore to 2.18 crore in last decade)
- **Doctoral enrolment has grown by over 135% in under a decade.**
- **Women graduates (age 15+): 9.6%** (vs 14.2% men) (PLFS 2025 Annual Report)

Economic Participation

- Women borrowers hold a credit portfolio of **Rs 76 lakh crore** (**26% of total credit) (NITI Aayog)**

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- **Female LFPR (15+):** 41.7% (2024) vs 23% in (2017). (**global average 47.2%**)
- **PM Jan Dhan Yojana:** 55.8% of account holders are women (32.21 crore).
- **80% of rural women** in India are engaged in agriculture and allied sectors
- Women spend an average of **4.7 hours/day** on unpaid domestic and caregiving work.
- **Gender Budget 2026-27: ₹5.01 lakh crore** (↑11.55% over previous year), accounting for **9.37%** of the total Union Budget.
- **PM Mudra Yojana: 69.2%** of total loans issued to women entrepreneurs
- **Women-led MSMEs** have nearly doubled to **1.92 crore** over the last decade

Decision-Making & Representation

- **Women in PRIs: 49.75%** (approx. 1.2 million) ↑ from 46% in 2024
- **Women-led MSMEs: 3.07 crore** registered enterprises (Udyam, Feb 2026)

Social Challenges

- **Major crimes against women (NCRB 2025/26):**
  - **Cruelty by husband/relatives:** Remains the largest category (~32%)
- **Child marriage: 23.3%** (NFHS-5); Rural - 27%, Urban - 14.7%
- Housewives account for approximately **14%** of total suicides in India (NCRB 2025)

Major Issues in Women's Empowerment

Social

1. **The enduring Devī-Dāsī dichotomy:** idolizing women as sacred yet accepting their subjugation. It reveals deep-rooted cultural norms that legitimize gender inequality.
2. **Child marriage**: **23.3%** of girls marry before 18 (NFHS-5)
3. **Triple Burden** : Household, Childcare, Work
4. **Media stereotypes**: Gender-biased portrayals reinforce traditional roles.
5. **Mental health crisis**: Depression rates **50% higher in women than men (WHO)**.
6. **Educational gaps**: Higher dropout rates among girls due to financial/cultural constraints.

Political and institutional

1. **Low representation in legislatures**: Women hold only **13.6% of Lok Sabha seats (18th Lok Sabha)** and **22.1% in state assemblies (2024)**.
2. **Tokenism in local governance**: Eg. "Sarpanch Pati" culture undermines effective female leadership.
3. **Gender Bias in armed forces** : Eg. recently SC flagged systemic gender bias in promotions and medical fitness evaluations of women officers in armed forces

Economic

1. **Underrepresentation in STEM**: Women constitute **43% of STEM graduates** but only **18.6%** of the STEM workforce
2. **Informal employment**: **90% of women in low-income countries** work in the informal sector (UN Women), lacking job security and benefits.
3. **Gender pay gap:** In India, the gap is estimated at **24.81%** (only 2026 report); mothers earn approx. **$0.74** for every dollar earned by men.
4. **Unpaid care burden leading to "Time Poverty":** Women spend **363 mins/day** on unpaid work vs. **123 mins/day** by men (Economic Survey 2026).
5. Women perform **84%** of unpaid care work in India **(economic value is 15-17% of GDP)**

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6. **"Stained Glass Ceiling" in capital access** due to **lack of collateral and systemic bias. Eg. "funding drought"** in **women-led startups** (receive **₹4 out of every ₹100** raised by founders)
7. **"Leaky Pipeline" syndrome:** Women drop out at mid-management levels due to motherhood and lack of flexible work infrastructure.
8. Rural women face **"Triple Burden" of Drudgery: Manual farm labor, unpaid domestic care, and managing resource scarcity** with limited land ownership (13%).
9. **Workplace barriers:** Lack of childcare and flexible policies restrict workforce participation; urban female LFPR remains significantly lower than rural (**~25.5%**).
10. **Patriarchal Resistance to Economic Activity: NFHS-5**: Only **~40% women** have control over their own earnings.

Security

1. **Trafficking**: India remains a hub, with **~150,000 women/girls trafficked annually (UNODC)**.
2. **Rising crimes**: As per **NCRB,** about **51 cases of crimes against women recorded every hour** in India

Feminization of climate vulnerability

1. **Collection Labor:** Women lose **150 million workdays annually** just to fetch water
2. **Health Hazard:** Water scarcity exposes women to water-borne diseases.
3. **Nutritional Deficit:** Food insecurity makes women 1.6x more likely to be anemic
4. Extreme heat and climate-driven pathogens (Zika, Malaria) significantly increase rates of stillbirth and neonatal complications.
5. Women are overrepresented in climate-sensitive sectors (agriculture/forestry)
6. **Resource shortages** can increase women's daily unpaid labor to **8.3 hours by 2050**.
7. Climate change could push **158.3 million more women into poverty** by 2050
8. The UN estimates **80% of climate-displaced people are women**
9. Post-disaster settings often see a rise in **trafficking and "transactional sex"** as survival mechanisms in overcrowded camps.

> Globally, unpaid care work keeps **708 million women out of the labor force.** In India, 93% women spend approximately **7 hours daily on unpaid work**, which has an economic value equivalent to **15-17% of the GDP. (men 28 mins)**

Steps Taken to Empower Women in India

Political Empowerment

1. **Nari Shakti Vandan Adhiniyam, 2023:** To reserve one-third of seats for women in Lok Sabha and state legislative assemblies.
2. **National Commission for Women (NCW):** Established to review the constitutional and legal safeguards for women, recommend remedial legislative measures, facilitate redressal of grievances, and advise the government on all policy matters affecting women.
3. **Capacity Building:** Through the **Gram Panchayat Development Plan (GPDP)** by the Ministry of Panchayati Raj, elected women are trained in leadership, budgeting, and digital literacy to move beyond the "Pradhan-Pati" (proxy) culture.

Economic Empowerment

1. **Sukanya Samriddhi Yojana:** A savings scheme for the girl child to ensure financial security. (4+ crore accounts; ₹3.33+ lakh crore deposits)
2. **Lakhpati Didi:** Aims to create **6 crore "Lakhpati Didis"**—women SHG members earning an annual household income of over ₹1 lakh through sustainable micro-enterprises.

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3. Over **10.05 crore rural women** are now mobilized into 90.90 lakh SHGs under the DAY-NRLM (Deendayal Antyodaya Yojana).
4. **Credit Support:** Women entrepreneurs now receive **90% credit guarantee coverage** (compared to 75% for others) under the CGTMSE scheme.
5. **Digital Banking:** Over **55.8% of PM Jan Dhan accounts** (approx. 32 crore) are owned by women, serving as the foundation for Direct Benefit Transfers (DBT) and credit access.
6. **Ujjwala Yojana:** Provided free LPG gas cylinder to around 10 crore women
7. 72% Women Ownership under Pradhan Mantri Awas Yojana - Gramin (PMAY-G)
8. **PMMY (MUDRA) & Stand-Up India:** 69% of loans sanctioned to women entrepreneurs under PMMY and 84% beneficiaries under Stand-Up India are women.
9. **Mahila Shakti Kendras:** To provide skills, employment, digital literacy to rural women.
10. **Mahila E-Haat:** Online marketing platform for women entrepreneurs.
11. **GeM Womaniya:** 2+ lakh women-led MSEs. ₹80,000+ crore procurement orders.

Social Empowerment

1. **Mission Shakti:**
   a. The **'Sambal(safety)**: One Stop Centre (OSC), Women Helpline (WHL), Beti Bachao Beti Padhao (BBBP) and Nari Adalat (Nari Adalat).
   b. The **'Samarthya'(empowerment)**: Shakti Sadan, Sakhi Niwas, Padhan Mantri Matru Vandana Yojana (PMMVY), Palna and SANKALP: Hub for Empowerment of Women.
2. **Beti Bachao Beti Padhao:** Aims to address the declining child sex ratio and promote the education and empowerment of the girl child. (Sex Ratio at Birth improved 918 → 929; girls' secondary GER 75.51% → 80.2%.
3. Bal Vivah Mukt Bharat Abhiyan to **reduce the prevalence of child marriage by 10% by 2026** and completely eradicate the practice by **2030**. Key components include
   a. A dedicated **Child Marriage Prohibition Officers** at district and sub-district levels
   b. **BVMB portal** for real-time reporting and monitoring.
   c. **community engagement** involving schools, Panchayats, NGOs, and religious leaders.
4. **Pradhan Mantri Matru Vandana Yojana (PMMVY):** Provides financial assistance to pregnant women and lactating mothers to improve their health and nutrition.
5. **Ujjawala Scheme (Anti-Trafficking):** Scheme to prevent trafficking and rescue, rehabilitation, and reintegration of victims of trafficking for commercial sexual exploitation.

Legal Empowerment

1. **POSH Act (2013):** Aims to ensure safe workplaces for women by addressing sexual harassment.
2. **Bharatiya Nagarik Suraksha Sanhita:**
   a. **Section 69:** Criminalizes sex based on false promises (employment/marriage).
   b. **Section 80:** Prescribes minimum 7 years to life imprisonment for causing a woman's dowry death.
   c. **Section 85:** Punishes husbands or relatives for physical or mental cruelty against a woman.
   d. **Section 173:** Allows women to file a "Zero FIR" at any police station regardless of jurisdiction.
3. **Code on Wages (2019):** Prohibits gender-based discrimination in wages and recruitment for the same or similar work.
4. **Maternity Benefit Act:** Grants 26 weeks of paid leave and mandatory crèche facilities in large offices.
5. **Social Security Code(2019):** Allows women to work night shifts provided employers ensure safety and transportation.
6. **Hindu Succession Act:** Retrospective equal inheritance rights for daughters.

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7. **Vishaka v. State of Rajasthan (1997)**: The Supreme Court laid down the Vishaka Guidelines to address sexual harassment at the workplace
8. **Shayara Bano v. Union of India (2017)**: The Court declared instant triple talaq unconstitutional
9. **Joseph Shine v. Union of India (2018)**: The Court decriminalized adultery, stating that the law treated women as male property and violated their dignity and autonomy.
10. **Indian Young Lawyers Association v. State of Kerala (2018)**: The Court allowed women of all ages to enter the Sabarimala temple
11. **Vineeta Sharma v. Rakesh Sharma (2020)**: The Court upheld daughters' equal rights in Hindu ancestral property.
12. **Secretary, Ministry of Defence v. Babita Puniya (2020)**: The Court ruled that women officers in the Indian Army are entitled to permanent commission and command roles.
13. **In Dr. Jaya Thakur case**, SC recognized **Menstrual Health and Hygiene (MHH) as a fundamental right under Article 21**.

Pre-Conception and Pre-Natal Diagnostic Techniques (PC&PNDT) Act, 1994 is a landmark social legislation in India designed to curb **female foeticide** and arrest the declining **child sex ratio**.

- It strictly bans **sex selection** (Section 3A) and mandates that all genetic counseling centers, laboratories, and clinics must be **registered**. Unregistered facilities are illegal.
- Diagnostic tests are permitted **only** for detecting genetic abnormalities, metabolic disorders, or congenital anomalies.
- **Any advertisement** (print, electronic, or otherwise) relating to pre-conception or prenatal sex determination is a **criminal offense.**
- The Act is monitored by the **Central and State Supervisory Boards**. It grants authorities extensive powers of **search and seizure** to inspect clinics for non-compliance.

Figure 8a. Sex Ratio by Birth when Child is not the Last

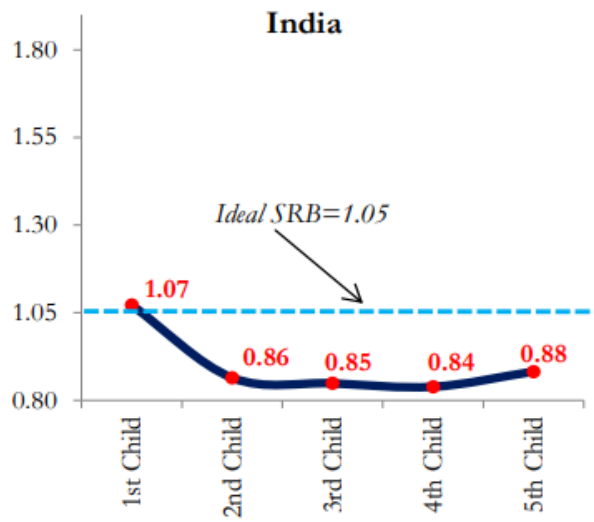
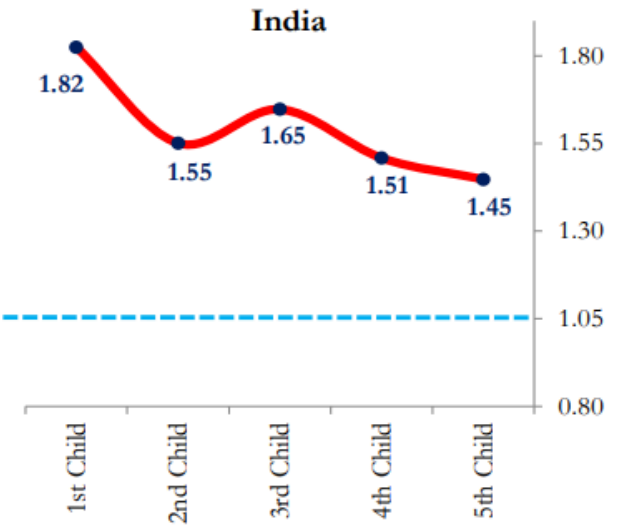


Figure 8b. Sex Ratio by Birth when Child is the Last



- All offenses under the Act are **cognizable, non-bailable, and non-compoundable**.
- Violators (including doctors and parents) face **3-5 years of imprisonment** and heavy fines ranging from **₹10,000 to ₹1,00,000+**.
- **The 2003 Amendment**: Expanded the Act's scope to include **pre-conception** sex selection and brought modern **ultrasound and imaging technologies** under strict regulatory control.

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Issues

- **Son Meta Preference**
- **Collusion:** mutual secrecy between service seekers and providers - low complaints
- **Infrastructure Deficits:** staff shortages, limited budgets, and inadequate training.
- **Judicial Ineffectiveness:** low conviction rates and significant delays in legal proceedings
- **Persistent Societal Demand:** Due to deep-rooted patriarchal norms and economic factors.
- **High profitability of illegal sex determination** leads practitioners to use portable machines and coded language to evade detection.

International Measures:

1. **UN SDG 5:** Gender equality and women's empowerment by 2030.
2. **CEDAW:** End discrimination; ensure equal rights.
3. **Beijing Declaration & Platform for Action (1995):** Economic participation and empowerment.

Measures to Further Strengthen Women's Empowerment in India

1. **Strengthen Legal and Institutional Frameworks**
   a. Establish **fast-track courts** for crimes against women as per **Justice Verma Committee** recommendations.
   b. Expand **One Stop Centres (OSCs)** under **Mission Shakti** for integrated legal, medical, and counselling support.
   c. Conduct **gender-sensitisation training** for police and enforce strict accountability mechanisms.
2. **Promote Gender-Responsive Education**: Scale up STEM scholarships and introduce gender-sensitisation modules in school curricula.
3. Expanding **"SHE-Mart" District Cooperatives** as specialized logistics and marketing aggregators for products made by rural SHGs
4. **Expanding the "Care Economy" Infrastructure:** Eg. **"Integrated Anganwadi-cum-Creche" centers.**
5. Apply the **ILO's 5Rs** (recognition, reduction, redistribution, reward, representation) **to address caregiving imbalances.**
6. **Enhance Political Representation**
   a. Expedite implementation of **Nari Shakti Vandan Adhiniyam (2023)** ensuring **33% reservation for women** in legislatures.
   b. Strengthen **Model Women-Friendly Gram Panchayats (2025)** and leadership training for women representatives.
7. **Ensure Comprehensive Healthcare**
   a. Use digital tools like **SUMAN SAKHI chatbot (MP)** for maternal care support.
   b. Address **anemia and malnutrition** through **Poshan Abhiyaan** and **ICDS**.
8. **Foster Digital and Financial Inclusion**
   a. Expand **PMGDISHA** and **Bank Sakhi** model for rural finance access.
   b. Promote women-led enterprises under **Mudra Yojana** and **Stand-Up India**.
9. **Empower Women in Informal and Gig Sectors**
   a. Extend **Social Security Code** benefits- maternity, health insurance-to gig and informal workers.
   b. Strengthen **SHG networks under DAY-NRLM** (10 crore women in 9 million SHGs) and **Lakhpati Didi** initiatives.
10. **Create Women-Centric Urban Infrastructure**
    a. **"Gender-Responsive Urbanism":** Eg. integrated **"Care-Corridors" (proximity to schools, clinics, and markets)** & **"Women's Safety and Utility Audits"**

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b. Expand **Sakhi Niwas hostels** under Mission Shakti (2024) for working women.
c. Increase **affordable childcare centres** under the **National Creche Scheme** to support working mothers.

Kolhapur's 'Palakmantri Makan-Dukan' scheme: Supports rural women in setting up grocery shops adjacent to their homes.

Human Trafficking in India

As per **UN,** "Human trafficking involves recruitment, transportation, transfer, harbouring, or receipt of persons, by means of the threat or use of force or other forms of coercion, for the purpose of exploitation."

Causes behind human trafficking

1. **Poverty and Debt:** Chronic financial distress forces families to send children to cities for work, leading to exploitation. **85%** of trafficked victims lived below the poverty line **(NSO)**
2. **Demand:** Demand for cheap domestic help, sex work, and surrogacy fuels trafficking networks.
3. **Skewed Sex Ratios:** Scarcity of women in states like Haryana/Rajasthan drives the demand for "bought brides" through **"Bride Trafficking"**
4. **Organised Trafficking Chains:** Trafficking networks operate through layered and fragmented structures across source, transit, and destination areas
5. **Climate-induced Distress Migration** makes victims in disaster-prone areas highly vulnerable to fraudulent recruiters.
6. **"Cyber-scam trafficking":** Traffickers use social media and gaming apps to lure youth with fake job/educational offers.
7. **Illegal Organ Trade:** High demand for transplants in the private sector lures poor donors into "legal-looking" organ rackets.
8. Despite **Bonded Labour Abolition Act 1976**, persists in brick kilns, stone quarries. **Eg. estimated 20-65 million Indians** in bonded labour
9. **Porous International Borders:** Soft borders with Nepal and Bangladesh facilitate the easy movement of victims without valid documents.
10. **Corruption in Local Administration: Eg.** "nexus networks" in red-light districts of Mumbai and Kolkata.
11. **Lack of Education and Awareness**: Illiteracy reduces awareness of rights, legal protections, and traffickers' tactics.

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12. **Weak Law Enforcement and Low Conviction Rates**
    a. **Conviction rate under the Immoral Traffic (Prevention) Act (ITPA)** is consistently below 20% (NCRB 2024).
    b. Victims are often **treated as offenders** rather than survivors, discouraging reporting.

Government Policies and Initiatives to Combat Human Trafficking

1. **Article 23(1)**: Prohibits trafficking in human beings and forced labour.
2. **Legal Measures:**
    a. **BNS Sections 143 and 144** provide stringent punishments, including life imprisonment for "aggravated trafficking."
    b. **Immoral Traffic (Prevention) Act (ITPA), 1956:** Criminalizes trafficking for the purpose of prostitution and provides for rescue, rehabilitation, and punishment.
    c. **POCSO Act, 2012:** Provides a gender-neutral framework to protect children from sexual trafficking.
    d. **Bonded Labour System (Abolition) Act, 1976:** Specifically targets labor-based trafficking.
    e. **Child Labour (Prohibition and Regulation) Act, 1986 (amended in 2016)**: Prohibits employment of children in hazardous occupations.
3. **Government Schemes/Policies:**
    a. **1,000+ Anti-Human Trafficking Units established across India for specialised investigation;**
    b. **Mission Shakti (Shakti Sadan):** Provides integrated relief and rehabilitation homes for victims.
    c. **NDHTO:** National Database of Human Trafficking Offenders for real-time tracking of criminals.
    d. **Operation AAHT:** A major railway-based rescue operation conducted by the Railway Protection Force (RPF).
    e. **Ujjawala Scheme:** Dedicated to the prevention, rescue, and rehabilitation of victims of commercial sexual exploitation.
    f. **Swadhar Greh Scheme**: Provides temporary shelter, food, clothing, counseling, and vocational training for women in difficult circumstances
    g. **Operation Smile & Operation Muskaan** (By Ministry of Home Affairs): To trace and rescue missing children
    h. **Nirbhaya Fund:** Continues to fund the **Women Help Desks (WHDs)** in over 13,000 police stations to make them "survivor-friendly" and approachable for victims seeking escape.
    i. **Cyber-Trafficking Units:**. CCPWC units utilize AI and OSINT to monitor digital grooming, dark web activities, and fraudulent online recruitment trafficking patterns.
4. **Supreme Court Judgments:**
    a. **Bachpan Bachao Andolan v. UoI (2011):** SC directed comprehensive measures for child trafficking & mandatory victim compensation.
    b. **Vishal Jeet v. Union of India:** SC ordered the Central Advisory Committee on prevention of trafficking.
    c. **Exploitation of Children in Orphanages (2017):** Established that "missing children" must be presumed trafficked.
    d. **Gaurav Jain v. Union of India:** Highlighted the right to rehabilitation and dignity for progeny of sex workers.
5. **International Measures:**
    a. **Palermo Protocol:** Emphasizes **3Ps approach**: **Prevention**, **Protection**, and **Prosecution**. India is a **signatory**.
    b. **UN Convention against Transnational Organized Crime**: Framework for **international cooperation**, **extradition**, and **mutual legal assistance**. **India ratified it in 2011.**
    c. **Operation Liberterra III (2026):** Global INTERPOL operation targeting transnational trafficking networks.
    d. **SAARC Convention:** Regional cooperation for preventing trafficking in women and children.

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e. **Bilateral Agreements:** Specific protocols exist with **Bangladesh, UAE, Cambodia, and Myanmar** for the joint investigation of trafficking syndicates and the safe return of victims.

Way Forward

1. **Speed up the passage** of the Trafficking in Persons (Prevention, Care and Rehabilitation) Bill
2. Recommendations of **National Commission for Protection of Child Rights**
   a. Identify **trafficking-prone areas** and map **children at risk** using defined indicators (school dropouts, migration etc).
   b. Ensure **convergence of welfare schemes** to address "push factors"
   c. Strengthen **Panchayat-level vigilance** through Village Child Protection Committees, Anganwadi workers, and local police.
   d. Sensitise and train **local police and RPF** to identify suspicious movement of children and accompanying adults.
   e. Conduct **regular inspections** of factories, brick kilns, massage parlours, domestic work sites etc.

Recently, SC laid down guidelines to evaluate evidence of victims in child trafficking cases.

- Judicial scrutiny must **recognise Socio-economic and cultural vulnerability of child** victim
- **Minor inconsistencies** in the child's testimony must not discredit his evidence.
- **Sole testimony of the victim is sufficient** if it is credible and convincing.
- Judicial evaluation **must show sensitivity** as recounting abuse causes **psychological trauma, stigma and fear of retaliation**.
- A trafficked child must be treated as an **"injured witness"**& not as an accomplice

- The **United States' Trafficking Victims Protection Act** offers a comprehensive legal framework, including protection, prosecution, and prevention.
- **Brazil's Blue Heart Campaign** uses media, community engagement, and public education to fight trafficking through grassroots mobilization.

Children

"The children of today will make the India of tomorrow. The way we bring them up will determine the future of the country." - **Jawaharlal Nehru**

"Children are the world's most valuable resource and its best hope for the future."- **John F. Kennedy**

"There can be no keener revelation of a society's soul than the way in which it treats its children." - **Nelson Mandela**

Constitutional Provisions

1. **Article 15(3):** Empowers the State to make **special provisions** for women and children.
2. **Article 21A:** Makes **Free and Compulsory Education** a Fundamental Right for children aged 6-14 (86th Amendment).
3. **Article 24:** Prohibits the employment of children below 14 years in any **hazardous factory or mine**.
4. **Article 39(e) & (f):** (DPSP) Mandates the State to ensure children are protected against exploitation and material abandonment.
5. **Article 45:** (DPSP) Directs the State to provide **Early Childhood Care and Education (ECCE)** for all children until they complete the age of six years.

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6. **Article 51A (k):** (Fundamental Duty) Casts a duty on parents/guardians to provide opportunities for education to their child between 6-14 years.

Major Laws

1. **Juvenile Justice (Care and Protection of Children) Act, 2015:** Focuses on rehabilitation of children in conflict with the law and those in need of care.
2. **POCSO Act, 2012:** A gender-neutral law providing stringent punishment for sexual offenses against minors.
3. **Prohibition of Child Marriage Act, 2006:** Bans marriage under 18 for girls and 21 for boys. (Note: As of 2026, the proposal to increase the age for girls to 21 is a key administrative debate).
4. **RTE Act, 2009:** Provides the legal framework for the implementation of Article 21A.
5. **Child Labour (Prohibition & Regulation) Amendment Act, 2016:** Complete ban on child labor below 14 years in all occupations.

Major Challenges in Child Empowerment in India

Health and Nutrition

1. **The "Double Burden" of Malnutrition:** Stunting (35.5% in NFHS-5) and wasting (19.3%) + rising trend of **childhood obesity** due to sedentary lifestyles and processed diets.

> Nearly **15 million children** (5-9 years) and over **26 million children** (10 - 19 years) in **India** were **overweight or obese** in 2025. (WHO)

1. **Anaemia Crisis:** Over **67% of children** aged 6–59 months are anaemic (NFHS-5).
2. **Emerging Mental Health Issues:** A rising burden of anxiety and "digital eye strain" due to increased screen time.

Education

1. **Learning Crisis: ASER 2023**: Only **42.8% Class 5 students** can read Class 2 text; only 25.6% can do basic division.
2. Under **NIPUN Bharat**, only **55%** of Grade 3 children have achieved "Grade Level Competency" in reading and math.
3. **High Drop-Out Rates: Secondary drop-out 14.1%**. Girls are particularly affected due to safety, menstruation, and marriage.
4. **Digital Divide in Education:** Rural children face limited internet, smartphone access.
5. **Poor School Infrastructure:** Many schools lack toilets, safe drinking water, electricity, libraries

Child Labour

1. **4.35 million** children (Census 2011)
2. **As per 2025 UNICEF-ILO** report, **8–10 million children work in** informal work.

Crimes Against Children

1. **Total Cases:** 1,77,335 cases registered (9.2% increase over the previous reporting year).
2. In nearly **96% of POCSO cases**, the offender was known to the victim.

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Rising Suicides

1. **Child Suicides (Below 18):** account for **6%** of total suicides in India.
2. **Student Suicides:** Recorded at approximately **13,892 cases** annually.

Digital Divide

1. **Access Inequality:** Rural children are significantly less likely to operate a computer (**12.6% males, 7% females**) compared to their urban counterparts (**37.5% males, 26.9% females**).
2. **Gendered Digital Gap:** Girls are **56% less likely** to use mobile internet than boys.
3. **"Filter Bubble":** Algorithms prioritize engagement over truth, often pushing young users toward **extreme content or misinformation.**

Girl Child Specific Issues

1. **Child Marriage Persistence: 23.3%** (NFHS-5)
2. **Skewed Child Sex Ratio:** Female foeticide continues in "pockets" of economic prosperity
3. **Invisible Labor:** Girls are often pulled out of school for **domestic chores** or sibling care
4. **Menstrual Hygiene Barriers:** lack of functional girls' toilets in nearly **50,000 schools**

Modern Challenges

1. **Digital addiction**: Children face risks like cyberbullying, inappropriate content, internet addiction, and negative effects on sleep and physical activity.
2. **Rising Inequality**: Disadvantaged children face poverty, reduced access to services, and poorer educational outcomes.

Government Schemes and Policies

Health and Nutrition

1. **PM POSHAN:** Provides one hot cooked meal to 11.8 crore school children. Includes **"Balvatikas"** (pre-school children) and **School Nutrition Gardens** for experiential learning.
2. **Saksham Anganwadi & Poshan 2.0:** Focuses on reducing stunting and wasting through "Supplementary Nutrition." (**9.5 crore+ beneficiaries**.)
3. **Rashtriya Bal Swasthya Karyakram:** A screening for the "4 Ds" (Defects at birth, Deficiencies, Diseases, Developmental delays) & tertiary care to children identified during bi-annual school/Anganwadi check-ups.
4. **Mission Indradhanush 6.0:** Aims for **95% full immunization coverage**. It utilizes the **U-WIN portal** to digitally track every pregnant woman and child's vaccination schedule.
5. **Janani Shishu Suraksha Karyakram:** Ensures free and cashless services to sick infants (up to 1 year) in government health institutions, including drugs, diagnostics, and transport.

Education and Empowerment

1. **Samagra Shiksha (2026-27):** Integrates pre-school to Class 12, focusing on universal access, inclusive tribal hostels, special needs support, and bringing out-of-school children back into formal education.
2. **NIPUN Bharat Mission:** Targets **Foundational Literacy and Numeracy (FLN)**. The 2026 goal is for every child to achieve basic reading and math competencies by the end of **Grade 3**.
3. **PM SHRI (Schools for Rising India):** These are "showcase" schools that implement the **NEP 2020** in letter and spirit. (over **14,500 schools** have been upgraded).

Child Labour and Rehabilitation

1. **PENCiL Portal:** It allows citizens to report cases and ensures seamless coordination between state and district officers for rescue and rehabilitation of Child Labour.
2. **National Child Labour Project (NCLP):** Focuses on "mainstreaming" rescued children into formal schools by providing them with specialized training, bridge courses, and a stipend.

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3. **Mission Vatsalya:** Provides a monthly grant of **₹4,000 per child** to vulnerable families to prevent children from entering the workforce due to economic distress.
4. **Operation Muskaan:** Rescue operation by the **Ministry of Home Affairs** and Railway Protection Force (RPF) to trace missing children and those working in hazardous conditions.

Digital Empowerment

1. **PM e-VIDYA: "One Class, One Channel"** (40+ DTH channels) and the **DIKSHA portal**, which provides curriculum-linked e-content in 30+ Indian languages.
2. **SWAYAM & SWAYAM Prabha:** Offers free online courses from Class 9 to Post-Graduation.
3. **Atal Innovation Mission (Atal Tinkering Labs):** Established in over **10,000 schools**. Children use 3D printers, robotics, and IoT kits to solve local community problems.
4. **Cyber Jaagrookta Abhiyaan:** A dedicated program to train students in **"Digital Hygiene."**

Crimes Against Children (Safety)

1. **Mission Vatsalya:** Centrally Sponsored Scheme focuses on **Children in Need of Care and Protection** (orphans, abandoned, or distressed children) and **Children in Conflict** with Law (juveniles).
2. **POCSO e-Box:** online complaint box managed by the **National Commission for Protection of Child Rights** where children or guardians can report sexual offenses directly and safely.
3. **Child Helpline 1098 + 112 Integration**
4. **Safe City Projects:** Implemented in 8 major metros, these use **AI-enabled CCTV surveillance** and "Panic Buttons" in public spaces
5. **National Database on Sexual Offenders (NDSO):** accessible to law enforcement to track repeat offenders.

Way Forward for Children Empowerment

Health and Nutrition

1. **Lifecycle Approach:** Expand focus from infant survival to adolescent health, integrating mental health into **RBSK** screenings.
2. **Nutri-Hubs:** Scale **Poshan Vatikas** (kitchen gardens) in Anganwadis to provide fresh micronutrients. (Odisha's "Mo Upakari Bagicha.")
3. **Shock-Responsive Systems:** Build climate-resilient health infrastructure to prevent stunting during disasters. (NITI-UNICEF SOI 2026)
4. **WASH Convergence:** Link **Jal Jeevan Mission** to schools to ensure functional sanitation and curb nutrient loss. Eg. Swachh Vidyalaya Puraskar.
5. Scale up **School Health & Wellness Ambassadors** (Ayushman Bharat).
6. Integrate **menstrual hygiene, nutrition education**, and **mental health curriculum**.

Education and Learning

1. **NEP 2020 Recommendation**: Universalization of quality **Strengthening Early Childhood Care & Education** via **Anganwadi-pre-primary integration**
2. **Outcome-Led Pedagogy:** Pivot from enrollment to Grade-3 competency via **NIPUN Bharat** targets. Eg. Delhi's "Mission Buniyaad."
3. **Vocational Mainstreaming:** Mandate "Bagless" vocational exposure (coding, green-skills) from Class 6. Eg. Germany's Dual System.
4. **Inclusive Tech:** Deploy **AI-personalized tools** for Children with Special Needs (CWSN). Eg. Kerala's Barrier-Free Resource Centres.
5. **Retention Grants:** Increase **conditional cash transfers** for secondary school girls to reduce dropout rates. Eg. West Bengal's "Kanyashree."

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Elimination of Child Labour

1. **Supply Chain Audits:** Legally mandate **"Child-Labour Free" certifications** for high-risk exports (gems/textiles). Eg. GoodWeave/Rugmark certification
2. **Migration Tracking:** Link **e-Shram** data with schools to ensure "Education Continuity" for migrant children.
3. Formalize **Village Child Protection Committees** to conduct social audits of family enterprises. (NCPCR)
4. Expand **Mission Vatsalya** sponsorship for families in the highest Multidimensional Poverty (MPI) brackets.

Digital Empowerment

1. Ensure **Digital Equity** (One student, one device policy).
2. **Cyber Hygiene:** Integrate **"Digital Safety and Ethics"** into the national curriculum to combat deepfakes and grooming. Eg. Telangana's "Cyber Congress."
3. Provide high-speed broadband to 100% of schools in **Aspirational Districts** under UNICEF's GIGA Initiative.
4. Enforce the **DPDP Act 2023** to ensure verifiable parental consent for children's data.
5. Scale the **DIKSHA portal** with gamified STEM content in regional languages.

Crimes Against Children

1. Upgrade **Anti-Human Trafficking Units** with AI to monitor the Dark Web for minor recruitment.
2. Establish **Exclusive POCSO Courts** to mandate trial completion within 12 months.
3. **Community Vigilance:** Train "Police Mitras" to recognize early signs of abuse at the ward level.
4. Define **"Child-Friendly Legal System"** standards.

Child Participation and Governance

1. Institutionalize **Children's Advisory Councils** at school, block, and district level.
2. Promote **digital platforms for child feedback** on education, health, and safety.
3. **Indian Best Practice**: Kerala's **Balasabha** (child parliaments) at panchayat level.

A **child-centric lens** and **'whole-of-society' approach** must be mainstreamed in every scheme, budget, and institution for moving from tokenism to true empowerment.

As India approaches **Amrit Kaal**, true progress will be measured by **how secure, educated, and empowered its youngest citizens are**.

In **State of Uttar Pradesh v. Anurudh & Anr. (2026),** the SC expressed concerns over misuse of POCSO Act.

POCSO Act

- Defines a child as any person below **18 years of age**.
- **Gender Neutrality:** The Act protects all children regardless of gender and recognizes that both men and women can be perpetrators.
- **Graded Punishments:** Penalties are based on the gravity of the offence:
  - **Sexual Assault:** 3 to 5 years.
  - **Aggravated Penetrative Sexual Assault:** Minimum 20 years, extendable to the **death penalty** (introduced via 2019 amendment).
- **Mandatory Reporting:** Any person (including doctors, teachers, or neighbors) who knows or suspects an offence must report it. Failure to report is a criminal offence.
- **Child-Friendly Procedures:** *
- The child's statement is recorded at their residence or a place of their choice.
- Police officers recording statements must not be in uniform.

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- The child is not brought face-to-face with the accused during trial.
- Mandates the setup of **Special Courts** for speedy trials, with a legal requirement to complete the trial within **one year** of taking cognizance.

Major Issues & Implementation Challenges

- **The "Consensual Adolescent" Dilemma ("Romeo-Juliet Clause")-** Nearly **25% of POCSO cases** involve consensual romantic relationships between teenagers.
- **Abysmally Low Conviction Rates:** Despite a high disposal rate of 109% by Fast Track Courts in 2025, the actual conviction rate has declined to approximately **29%**.
- **Trial Delays and Pendency:** the average case duration is **510 days**. Over **46% of pending cases** have been stuck for more than two years.
- **Secondary Victimization:** many Special Courts lack separate waiting rooms or trained support persons, forcing children into intimidating judicial environments.
- **Weaponization in Family Disputes:** The law is sometimes misused by parents to break up socially disapproved consensual relationships or as leverage in matrimonial disputes.
- **Settlement Pressure:** Instances of families of victims facing extreme pressure from the accused's community to reach an out-of-court "settlement."

Arguments in favor of reducing age of consent

- **Adolescent Capacity:** 16-18 year-olds possess the cognitive maturity and digital awareness to make informed choices about relationships.
- Nearly 25% of POCSO cases involve consensual peers, making the current law out of sync with social reality.
- Prevents parents from **using POCSO as a tool to criminalize romantic choices**.
- **Global Benchmarks** is 16 (supplemented by "close-in-age" exemptions)
- Comprehensive sex education as a more effective deterrent for unsafe behavior than rigid criminal punishment.

Arguments Against

- Create loopholes for traffickers and predators to exploit minors under the guise of "consent."
- **Illusory Consent:** Studies show over 50% of abusers are known to the child; apparent consent in these cases is often the result of manipulation or dependency.
- **Risk of Silencing:** A lower threshold could discourage victims from reporting abuse.
- **Institutional Opposition:** The Law Commission (2023) warn that lowering the age would undermine efforts against child marriage and prostitution.
- **Social Maturity:** Adolescents lack the emotional maturity to handle the long-term consequences of premature sexual activity.

Juvenile Justice Act, 2015

- **Categorization of Offences:**
  - **Petty:** Punishment up to 3 years.
  - **Serious:** Punishment between 3 to 7 years.
  - **Heinous:** Punishment above 7 years.
- Children aged 16-18 accused of **heinous offences** can be tried as adults if the Juvenile Justice Board conducts a "preliminary assessment" of their mental and physical capacity
- **Juvenile Justice Board (JJB) & Child Welfare Committee (CWC):** *

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- **JJB:** Handles "Children in Conflict with Law" (CCL).
- **CWC:** Acts as the final authority for "Children in Need of Care and Protection".
- **Mandatory Registration of** Child Care Institutions (CCIs), whether run by the State or NGOs, must be registered. Non-compliance attracts a jail term and fines.
- The Act gave statutory status to the **Central Adoption Resource Authority (CARA)**, making the adoption process faster and more transparent.
- **No Death Penalty or Life Imprisonment:** No child, regardless of the crime, can be sentenced to death or life imprisonment without the possibility of release.

Issues highlighted in India Justice Report, 2025.

- Over **55%** of cases before **Juvenile Justice Boards (JJBs)** were pending
- **24% of JJBs** were **not fully constituted**, **30% of JJBs lack a legal services clinic**
- Lack of **inter-agency coordination**, weak **data-sharing**, and poor **monitoring mechanisms**

Child adoption in India

Recent News

- Recently, the SC ruled that **all adoptive mothers are entitled to 12 weeks of maternity leave**, regardless of the child's age
- **Waitlist Crisis:** for every **1 child** legally free for adoption, there are now **13 prospective adoptive parents (PAPs)** waiting.

Key Legal Provisions and Process

- **Hindu Adoption and Maintenance Act (HAMA), 1956:** Applies to Hindus, Buddhists, Jains, and Sikhs. It is a private act where adoption is finalized through a registered deed. (HAMA adoptions do not require CARA registration, a loophole often flagged for trafficking risks.)
- **Juvenile Justice (JJ) Act, 2015:** It mandates registration on the **CARINGS** (Child Adoption Resource Information and Guidance System) portal.
- **The Process (JJ Act):**
  - **Registration:** PAPs register on the CARINGS portal.
  - **Home Study Report (HSR):** A Specialized Adoption Agency (SAA) conducts a social and financial audit of the parents.
  - **Referral:** Parents are shown profiles of children legally free for adoption.
  - **Pre-Adoption Foster Care:** Once matched, the child moves in with the parents while legalities are finalized.
  - **Court Order:** The District Magistrate (DM) now has the power (since 2022) to issue adoption orders to speed up the process.

Major Issues

- **Wait-Time Inflation:** The average waiting period has risen to **3.5 years** due to the sheer volume of PAPs (over 30,000) versus available children (~2,500).
- Thousands of children live in Child Care Institutions (CCIs), but **only a fraction are declared "legally free" for adoption** due to slow bureaucratic social investigation reports.
- Over **80% of parents** only want infants (0–2 years), leaving older children and teenagers to "age out" of the system.
- **Return of Children (Dissolution):** A concerning trend where parents return children (often older or with special needs) after failing to bond, leading to severe trauma.

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- **HAMA vs. JJ Act Inconsistency:** lack of a central registry for HAMA adoptions makes it difficult to track children and prevent illegal "baby-selling" under guise of private adoption.
- **Special Needs Neglect:** Only a tiny percentage of domestic adoptions involve children with physical or mental disabilities.
- **Post-Adoption Support:** There is a critical lack of professional psychosocial counseling for families after the adoption is finalized.
- **Illegal Markets:** High wait times drive desperate parents toward illegal "gray market" adoptions, bypassing safeguards.

Way Forward

- All Child Care Institutions must be digitally linked to the CARA database to ensure every orphan/abandoned child is identified and processed for adoption within 6 months.
- **Unified Law:** There is a growing call to bring all adoptions (including HAMA) under a single, transparent regulatory umbrella to ensure child safety.
- **Government subsidies or enhanced healthcare benefits** for families adopting older or special-needs children could help balance the "infant-only" preference.
- **Foster-to-Adopt Programs:** Strengthening "Foster Care" as a transition phase can help older children find permanent homes more effectively.
- **Professionalizing Social Work:** District Child Protection Units (DCPUs) need more trained social workers to complete "legal freedom" investigations in a time-bound manner

Scheduled Caste

"Social justice is achieved when the most marginalized are heard and uplifted."

"The test of a civilization is the way that it cares for its helpless members." - **Pearl S. Buck**

Under **Article 366 (24)** of the Constitution of India, Scheduled Castes are defined as such castes, races, or tribes as are deemed under **Article 341** to be Scheduled Castes.

They represent communities historically subjected to the practice of untouchability and social exclusion.

Key Facts

- **Population: 20.14 crore (~16.6%)** of India's population (Census 2011).
- **1,234+ communities** notified as Scheduled Castes across India.
- The term **"Scheduled Caste"** was first used in the **Government of India Act, 1935** under British rule.

Key Constitutional Provisions & Laws

1. **Article 17:** Abolition of Untouchability; its practice in any form is a punishable offense.
2. **Article 15(4) & 16(4):** Empowers the State to make special provisions for the advancement of SCs and reservation in public employment.
3. **Article 46:** Directs the State to promote the educational and economic interests of SCs/STs and protect them from social injustice.
4. **Article 330 & 332:** Reservation of seats in Lok Sabha and State Legislative Assemblies.
5. **Article 338: National Commission for Scheduled Castes (NCSC)** to monitor safeguards.
6. **The SC/ST (Prevention of Atrocities) Act, 1989:**

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a. 22 atrocity offences
b. **Special Courts** in every district
c. Mandatory FIR registration
d. Time-bound investigation (60 days)
e. Witness protection
f. Compensation provisions

7. **Protection of Civil Rights Act, 1955:** Prescribes punishment for the practice of untouchability.

Major Issues Faced by Scheduled Castes

1. **Caste-Based Atrocities** Despite stringent laws like the POA Act. **Eg.** Una flogging case (2016)
2. **Caste-Based Discrimination in Daily Life:** 27% Indians admit practising untouchability in some form. (**NSO survey 2024**) **Eg. Two tumbler system** in tea stalls in rural India
3. **Persistent Manual Scavenging** despite legislative bans and mechanization efforts. (96% manual scavengers are dalits)
4. **Land Alienation: Eg.** 71% of SC households in rural India are "landless" or "near-landless"
5. **Digital Exclusion: Eg.** NFHS-6 preliminary data indicates a 15% digital literacy gap between SCs and general categories.
6. **Gendered Caste Violence:** Dalit women face **triple marginalization**-of caste, gender, and class. Eg. Hatras rape case
7. **Residential Segregation:** Informal "social boundaries" in villages and ghettos in cities restrict access to quality infrastructure.
8. **Barriers to Higher Education:** High dropout rates at the PG and PhD levels due to institutional bias and financial distress. Eg. Rohith Vemula case
9. **Limited Entrepreneurial Credit:** Difficulty in accessing formal bank loans due to lack of collateral and subtle systemic discrimination.
10. **Occupational "Enclosure":** Over-representation in hazardous informal sectors like leather tanning, stone crushing, and sanitation work.
11. **Justice Delivery Delays:** Excessive pendency of POA cases in Special Courts leads to low conviction rates and victim fatigue.
12. **The "Glass Ceiling" in the Private Sector due to l**ack of affirmative action. **Eg.** Dalit representation in boardrooms remains below 2%.
13. **Class within caste**: "Creamy layer" within SCs absorbs benefits

Steps Taken to Empower Scheduled Castes in India

1. **Pradhan Mantri Anusuchit Jaati Abhyudaya Yojana (PM-AJAY) - Umbrella scheme** combining 3 schemes:
   a. **Adarsh Gram Yojana**: Develops 50%+ SC population villages
   b. **Grants-in-Aid to States** for SC welfare
   c. **Hostels for SC Students Outlay:** ₹2,000+ crore annually.
2. **Reservation in Education and Jobs**
   a. **15% in central educational institutions** and **central government jobs**
   b. **Reservation in promotions** (Article 16(4A))
3. **Pre-Matric & Post-Matric Scholarships for SCs - 40+ lakh students** benefited annually
4. **SHRESHTA Scheme:** Provides high-quality residential education in private schools for meritorious SC students who lack access to top-tier schooling.
5. **Vigyan Dhara (2025)** Includes specific components for **inclusive participation in R&D and deep-tech**, ensuring SC representation in India's digital economy.

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6. **National Overseas Scholarship:** For SC students for higher studies abroad (Master's, PhD).
7. **Self-Employment Scheme for Liberation of Scavengers (SRMS):** Rehabilitation of manual scavengers under **Manual Scavenging Act, 2013**.
8. **NAMASTE Scheme (2022) - National Action for Mechanized Sanitation Ecosystem**: Eliminate manual sewer cleaning by 2026.
9. **Special Component Plan / Scheduled Castes Sub-Plan (SCSP): Mandates ~16% of central budget** allocation for SC welfare.
10. **NSFDC (National Scheduled Castes Finance and Development Corporation):** Concessional loans for SC entrepreneurs.
11. **Stand-Up India (2016):** Loans **₹10 lakh - ₹1 crore** for SC/ST/women entrepreneurs.

Major Supreme Court Judgments

1. **State of Kerala v. N.M. Thomas (1976): Reservation in promotions** upheld
2. **M. Nagaraj v. UoI (2006): Reservation in promotions** subject to **Backwardness, Inadequate representation, Administrative efficiency**
3. **Jarnail Singh v. Lachhmi Narain Gupta (2018):** Modified Nagaraj judgment to ease promotion reservation. States need not show **quantifiable data** for backwardness of SCs/STs
4. **State of Punjab v. Davinder Singh (August 2024):** States can **sub-classify Scheduled Castes** for reservation
5. **Subhash Kashinath Mahajan:** Diluted SC/ST PoA Act provisions. **SC/ST PoA Amendment Act, 2018 reversed Mahajan judgment**
6. **Shakti Vahini v. UoI (2018):** SC issued guidelines against **honour killings** and **khap panchayats** targeting inter-caste couples.
7. **State of Karnataka v. B.K. Pavitra (2019):** Upheld **consequential seniority** for SCs/STs in promotions.
8. **Safai Karamchari Andolan & Ors. v. Union of India & Ors. (2014) - SC ordered:**
   a. The identification of all manual scavengers.
   b. The provision of alternative employment and rehabilitation measures for them.
   c. Compensation of ₹10 lakh to the families of those who died while cleaning sewers and septic tanks.
9. **Bandhua Mukti Morcha v. Union of India (1984)**: The Supreme Court recognized bonded labor as a violation of human rights

Way Forward

1. **Implementing Sub-Classification: Justice Usha Mehra Commission** (2007) recommendations can serve as a blueprint for identifying internal backwardness.
2. Completing **100% mechanization of sewer cleaning** via drones and robots under the **NAMASTE** scheme. **Eg.** using the **Bandicoot robot**
3. **Data-Driven Affirmative Action: Eg.** Creating a **National Diversity Index** for government and public sector undertakings.
4. **Mandatory Diversity Audits:** Encouraging the private sector to adopt voluntary diversity audits and inclusive hiring policies.
5. **Specialized Judicial Training:** Sensitizing judges and police officers to the "nuances of caste-based prejudice" to improve POA conviction rates.
6. **Land Reform Reinvigoration: Eg.** Promoting the "Land Bank" model where the state purchases land for landless SC cooperatives.
7. **Promoting "Dalit Capitalism":** Incentivizing SC-owned MSMEs through a 4% mandatory procurement policy by Central Ministries.

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8. **Mental Health and Counseling:** Establishing dedicated counseling centers in universities to address "institutional casteism" and student suicides. (**Justice Roopanwal Committee**)

Tribals

Scheduled Tribes (STs) are **indigenous communities** who are **historically marginalized and socially disadvantaged**, and are officially recognized under **Article 342**.

- There are over 730 Scheduled Tribes notified under Article 342 of the Constitution
- **Population:** Over **10.45 crore** (as per Census 2011).
- **Literacy Rate:** ~59% (National avg: ~74%), with tribal female literacy lagging at ~49%.
- **Health:** IMR is ~44 (national average ~35); high prevalence of **Sickle Cell Anemia**.
- **Urbanization:** Only **10% of STs** live in urban areas

Common Criteria Used for Identification (Lokur Committee, 1965):

1. **Primitive traits**: Lack of modern technological advancement and a deep connection to ancestral traditions. Tribal advocates criticize the word "primitive" as derogatory and outdated.
2. **Distinctive Culture:** Unique customs, language, and social structures.
3. **Geographical Isolation:** Historical residence in remote or hilly terrains.
4. **Shyness of Contact:** Reluctance to interact with the community at large.
5. **Backwardness:** Significant socio-economic disparities compared to the national average.

Particularly Vulnerable Tribal Groups (PVTGs)

These are the most marginalized and disadvantaged among tribal communities in India. Originally called "Primitive Tribal Groups," they were renamed in 2006. There are **75 recognized PVTGs** across 18 States and one UT (A&N Islands).

Criteria (Dhebar Commission):

- **Pre-agricultural level of technology:** Reliance on hunting/gathering.
- **Low level of literacy:** Significant gap compared to other STs.
- **Economic backwardness:** Subsistence-level economy.
- **Declining or stagnant population:** Critical demographic vulnerability.

Denotified Tribes (DNTs)

- Denotified Tribes are communities that were unjustly classified as **"criminal tribes"** under British colonial rule. The **Criminal Tribes Act of 1871** branded them as inherently criminal, imposing severe restrictions and constant surveillance.
- After India's independence in 1947, the law was abolished, and these groups were officially **"denotified."**

Major Issues Faced by Tribal Population

1. **Development-Induced Displacement** (Internal Colonialism - A.R. Desai): Tribal communities form over 60% of displaced. Eg. Sardar Sarovar Dam
2. **Loss of Forest Rights**: Only 47% of individual claims under the Forest Rights Act (2006) have been approved, with widespread rejections.
3. **Land Alienation:** Non-tribals encroaching on protected lands through illegal transfers despite protective laws like the PESA Act.

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4. **Linguistic Erosion: Eg.** Over **196 tribal languages** in India are currently classified as "vulnerable" or "endangered" (UNESCO 2026).
5. **Sickle Cell Disease:** Genetic predisposition combined with poor specialized healthcare infrastructure in remote tribal belts.
6. **Institutional Education Gap: Eg. ASER 2025** highlights a 15% transition-gap for ST students from middle to high school.
7. **Environmental Vulnerability:** Tribals are "climate-change refugees," losing livelihoods due to erratic rains and forest degradation.
8. **Digital Exclusion:** Due to lack of electricity and internet hinders access to DBT and online education.
9. **Exploitation by Middlemen:** Minor Forest Produce collectors forced to sell below MSP due to lack of market-linkages. **Eg. Santhal** honey collectors in Jharkhand.
10. **Ethnic Conflict and Security: Eg.** The **Manipur ethnic crisis (2023-26)**
11. **Health Insecurity and Malnutrition:** High rates of child malnutrition, anemia, and lack of access to health services.
12. **Token Representation:** Despite reservations, tribal voices are excluded from key decisions on land and development. Eg. PESA violations during the Niyamgiri mining case.
13. **Naxalism**: In areas like Bastar (Chhattisgarh), tribals face violence and surveillance from both insurgents and state forces, deepening mistrust and alienation.

Recent Case Studies

- **Dhamtari Habitat Rights (2025):** The first instance of **Kamar PVTG** receiving comprehensive habitat rights in Chhattisgarh.
- **Hasdeo Aranya Resistance:** Ongoing tribal-led movement against coal mining
- **Dongria Kondh vs. Mining:** Successful use of the "Gram Sabha" power to stop mining on the **Niyamgiri Hills**.
- **Ziro Valley Sustainability:** The **Apatani tribe's** unique wet rice-cum-fish cultivation being promoted as a "Climate Best Practice."
- **Sahariya Tribe, Madhya Pradesh (2023)** Severe malnutrition deaths in Shivpuri and Sheopur districts.
- **Manipur Ethnic Conflict (2023)** Violent clashes over Meitei demand for ST status led.

Steps Taken to Empower Tribal Population in India

1. **Forest Rights Act (FRA), 2006** recognizes **individual and community forest rights**. **SC/ST (Prevention of Atrocities) Act, 1989** ensures **legal protection** against discrimination and violence.
2. **PM Janjatiya Unnat Gram Abhiyan:** Targets 63,843 tribal-majority villages across 549 districts. 25 specific interventions by 17 ministries to saturate gaps in housing (PMAY-G), water (Jal Jeevan Mission), and mobile connectivity.
3. **PM-JANMAN (₹24,104 Cr):** Focused on saturating 75 PVTGs with housing, water, and connectivity.
4. **Vanbandhu Kalyan Yojana** for **holistic development** (education, health, livelihood).
5. **Eklavya Model Residential Schools (EMRS):** Quality residential schooling on par with Navodaya Vidyalayas.
6. **TRIFED (Van Dhan Vikas Kendra):** Promoting tribal startups through Minor Forest Produce value addition.
7. **PM Janjatiya Vikas Mission:** Strengthening tribal entrepreneurship with a ₹1612 Cr outlay.
8. **Sickle Cell Elimination Mission:** Dedicated focus on screening and counseling in tribal clusters.
9. **Minimum Support Price (MSP) for MFP:** Covers **87 items** including honey, tamarind, and mahua.

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10. **Development of PVTGs Scheme:** Demand-driven funding for infrastructure in PVTG habitats.
11. **National Overseas Scholarship:** Financial aid for ST students to pursue Masters/PhD abroad.
12. **Adivasi Mahila Sashaktikaran Yojana:** Loans at highly concessional rates for tribal women entrepreneurs.
13. **Dharti Aba Janjatiya Gram Utkarsh Abhiyan:** Integrating 17 ministries for saturation of services in 63,000 villages.
14. **Janjatiya Gaurav Divas** honours **tribal freedom fighters** and cultural pride
15. **PESA Act 1996:** Extend Panchayati Raj provisions to Fifth Schedule Areas, empowering tribal communities through self-governance.

SC Judgments

1. **Davinder Singh (2024):** Permits sub-classification within STs to prioritize benefits for the most backward sub-groups based on empirical data.
2. **Samatha Case (1997):** Prohibits leasing tribal land to private companies; mandates 20% profit-sharing for local socio-economic development.
3. **Niyamgiri Case (2013):** Empowers Gram Sabhas with veto power over projects infringing upon tribal cultural or religious rights.
4. **Madhu Kishwar (1996):** Protects tribal women's right to livelihood by allowing them to possess and cultivate ancestral land.

Way Forward

1. **Strengthening Gram Sabhas:** Shift from "consultation" to "mandatory consent" for all land-use changes. (**Xaxa Committee**)
2. **"Right to Return"** for unutilized acquired land. (**Xaxa Committee**)
3. **Multilingual Pedagogy:** Implement primary education in **tribal mother tongues** to improve retention.
4. **Digital "Last Mile":** Use **LEO satellites** to provide internet to remote tribal "shadow" areas.
5. **Habitat Rights Saturation:** Expedite habitat rights for all 75 PVTGs to ensure ancestral territory security.
6. **Organic Branding:** Use Blockchain to trace and market "Tribal-Organic" products for global exports.
7. **Healthcare Mobility:** Deploy **"Hospital-on-Wheels"** specifically for remote PVTG clusters.
8. **Climate-Resilient Livelihoods:** Integrate **Traditional Ecological Knowledge (TEK)** into the National Action Plan for Climate Change.
9. **VPPs (Village Protection Plans):** Formalize village committees to monitor and stop land alienation in real-time.
10. **Sustainable Eco-Tourism:** Promote "Community-led Tourism" where tribes own the revenue, preventing "human zoo" tourism.

Tribal Panchsheel policy

- **Non-Imposition (Developing on Own Genius)**
- **Respect for Land and Forest Rights**
- **Indigenous Administration:** Minimize the influx of "outsiders"
- **Avoidance of Over-Administration:** Work through existing social and cultural institutions (like the Gram Sabha).
- **Quality over Statistics:** Judge results by the degree to which the community has become self-reliant and confident without losing its identity.

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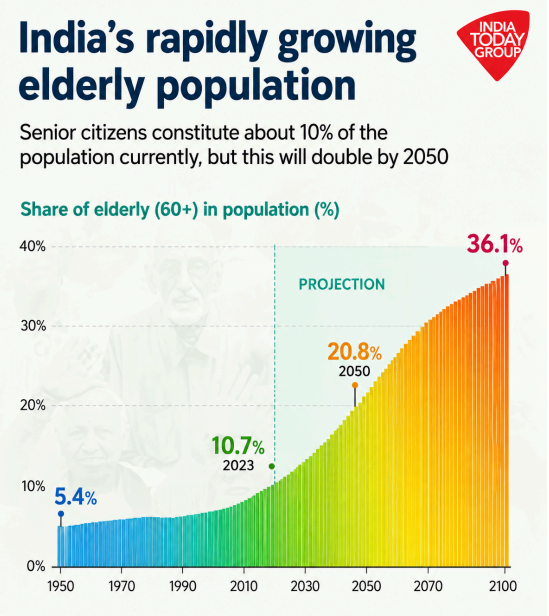
Elderly

According to the **Maintenance and Welfare of Parents and Senior Citizens Act, 2007**, a **"Senior Citizen"** is any person who is a citizen of India and has completed the age of **60 years or above**.

Key Data

1. Approximately **11.5%** of India's population is over 60 (projected to reach 20% by 2050).
2. **Feminization of Ageing: 53%** of the elderly are women
3. The **old-age dependency ratio** has climbed to **17.5** per 100 working-age people.
4. Nearly **71%** of India's elderly reside in rural areas with limited healthcare access.
5. only ~18% India's elderly are insured



Constitutional and Legal Measures

1. **Article 41:** Mandate the State to provide public assistance in cases of old age.
2. **Maintenance and Welfare of Parents and Senior Citizens Act, 2007:** Makes it a legal obligation for children/heirs to provide maintenance; provides for "Senior Citizen Tribunals."
3. **Hindu Adoption and Maintenance Act, 1956:** Specifically identifies the obligation of children to maintain aged parents.

Major Initiatives for Elderly

1. **Atal Vayo Abhyuday Yojana (AVYAY):** This is the umbrella scheme of the Ministry of Social Justice and Empowerment. It covers several critical sub-programs:
   a. **Rashtriya Vayoshri Yojana (RVY):** Provides free assistive devices (wheelchairs, hearing aids, spectacles, and dentures) to senior citizens from BPL backgrounds or those with a monthly income below ₹15,000.
   b. **SACRED Portal:** An IT portal that matches senior citizens seeking work with private sector recruiters.
   c. **SAGE:** A platform to support startups developing innovative products for the elderly, such as smart healthcare devices and specialized housing.
   d. **Elderline (14567):** A national, 24/7 toll-free helpline providing emotional support, legal guidance, and information on government schemes.
2. **PM Vaya Vandana Yojana:** Pension scheme providing an assured net return of ~7.4% per annum.
3. **National Programme for Health Care of the Elderly 2011(NPHCE):** Specialized geriatric units in district hospitals and CHCs.
4. **Indira Gandhi National Old Age Pension Scheme (IGNOAPS):** Monthly financial assistance for elderly below the poverty line.
5. **National Policy on Older Persons (NPOP), 1999** focuses on shelter, healthcare, welfare, and financial security.
6. The **2020 draft National Policy for Senior Citizens i**ntroduced integrated insurance, savings schemes, and tax structures to support the silver economy.
7. **Atal Pension Yojana** encourages retirement savings for informal sector workers, with pension starting at 60.

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Major Issues Faced by Elderly

1. **Income Insecurity: Eg.** 70% of the elderly is dependent for everyday maintenance, and **78% is living without any pension cover.**(NITI Report)
2. **Feminization of Poverty:** LASI report highlights that 65% of elderly women are fully dependent on others.
3. **Low Social Security:** Only 18% reported being covered by any health insurance, 28% are aware of any concession for senior citizens, and 24% reported problems in providing documents to avail services
4. Over **70% of widowed elderly in India are women**. They face **triple marginalization** - by age, gender, and widowhood - with higher poverty, lower pension access, and social exclusion.
5. **Digital Exclusion:** Shift to "Digital India" for banking and health (ABHA) alienates those without tech-literacy. Widespread mobile use exposes **digitally inexperienced seniors to scams,** turning a vital utility into risk.
6. **Mental Health & Loneliness:** One in three reported having depressive symptoms, and 32% reported low life satisfaction (NITI Report)
7. **Elder Abuse:** At least 5% of India's elderly population reported having experienced abuse which can be physical, sexual, psychological, or financial.(NITI Report)
8. **Chronic Geriatric Ailments: Eg.** NFHS-6 data shows 75% of elderly suffer from at least one chronic disease like cardiovascular issues and diabetes.
9. **Inadequate Geriatric Infrastructure:** India has fewer than 5000 specialized geriatricians for 150 million seniors and less than 50000 trained caregivers
10. **Economic Dependency Ratio:** Increasing pressure on the shrinking working-age population to fund healthcare and pension bills.
11. **Lack of Palliative Care:** Minimal access to end-of-life care and pain management in rural and semi-urban India.

Global Initiatives

1. **UN Decade of Healthy Ageing (2021–2030):** A global collaboration to improve the lives of older people.
2. India is signatory to the **United Nations Convention on Rights of Persons with Disabilities**
3. **Madrid International Plan of Action on Ageing (MIPAA):** First global agreement to recognize ageing as a developmental issue.
4. **SDG Goal 3:** Targets ensuring healthy lives and promoting well-being for all ages.

Global & State Level Best Practices

1. **Japan's Long-Term Care Insurance (LTCI):** A mandatory social insurance system providing comprehensive care to those over 65.
2. **Singapore's "Intergenerational Housing":** Colocating elderly nursing homes with childcare centers to foster social bonding.
3. **Kerala's "Sayamprabha Home":** State-run day-care centers providing recreation and health check-ups for the elderly.
4. **Netherlands' "Dementia Village" (Hogeweyk):** A gated community designed specifically for seniors with memory loss to live normally.
5. **Tamil Nadu's "Vayodhika Seva":** Mobile geriatric units that provide door-to-door healthcare in rural districts.
6. **Switzerland's "Time Bank":** A system where volunteers "save" hours by helping seniors, redeemable for their own future care.

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Fig- Silver Economy

Way Forward

1. **Universal Pension:** Transitioning from targeted to universal basic pension to ensure financial dignity for all.
2. **Comprehensive Care Packages:** Transition from episodic treatment to integrated care packages that include preventive, curative, and palliative services.
3. **Recognize caregiving** as a skilled profession with standardized training, minimum wages, and social security benefits.
4. **Silver Economy Incentives:** Tax breaks for companies developing affordable assistive technologies for the elderly.
5. **Geriatric Training:** Making "Geriatric Medicine" a mandatory specialization in all government medical colleges.
6. **Community-Based Care:** Promoting "Home-care" models over "Old Age Homes" to ensure emotional well-being.
7. **Digital Literacy Drives:** Specialized "Digital Saksharta" camps designed for seniors to learn safe online banking.
8. **CSR Mandates:** Encouraging corporates to spend CSR funds on building palliative care and dementia centers.
9. **Inheritance Law Reform:** Speeding up Senior Citizen Tribunals to settle maintenance disputes within 60 days.
10. **Intergenerational Programs:** Curriculum changes in schools to include "intergenerational bonding" activities with local seniors.
11. **Age-Friendly Urban Design:** Mandatory "Ageing-Audit" for public spaces, transport, and parks (Universal Design).
12. **Productive Ageing:** Expanding the **SACRED** portal to encourage "Work from Home" opportunities for retired professionals.

Maintenance and Welfare of Parents and Senior Citizens Act (MWPSC), 2007

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- Allows elderly (aged 60 and above) who cannot support themselves through their earnings or property to file a suit for maintenance against their children or relatives (legal heirs).
- Establishes dedicated **tribunals** to hear maintenance suits and **appellate tribunals**

Persons with Disabilities (PwDs)

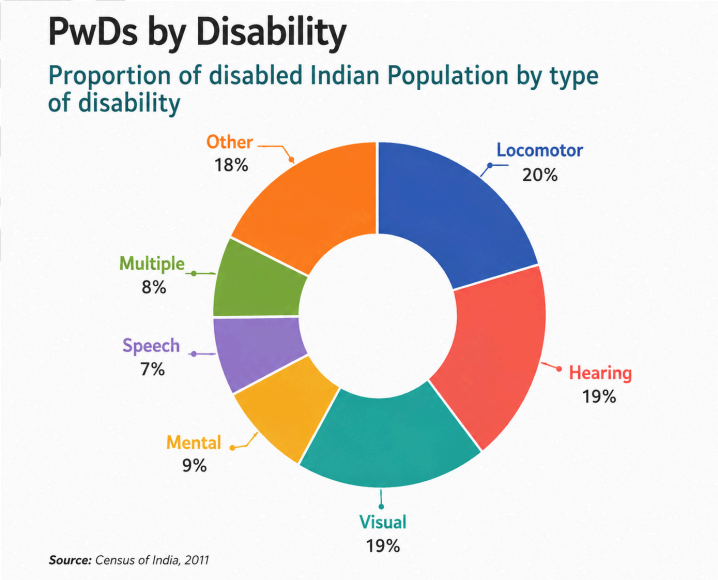
World Health Organization (WHO) defines disability as impairment, limitation, or restriction in activity caused mainly by health issues and environmental factors

According to the RPwD Act, 2016:

1. **PwD is a** person with long-term physical, mental, intellectual, or sensory impairments which, in interaction with barriers, hinders full and effective participation in society.
2. **Benchmark Disability:** A person with **not less than 40%** of a specified disability.
3. **Specified Disabilities:** The Act increased the categories from 7 to **21**, including Acid Attack victims, Parkinson's disease, and Blood disorders

Key Data Points

1. **Population Share:** ~2.21% of the total population (Census 2011), expected to be higher in the 2027 Census due to expanded categories.
2. **Rural-Urban Split:** ~69% of PwDs reside in rural areas (Census 2011).
3. PwDs constitute less than **1% of the workforce** in 800+ listed Indian companies.
4. **Literacy:** Only **~54%** of PwDs are literate, significantly lower than the national average.
5. Globally, **15% of the population** live with some form of disability. (**WHO**)



Major Issues Faced

1. **Inaccessible Built Infrastructure:** Less than 30% of public buildings meet **"Accessible India"** standards. (CAG)
2. **Educational Exclusion:** Only 61% of disabled children (5–19 years) attend educational institutions (UNICEF 2025).
3. **Low Workforce Participation:** Systemic bias and lack of "Reasonable Accommodation" prevent PwDs from securing or retaining private-sector jobs.
4. **Digital Divide:** Websites and apps often lack screen-reader compatibility or captions, excluding PwDs from the digital economy.
5. **Healthcare Inaccessibility:** Hospitals often lack sign-language interpreters and specialized geriatric/disability wards for therapeutic care.
6. **Social Stigma and Isolation:** Regressive belief that disability is a result of "bad karma" or misdeeds committed in a past life.
7. **Inadequate Legal Implementation:** Many states have not yet notified rules for District Disability Committees, delaying grievance redressal.

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8. **Double Marginalization of Women:** Disabled women face heightened risks of sexual abuse, domestic violence, and lack of reproductive healthcare.
9. **Vulnerability of Specific Groups:** This exclusion is particularly severe for those with intellectual disabilities or mental illness, who may be subjected to dehumanizing treatment
10. **Financial Exclusion:** High costs of assistive devices and lack of specialized insurance products lead to "Disability Poverty."
11. **Lack of Political Representation:** Absence of reservation in Parliament/Assemblies means issues of PwDs are rarely prioritized in policy debates.

Steps Taken

1. **RPwD Act, 2016:** Increased disability categories to 21 and mandated **4% reservation in government jobs** for persons with benchmark disabilities and **5% in higher education**
2. **Scheme for Implementation of Rights of Persons with Disabilities Act 2016 (SIPDA):** initiative providing financial and technical aid to implement the RPwD Act
3. **Mental Healthcare Act, 2017:** Rights-based approach to mental illness.
4. **Accessible India Campaign (Sugamya Bharat):** A nationwide flagship campaign for universal **accessibility in buildings, transport, and ICT.**
5. **UDID Project:** Creating a national database and issuing **Unique Disability ID** cards for seamless benefit access.
6. **Divyang Sahara Yojana (2026):** Targeted funding for AI-driven indigenous assistive technology development.
7. **Entrepreneurial Self-Reliance:** NDFDC provides concessional credit, disbursing ₹1,330 crore to foster 'Divyang' entrepreneurs, bypassing corporate hiring lags through financial inclusion.
8. **Inclusive Education:** Standardizing Indian Sign Language (ISL) through a 24/7 DTH channel and digital repositories ensures equal academic access for students.
9. **Mental Health Support:** Tele-MANAS 2.0 and the 'Asmi' chatbot provide accessible, screen-reader-compatible psycho-social support to combat isolation and digital exclusion.
10. **Deendayal Disabled Rehabilitation Scheme (DDRS):** Financial assistance to NGOs for providing education and vocational training.
11. **ADIP Scheme:** Assistance to Disabled Persons for purchase/fitting of sophisticated aids and appliances.
12. **Niramaya Insurance Scheme:** Provides affordable health insurance (up to ₹1 lakh) for persons with Autism, Cerebral Palsy, etc.
13. **Standardization of ISL:** Establishment of the **ISLRTC** to standardize and promote Indian Sign Language nationwide.
14. **Para-Athlete Ecosystem:** 'Khelo Bharat Niti 2025' provides parity in funding and training, supporting 52 para-athletes for the 2028 LA Olympics.

Major SC Judgments

1. **Vikash Kumar v. UPSC (2021):** Ruled that "Reasonable Accommodation" (like a scribe) is a fundamental right, not a charity.
2. **Rajive Raturi v. Union of India (2017):** SC directed the Centre to frame mandatory rules under the RPwD Act, 2016 to ensure accessibility in public places and services.
3. **Judicial Service Ruling (2024):** Quashed rules barring visually impaired candidates from judicial services.

Way Forward

1. **Universal Design:** Mandate "Accessibility Audits" for all new infrastructure at the design stage itself.

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2. **Inclusive Pedagogy:** Train all teachers in "Special Education" basics to make every classroom truly inclusive.
3. **Private Sector Incentives:** Provide tax breaks for companies achieving high "Diversity and Inclusion" scores.
4. **Digital Saturation:** Ensure all government apps (UPI, Cowin, etc.) are 100% WCAG-compliant.
5. **Community-Based Rehabilitation (CBR):** Utilize ASHA and Anganwadi workers for early disability detection in rural areas.
6. **Economic Empowerment:** interest subventions for PwD-led startups under the "Priority Sector Lending" category.
7. **Political Reservation:** Explore mandatory representation for PwDs in local bodies (Panchayats) and Legislatures.
8. **Mental Health Focus:** Integrate therapeutic and counseling services into the **Ayushman Bharat** wellness centers.
9. **Climate Justice:** Recognize PwDs as a specific vulnerable group in the **National Action Plan on Climate Change**.
10. **Social Sensitivity:** National "Divyang Awareness" campaigns to move society from "sympathy" to "empathy and respect."

Global Initiatives:

1. **UN CRPD (2006):** The core international treaty protecting the rights and dignity of PwDs.
2. **SDG 2030:** Goals 4, 8, 10, 11, and 17 explicitly mention disability-inclusive development.
3. **Marrakesh Treaty:** Facilitates access to published works for visually impaired persons.
4. **Incheon Strategy:** First set of disability-inclusive goals for the Asia-Pacific region.
5. **WHO "GATE" Initiative:** Global cooperation to increase access to high-quality assistive technology.

State-Level Best Practices:

1. **Goa's Purple Fest:** Model for large-scale social integration and disability branding.
2. **Kerala's "Barrier-Free Kerala":** First state to aim for 100% accessibility in all local government buildings.
3. **Odisha's Madhu Babu Pension:** Simplified, technology-enabled pension delivery for disabled citizens.
4. **Tamil Nadu's "Vayodhika Seva":** Specialized mobile units for door-to-door therapeutic care for the elderly disabled.

SC Guidelines to Prevent Stereotyping and Discrimination of PwDs

- **Avoid Derogatory Language:** Use respectful terms; avoid words like **'cripple,' 'spastic,' 'victim,' or 'afflicted'** that reinforce stigma and negative self-image.
- **Ensure Accurate Representation:** Visual media must portray disabilities **realistically and sensitively**. Follow the principle **"Nothing about us, without us"** by involving PwDs in content creation.
- **Limit to Creative Freedom: Artistic freedom** cannot justify **mocking or misrepresenting** marginalized groups. Content violating the dignity or rights of PwDs will not be constitutionally protected.
- **Collaborate with Advocacy Groups** for consultation and training of **writers, directors, and actors** to ensure inclusive and dignified portrayals.

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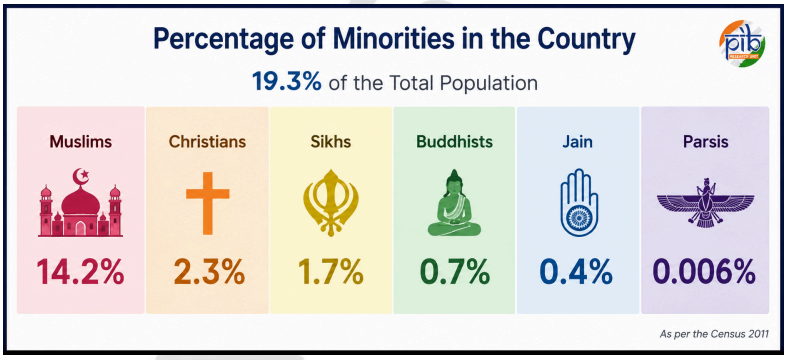
Minorities

The Constitution of India uses the term **"Minority"** in Articles 29–30 and 350A–350B but does not explicitly define it.

1. **National Level:** Six communities are notified as "Religious Minorities" under Section 2(c) of the National Commission for Minorities (NCM) Act, 1992
2. In the **T.M.A. Pai Case (2002)**, the SC ruled that minority status (linguistic or religious) must be determined in relation to the **population of the State**, not the entire country.
3. **Population Share:** approximately **19.3%** (Census 2011).



Key Constitutional and Legal Provisions

1. **Religious Freedom (Articles 25–28):** Every person has the freedom of conscience and the right to profess, practice, and propagate their religion. Groups can also manage their own religious affairs and maintain institutions.
2. **Article 29:** Protects the right of "any section of citizens" to conserve their distinct language, script, or culture.
3. **Article 30:** Grants all minorities the right to **establish and administer educational institutions** of their choice.
4. **Article 350B:** Provides for a **Special Officer for Linguistic Minorities** to investigate matters relating to safeguards.
5. **Article 15 & 16:** Prohibits discrimination on grounds of religion, race, caste, sex, or place of birth in public employment and social access.
6. **National Commission for Minorities Act, 1992:** A statutory body to monitor the working of safeguards provided in the Constitution.

Major Issues Faced by Minorities

1. **Educational Lag in Specific Pockets:** Disproportionate **school dropout** rates among certain minority groups due to economic distress and lack of localized schools. (Sachar Committee)
2. **Identity Crisis:** due to socio-cultural differences and historical contexts, which can lead to **social alienation.**
3. **Lack of Representation:** The 18th LS has only 4.4%% Muslim MPs (lowest in six decades)
4. **Poverty and Unemployment:** 31% of Muslims live below the poverty line compared to 22% of the overall population. (**Sachar Committee**)
5. **Vacancies** in the National Commission for Minorities hinder the quasi-judicial resolution of discrimination complaints.
6. **Communal polarisation during election:** Rising reports of hate speech affecting the psychological well-being and security of minority youth. Eg. Delhi Riots
7. **Ghettoization and Urban Exclusion:** Voluntary or forced residential segregation in urban areas limits access to high-quality public infrastructure.
8. **Linguistic Erasure: Eg.** Endangered status of languages like **Santhali** or **Ladakhi** scripts due to lack of primary-school teachers.
9. **Gender Dual-Marginalization:** Minority women face a "double burden" of gender discrimination and communal marginalization, affecting their workforce participation.

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10. **Job Market Stereotyping: Eg.** Diversity audits in 2025 revealed that minority representation in corporate boards remains below 5%.
11. **Digital Illiteracy in Artisan Hubs:** Minority-led traditional craft clusters (handloom, brassware) struggle to transition to global e-commerce due to lack of tech-training.

Major Steps Taken by Government

1. **PM VIKAS (Pradhan Mantri Virasat Ka Samvardhan):** A mega-scheme converging five erstwhile programs to provide skilling, education, and leadership training.
2. **PMJVK (Pradhan Mantri Jan Vikas Karyakram):** Multi-sectoral development program providing schools, hospitals, and toilets in Minority Concentrated Areas.
3. **National Overseas Scholarship:** Financial assistance for meritorious minority students to pursue Masters/PhD in top global universities.
4. **Begum Hazrat Mahal Scholarship:** Specifically targeted at minority girls to prevent dropouts at the secondary education level.
5. **Digitalization of Waqf Records (QUBSI):** t to digitize land records to prevent encroachment and increase revenue.
6. **Nai Roshni (now part of PM VIKAS):** Focused on leadership development and "Biz-Sakhi" entrepreneurship modules for minority women.
7. **Hunar Haat:** national exhibitions providing a "Lab-to-Market" platform for minority artisans.
8. **USTTAD (under PM VIKAS):** Preserving traditional arts/crafts and establishing linkages with e-commerce giants like Amazon/Flipkart.
9. **Hamari Dharohar:** A scheme to preserve the rich heritage of minority communities under the umbrella of Indian culture.

Way Forward

1. **Sachar Committee (2006)**
   a. **Equal Opportunity Commission** to handle discrimination in housing, jobs, and social access
   b. **Representation:** nomination of minorities in public sector boards and government recruitment committees.
2. **Justice Rangnath Misra Commission (2007)**
   a. **15% reservation** for minorities in government jobs and education.
   b. **Financial Support:** creating "Minority Finance Corporation" in every state.
3. **Amitabh Kundu Committee:** "Diversity Index" for institutions to incentivize inclusive hiring.
4. Ensuring **100% reach of central welfare schemes** in "Minority Concentrated Blocks" using the **Aspirational Districts** model.
5. **Modernizing Madrasas:** Integrating STEM (Science, Tech, Engineering, Math) into traditional curricula to ensure employability.
6. **Leveraging Waqf Assets:** Converting unutilized Waqf lands into **"Social Infrastructure Hubs"** (hospitals/technical colleges).
7. **Countering Polarization:** Strengthening **"Mohalla Committees"** and peace forums to build inter-community trust at the grassroots level.
8. **Women's Leadership:** Scaling the **"Biz-Sakhi"** model to create 1 million minority women entrepreneurs by 2030.
9. Conducting a periodic **"Socio-Economic Assessment"** to monitor real-time progress of minority groups on SDGs.

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Transgenders

1. Under the **Transgender Persons (Protection of Rights) Act, 2019**, a transgender person is one whose gender does not match the gender assigned at birth.
2. It includes trans-men, trans-women, persons with intersex variations, gender-queers, and socio-cultural identities such as kinnar and hijra.
3. The **2026 Amendment Act** provides a "Medical-Administrative Model," mandating certification by a medical authority and a District Magistrate (DM) for legal recognition.

- **Official Census (2011):** Recorded **4.9 lakh** persons under the "Other" category.
- **Experts estimate the** actual population to be between **2 million and 5 million**.
- **The literacy rate** of **50-55%** is nearly 20% lower than the national average.
- Over **90%** of the community remains dependent on the informal economy (begging, sex work, or **badhai**) due to workplace discrimination.
- Only **12%** of the community has successfully accessed gender-affirming surgeries through formal government channels.
- Nearly 50% of transgenders never attend school, and only 6% are employed in formal sectors. **(NHRC)**

Reason For Discrimination Against Transgenders

1. **Binary Gender Norms (Gender Essentialism):** Society enforces rigid male/female roles, marginalizing those outside this binary.
2. **Colonial Legal Legacy:** Laws like **Section 377** and Criminal Tribes Act historically labeled trans people as **deviant**.
3. **Cultural Stigma and Stereotyping**: Trans identities are often associated with superstition, entertainment, or deviance, leading to ridicule and isolation.
4. **Lack of Legal and Institutional Inclusion (Recognition Deficit):** Delayed recognition in law and poor implementation of the **2019 Trans Act** perpetuate invisibility.
5. **Economic Marginalization**: Trans persons are excluded from formal employment, forcing many into begging or sex work.
6. **Family Rejection and Social Abandonment**: Non-conforming gender identity often leads to early expulsion from families, especially during adolescence.
7. **Religious and Moral Prejudice**: Conservative interpretations of religion and morality label trans identity as unnatural or immoral.

Major Issues Faced by Transgenders

1. **Identity Verification Barriers:** The shift from "self-identification" to "mandatory medical filtering" creates bureaucratic hurdles and potential harassment.
2. **Pervasive bullying and rigid gender binaries in schools** lead to high dropout rates. **Eg.** ASER 2025 reports highlight that 60% of trans-youth drop out before Class 10.
3. **Due to societal stigma** and **mental distress 31% of transgender persons die by suicide**
4. **Economic Destitution:** Systemic stigma in formal workplaces forces the majority in informal sectors. **Eg.** over 31,000 individuals engaged in begging identified under SMILE scheme
5. **Healthcare Deficits:** Lack of sensitized medical staff and specialized gender-affirming infrastructure. (**HIV prevalence was 3.8% in 2021 (20 times national average)**.)
6. Economic exclusion leads to a poverty cycle and **makes them susceptible to bonded labor and human trafficking.**

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7. **Residential Segregation (Ghettoization): Eg.** HelpAge India (2025) notes "Housing Discrimination" is the top barrier for trans-seniors.
8. **Civil Law Invisibility: Eg.** The 2026 Act remains silent on the legal status of "trans-kinship" or families.
9. **Violence and Criminalization:** High vulnerability to physical and sexual violence, often with low reporting due to police bias.
10. **Mental Health Crisis:** Social exclusion and "minority stress" lead to high rates of anxiety and suicidality. **Eg.** TISS Research (2025) links 50% of trans-youth suicides to familial rejection.
11. **Lacking Political Voice: Eg.** There is currently zero transgender representation in the 18th Lok Sabha (2026).

Major Steps Taken for Upliftment

1. **SMILE Scheme:** Provides comprehensive rehabilitation, including healthcare, skill development, and shelter.
2. **Garima Greh:** Dedicated shelter homes providing food, medical care, and vocational training.
3. **National Portal for Transgender Persons:** Online application for Identity Certificates to minimize physical interface.
4. **Ayushman Bharat TG Plus:** Dedicated health insurance covering Hormone Therapy and SRS (Sex Reassignment Surgery).
5. **Transgender Protection Cells:** MHA advisory (2025) to set up dedicated police cells in each district.
6. **National Council for Transgender Persons (NCT):** A statutory body to advise the government on policy and rights.
7. **Horizontal Reservation (States):** States like Karnataka and Tamil Nadu have implemented 1% reservation in jobs.
8. **Skill India Integration:** Specialized PMKVY training batches for transgender artisans and entrepreneurs.
9. **Inclusive NEP 2020:** Recognition of trans-children as "Socio-Economically Disadvantaged Groups" (SEDGs).
10. **Ministry of Social Justice:** The "Equal Opportunities Policy" mandates workplaces to eradicate discrimination, promote dignity, and provide equal professional opportunities.

Transgender Amendment Act, 2026

- **Removal of Self-Identification:** It deletes the provision that allowed individuals to "self-perceive" their gender identity.
- **Mandatory Medical Certification** by the **District Magistrate (DM)** only after a recommendation from a **Medical Board** (headed by a CMO/Deputy CMO).
- **Narrowed Definition:** It replaces the 2019 broad definition with a "closed list" including socio-cultural identities (Hijra, Kinner, Aravani, Jogta, Eunuch) and persons with specific **biological congenital variations**. (removes categories of **trans-man, trans-woman, and genderqueer**)
- **Criminalizing Forced Identity:**
  - **Kidnapping adults** to force a transgender identity: 10 years to Life Imprisonment + ₹2 lakh fine.
  - **Kidnapping children** for the same: Life Imprisonment + ₹5 lakh fine.
  - **Forced Begging/Servitude** while presenting as transgender: 5–10 years rigorous imprisonment.

Major SC Judgments

1. **NALSA judgment:** recognized the **"Third Gender"** & upheld the right to **self-attested gender identity**. It directed the government to provide reservations in education and employment.

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2. **Navtej Singh Johar:** Decriminalized consensual same-sex relations by striking down parts of **Section 377**, affirming the right to intimacy and dignity for the LGBTQ+ community.
3. In **Jane Kaushik case,** SC held that **failure to implement transgender protections amounts to "omissive discrimination"& violates fundamental rights**.

Way Forward

1. Replace the broad **"transgender" category** with **GIESC (Gender Identity, Expression, Sex Characteristics)** classification.
2. Enact a dedicated **Intersex Rights law** to ban **non-consensual "normalizing" surgeries** on intersex infants.
3. **Implementing Horizontal Reservation:** Uniform national policy for 1% reservation in all public employment and education.
4. **Sensitized Policing:** Mandatory "Trans-inclusion" modules in police training academies.
5. **Family Law Reforms:** Amending the Special Marriage Act and Adoption laws to include transgender persons.
6. **Universal Health Access:** Establishing "Gender Clinics" in every Government Medical College.
7. **Campus Inclusivity:** Mandating "Gender-Neutral" washrooms and hostels in all UGC-recognized universities.
8. **Corporate Diversity Tax Breaks:** Incentivizing companies that achieve a 2% "Trans-inclusion" hiring rate.
9. **Fast-Track Courts:** Specialized judicial benches to handle "Atrocity cases" against transgender persons.
10. **Digital Locker Integration:** Linking UDID and Transgender ID cards for "one-click" verification.

Global & State Level Best Practices

1. **Argentina (Gender Identity Law):** Gold standard for "Self-Identification" without any medical or psychological filters.
2. **Kerala (Transgender Policy):** First state to launch a dedicated policy; includes a 24/7 helpline and education grants.
3. **Tamil Nadu (Transgender Welfare Board):** Pioneer in providing free SRS in government hospitals and housing schemes.
4. **Karnataka (Reservation):** First state to implement 1% horizontal reservation in civil services.
5. **Kineer Services Pvt. Ltd.** - founded by transgender activist **Laxmi Narayan Tripathi**

"Gender identity is integral to the dignity of an individual and is at the core of personal autonomy." - **Justice K.S. Radhakrishnan**

"No one should be left behind." - UN. India's journey toward transgender empowerment must align with the **SDG 5 (Gender Equality)**

LGBTQI+

LGBTQI+ is an umbrella term representing a diverse range of sexual orientations, gender identities, and sex characteristics.

1. **L, G, B, Q:** Refer to sexual orientation (Lesbian, Gay, Bisexual, Queer).
2. **T, I:** Refer to gender identity and biological variations (Transgender, Intersex).

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3. **+:** Represents other identities like Asexual, Pansexual, and non-binary individuals.

Constitutional & Legal Rights (Same as for transgenders)

Key Issues Faced by the LGBQI+ Community

1. **Legal Gaps:** Despite decriminalization, the absence of marriage equality denies couples essential rights like joint insurance, inheritance, and adoption.
2. **Social Stigma:** Deep-rooted societal bias views non-heterosexual identities as "abnormal" or "deviant," forcing individuals into exhausting, secretive lives.
3. **Family Rejection**: high levels of rejection and discrimination within families lead to **homelessness,** and a lack of support systems.
4. **Intersex Rights:** Infants often undergo non-consensual "normalizing" surgeries, causing lifelong physical pain and violating their fundamental bodily autonomy.
5. **Healthcare Bias:** Heteronormative medical bias and lack of provider training lead to misdiagnosis or avoidance of essential healthcare services.
6. **Workplace Barriers:** Informal discrimination and "glass closets" persist, where micro-aggressions prevent individuals from bringing their authentic selves to work.
7. **Mental Health:** Minority stress drives disproportionately higher rates of anxiety, depression, and suicide compared to the general population.
8. **Cultural Isolation:** Hostile religious or cultural environments force a painful choice between personal identity and communal or spiritual belonging.

Major Steps Taken

1. **Transgender Persons Act, 2019:** Provided the first legal framework for non-discrimination.
2. **National Council for Transgender Persons (NCTP):** A statutory body to advise the government on all policy matters.
3. **SMILE Scheme:** Provides comprehensive rehabilitation, including "Garima Greh" shelter homes.
4. **Ayushman Bharat TG Plus:** Offers ₹5 lakh health insurance covering SRS and hormone therapy.

Major Supreme Court Judgements

1. **NALSA v. Union of India (2014):** Recognized transgender persons as the **"Third Gender,"**.
2. **Puttaswamy v. Union of India (2017):** Declared **privacy a fundamental right,** explicitly protecting sexual orientation as an essential, private attribute of human dignity.
3. **Navtej Singh Johar v. UOI (2018): Decriminalized** consensual same-sex acts by striking down Section 377, prioritizing "constitutional morality" over traditional social prejudices.
4. **Supriyo v. Union of India (2023):** SC held that **right to marriage is not FR** and denied recognition to same sex marriage. It transferred the ball in the legislature's court to make relevant laws for the community.

Way Forward

1. **Anti-Discrimination Law to** legally prohibit discrimination in housing, education, and private employment.
2. **Inclusive Education:** Integrating LGBTQI+ history and diversity into school curriculums fosters empathy and prevents bullying by normalizing non-binary identities early.
3. **Legalize Civil Unions:** Provide legal status to same-sex relationships for insurance, inheritance, and medical consent.
4. **Horizontal Reservation:** Implement a 1% "trans-quota" across all central and state departments.

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5. Setting up **localized "Queer Resource Centers"** provides immediate legal aid, mental health counseling, and emergency shelter for those rejected.
6. **Rainbow Economy:** Providing tax incentives for "Queer-Sourcing" from LGBTQI+-owned MSMEs.
7. **Digital Safety:** Strict enforcement of data privacy laws is essential to prevent "outing" and online harassment.

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Chapter 2

Issues Relating to Development and Management of Social Sector Services

Health, Education, Human Resources.

PYQs

[2021] "Besides being a moral imperative of the Welfare State, primary health structure is a necessary pre-condition for sustainable development." Analyze. (10)

[2020] In order to enhance the prospects of social development, sound and adequate health care policies are needed particularly in the fields of geriatric and maternal health care. Discuss. (10)

[2018] Appropriate local community level healthcare intervention is a prerequisite to achieve 'Health for All' in India. Explain. (10)

[2020] - National Education Policy 2020 is in conformity with the Sustainable Development Goal-4 (2030). It intends to restructure and reorient the education system in India. Critically examine the statement. (15)

[2021] - "'Earn while you learn' scheme needs to be strengthened to make vocational education and skill training meaningful." Comment. (10)

[2022] - The Right of Children to Free and Compulsory Education Act, 2009 remains inadequate in promoting incentive-based systems for children's education without generating awareness about the importance of schooling. Analyse (15)

[2023] - Skill development programs have succeeded in increasing human resources supply to various sectors. In the context of the statement, analyze the linkages between education, skill and employment. (15)

[2024] - In a crucial domain like the public healthcare system the Indian State should play a vital role to contain the adverse impact of marketisation of the system. Suggest some measures through which the State can enhance the reach of public healthcare at the grassroots level. (15)

[2026] - Should education be treated primarily as a welfare obligation of the state or as a strategic investment for building a globally competitive, knowledge-driven nation? Critically evaluate. (15)

"Health is a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity." **(WHO)**

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Dimensions of health

1. **Physical:** The functional operation of the body, including fitness, nutrition, and physiological homeostasis.
2. **Mental/Emotional:** The ability to process feelings, manage stress, and maintain a positive outlook.
3. **Social:** The ability to form meaningful relationships, interact with others, and adapt to social situations.
4. **Intellectual:** Maintaining curiosity, lifelong learning, and the ability to solve problems effectively.
5. **Spiritual:** Finding purpose, value, and meaning in life, with or without organized religion.

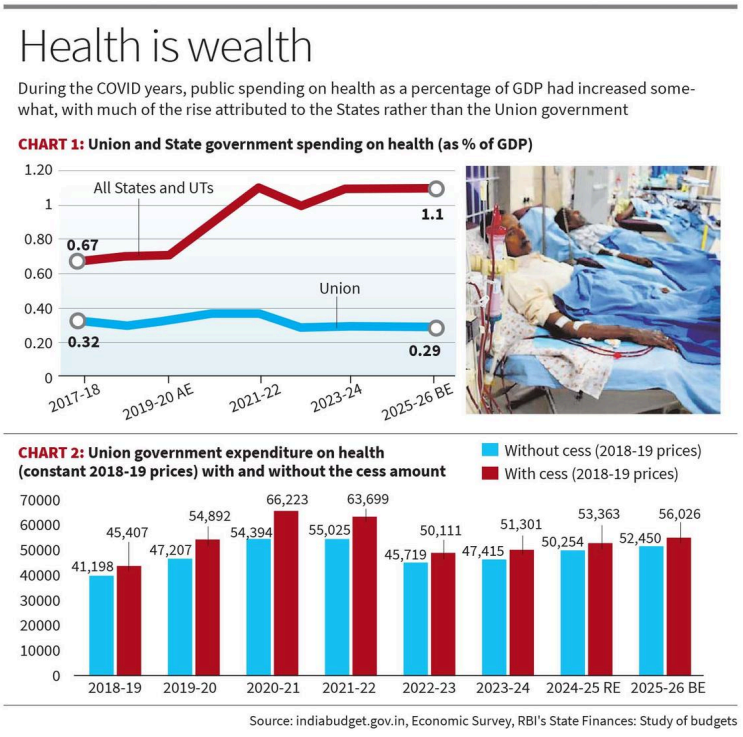
Data

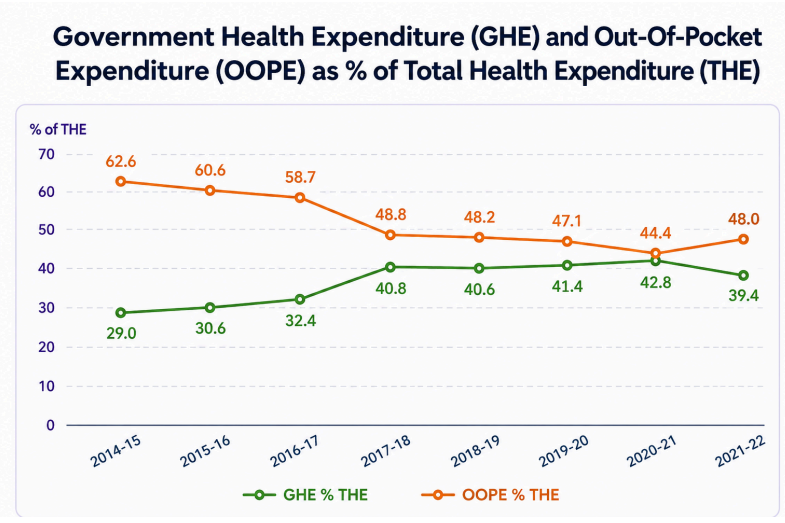
Mortality and Life Expectancy

1. **Infant Mortality Rate (IMR):** Reduced to **25 per 1,000 live births**.
2. **Under-5 Mortality Rate (U5MR):** Declined to **28 per 1,000 live births**.
3. **Neonatal Mortality Rate (NMR):** Dropped to **17 per 1,000 live births** nationally.
4. **Maternal Mortality Ratio (MMR):** Significant decline to **80 per lakh live births**.
5. **Life Expectancy:** Estimated at **~71.5 years** for the total population in 2026.
6. **Total Fertility Rate (TFR):** Consistent at **2.0**, below the replacement level of 2.1.
7. **Institutional Births:** Increased to **~89%** nationally
8. **Full Immunization Coverage (FIC)**reached **93.5%** under Mission Indradhanush 5.0.

Health Expenditure

1. **Public Health Expenditure: 1.9% of GDP.**
   a. Centre's share: **0.29% of GDP** vs aim of **1% of GDP** (**40% of total public spending**).
   b. **States** share **increased** from **0.67%** (2017-18) to **1.1% of GDP** (2025-26)
2. Combined **Health Expenditure (Public + Private) - 3.8% of GDP.**
3. **Out-of-Pocket Expenditure (OOPE)** declined from 55% (2017-18) to **39.4% (2021-22).**
4. **"Missing Middle": 30% of the population** remains without any health insurance.(NITI Report)





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Public Health Infrastructure and Schemes

1. Over **1,85,000 Ayushman Arogya Mandirs** (formerly HWCs) are operational
2. Over **43.52 crore Ayushman cards created**
3. **93 Lakhs Ayushman Vay Vandana cards** issued to elderly (70+), providing ₹5 lakh cover.
4. Over **11.69 crore hospital admissions** authorized under PMJAY, worth ₹1.73 lakh crore.
5. **86.64 crore Ayushman Bharat Health Accounts (ABHA)** created to digitize health records.

Disease Burden

1. **NCD Burden:** Non-Communicable Diseases account for **~66% of total deaths** in India. Cardiovascular diseases (28%) and chronic respiratory diseases (12%) are the leading causes.
2. The share of **communicable, maternal, and neonatal diseases** in the overall disease burden fell to **33%** (down from 61% in 1990).
3. TB incidence declined by **21%** (from 237/lakh in 2015 to **187/lakh in 2024**). Mortality reduced to **21 per lakh**.
4. A 2023 **Lancet study** estimates over **100 million Indians live with diabetes**, while **130 million more are pre-diabetic.**
5. As per the **(NFHS)-5 (2019-21),** overall, **24% of Indian women and 23% of Indian men** are overweight or obese
6. India has the **2nd-highest** number of children with high BMI globally, totaling **41 million**.

Structure of Public Healthcare System in India

Level	Institution	Functions
Primary Level	**Sub-Centre (SC)** and **Primary Health Centre (PHC),** Ayushman Arogya Mandirs (AAMs)	First contact point; maternal and child health, immunization, family welfare.
Secondary Level	**Community Health Centre (CHC)** and **Sub-District Hospital**	Referral services, specialist care, minor surgeries.
Tertiary Level	**District Hospitals, Medical Colleges, AIIMS, Specialized Institutes**	Advanced diagnostic, treatment, and research services.

Issues with the Public Healthcare System

Policy issues

1. **Weak Preventive and Primary Care (Inverse Care Law):** Over-emphasis on tertiary care; only ~40% of public funds go to primary care despite the NHP 2017 target of two-thirds.
2. **Inefficient Governance**: Health is a **State subject.** Lack of coordination between Centre and States affects service delivery.
3. **Lack of Research & Development (R&D):** India is the "Pharmacy of the World" for generics but lacks innovation in "New Chemical Entities" (NCEs).
4. **Nutritional Morbidity (Hidden Hunger):** Caloric security (NFSA) has not translated into nutritional security. **Eg. 57% of women** remain anaemic.

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5. **Reactive Surveillance:** Surveillance remains **event-based rather than predictive.** Current policy relies on fragmented, vertical silos where data for communicable diseases, non-communicable diseases (NCDs), and animal health are collected separately.

Economic issues

1. **Stagnant Public Expenditure:** Government spending remains around **1.9% of GDP**, missing the National Health Policy 2017 target of 2.5% by 2025.
2. **Inefficient cess utilization**: Over **₹43000 crore gap** in the transfer of health cess to the **Pradhan Mantri Swasthya Suraksha Nidhi (PMSSN). (CAG report)**
3. **The "Missing Middle" Crisis:** Approximately **30% of the population** lacks health insurance, being too rich for Ayushman Bharat but too poor for private cover.
4. **High Out-of-Pocket Expenditure (OOPE):** Despite declines, OOPE still accounts for **39.4% of total health spending**, pushing millions into poverty annually.
5. **Profit-Driven Medical Education:** High costs of private medical education lead to reluctance to serve in public/rural sectors.

Institutional issues

1. **Rural-Urban Specialist Gap:** Rural Community Health Centers (CHCs) face a **~80% shortage of specialists**.
2. **73% of public hospital beds are in urban areas**, despite 70% of the population living in rural areas.
3. **Hollowed-Out Primary Healthcare: Eg.** Only 60% of AAMs reported a "dependable supply" of essential NCD drugs.
4. **Mental Health Infrastructure Deficit: Eg.** India has only **0.75 psychiatrists per 100,000 people**, far below the recommended 3.0.
5. **Lack of human resources**: **Eg.** India requires an additional **6.5 lakh nurses** by 2030 to meet basic WHO standards.

Social and environmental issues

1. **The "Silver Burden" (Geriatric Care):** Elderly population is projected to reach 20% by 2050. Those aged 60+ face an ailment frequency of **43.9%**, four times the national average.
2. **Triple Burden of Disease:** India simultaneously battles infectious diseases, rising Non-Communicable Diseases (NCDs), and emerging zoonotic threats.
3. **Antimicrobial Resistance (AMR):** Over-prescription of antibiotics has made India a "global hotspot" for drug-resistant "superbugs."
4. **Climate-Health Sensitivity:** Rising temperatures are expanding the geographical range of vector-borne diseases like Dengue and Malaria.

Steps taken

Hospital Infrastructure

- Over **1.85 lakh Ayushman Arogya Mandirs (AAMs)** are operational, providing 12 essential service packages including NCD screening.
- **PM-Ayushman Bharat Health Infrastructure Mission (PM-ABHIM):** to build critical care hospital blocks in all districts with a population over 5 lakhs and establish **3,382 Block Public Health Units**.
- **District Integrated Labs** to ensure access to complex diagnostics to rural citizens
- Increased the number of AIIMS to 23, ensuring top-tier medical facilities are available across different states, including the Northeast.

Financial Protection & Insurance

- **Ayushman Bharat - PM Jan Arogya Yojana (PM-JAY, 2018)**

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- Covers **12.37 crore families** - benefiting **~55 crore individuals** (bottom 40%)
- **₹5 lakh annual health cover** per family
- Over **43.52 crore Ayushman cards** have been created
- **All senior citizens aged 70 and above - ₹5 lakh health cover** under AB-PMJAY.
- **ASHA workers, Anganwadi Workers (AWWs), Anganwadi Helpers (AWHs)** included under PM-JAY

Affordable Medicines

- **Pradhan Mantri Bhartiya Janaushadhi Pariyojana (PMBJP)** - Network of **Jan Aushadhi Kendras** across India (over 18000). **Generic medicines at 50-90% lower prices**
- **AMRIT Pharmacies (Affordable Medicines and Reliable Implants)** - For **tertiary healthcare** drugs, **surgical items, & medical implants**

Digital Transformation (Ayushman Bharat Digital Mission)

- Built on **CoWIN, Aarogya Setu** success
- **ABHA Health Accounts: Over 86 crore ABHA accounts** have been generated, creating a longitudinal digital health record for citizens that is interoperable between hospitals.
- **e-Sanjeevani:** Clocked over **20 crore consultations**, enabling rural patients to consult urban specialists via video.
- **SAHI (Strategy for AI in Health)** framework launched in 2026 is standardizing the use of AI for early TB detection and retinal screening for diabetic patients in government clinics.

CHILD AND MATERNAL HEALTH

- **Universal Immunisation Programme (UIP)**
- Targets **2.6 crore newborns + 2.9 crore pregnant women** annually
- **Full Immunisation Coverage (FIC): 94.1%**
- Vaccines: BCG, OPV, Pentavalent, MR
- **U-WIN Platform - 10.48 crore beneficiaries registered. 41.73 crore vaccine doses recorded**
- **Maternal Health Schemes**
- **JSSK: 16.60+ crore beneficiaries** since 2014-15
- **JSY: 11.07+ crore women** benefitted (March 2025)
- **SUMAN: 90,015 health facilities** notified (March 2025)
- **Medical Termination of Pregnancy (Amendment) Act, 2021:** Expanded access to safe and legal abortion services.

MEDICAL EDUCATION TRANSFORMATION

- The **National Medical Commission (NMC)** has removed the population-based cap on new colleges. India is targeting an addition of **75,000 medical seats** over five years to hit a 1:1 ratio for UG and PG seats.
- **Caregiver Training Program** under Budget 2026 to train **1.5 lakh certified caregivers** to support the burgeoning geriatric (senior) population and home-care needs
- **Infrastructure (May 2025):**
- **23 AIIMS** (including first in Northeast - Assam)
- **2,045 medical colleges** (780 Allopathy, 323 Dental, 942 AYUSH)
- **MBBS seats:** 51,348 → **1,18,190** (130% increase)
- **PG seats:** 31,185 → **74,306** (138% increase)
- **Nursing Reforms:**
- **157 co-located nursing colleges** approved
- **B.Sc. Nursing seats: 1,27,290** (53% increase)

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- **M.Sc. Nursing seats: 14,986** (39% increase)
- **NRTS (2019): 12.39 lakh nurses** in digital, Aadhaar-linked database
- **Legal Reforms:**
- **National Medical Commission Act, 2019** - replaced MCI
- **National Commission for Allied & Healthcare Professions Act, 2021**
- **National Dental Commission Act, 2023**
- **National Nursing and Midwifery Commission Act, 2023**
- **NEET (2016)** - uniform, transparent admissions

NATIONAL HEALTH MISSION (NHM)

- **NHM Framework**
- **NRHM (Rural) + NUHM (Urban)** sub-missions
- Aligned with **National Health Policy 2017**
- Targets **SDG 3** - health and well-being for all
- Supports **Universal Health Coverage (UHC)**
- **Ayushman Arogya Mandirs (AAMs) -** Formerly **Health & Wellness Centres** - major pillar of UHC at primary level.
- **Major NHM Programmes**
- **Reproductive, Maternal, Neonatal, Child, and Adolescent Health (RMNCH+A)** - Janani Shishu Suraksha Karyakaram (JSSK), Rashtriya Bal Swasthya Karyakram (RBSK), the Universal Immunisation Programme, Pradhan Mantri Surakshit Matritva Abhiyan (PMSMA), and the LaQshya programme for improving quality of care in labour rooms.
- **To tackle nutritional deficiencies -** National Iodine Deficiency Disorders Control Programme, MAA (Mothers' Absolute Affection) programme for infant and young child feeding, and the National Iron Plus Initiative for anaemia control.
- To address **communicable diseases** - Integrated Disease Surveillance Programme (IDSP), National Leprosy Eradication Programme (NLEP), National Vector Borne Disease Control Programme (NVBDCP), National AIDS Control Programme (NACP), Pulse Polio Programme, National Viral Hepatitis Control Program (NVHCP), and the National Programme on Containment of Anti-Microbial Resistance (AMR).
- To address **Non-communicable diseases** - National Tobacco Control Programme (NTCP), National Programme for Prevention and Control of Cancer, Diabetes, Cardiovascular Diseases & Stroke (NPCDCS), National Mental Health Programme, National Programme for Health Care for the Elderly (NPHCE).
- **National TB Elimination Programme (NTEP):** TB incidence dropped 17.7% and deaths 21.4% between 2015 and 2023. Treatment coverage rose to 85%.
- **National Sickle Cell Anaemia Elimination Mission:** Targeting eradication by 2047, 5.72 crore people were screened by June 2025.
- **Pradhan Mantri National Dialysis Programme (PMNDP):** 27.86 lakh patients accessed 342.25 lakh dialysis sessions across 749 districts.
- **National Free Drugs & Diagnostic Services:** Ensures the availability of essential drugs and diagnostic facilities to reduce out-of-pocket expenditure.
- **Comprehensive Cancer Care:** Mass screenings reached over 58 crore people for oral, breast, and cervical cancers.

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National Health Policy, 2017

- **Vision:** To attain the highest possible level of health and well-being for all ages through preventive and promotive health.
- **Key Principles:**
  - **Equity:** Minimizing the gap between the rich and poor, and urban and rural.
  - **Universality:** Moving toward Universal Health Coverage (UHC).
  - **Patient-Centered:** Ensuring dignity, confidentiality, and quality in care.
  - **Professionalism:** High standards of ethics and integrity.

Quantitative Targets

- **Health Status and Impact**
- **Life Expectancy:** Increase to **70 years** by 2025 (Current status: ~71.5 years).
- **Total Fertility Rate (TFR):** Reduce to **2.1** (Current: 2.0).
- **Mortality:**
  - **MMR:** Reduce to **100** by 2020 and **70** by 2030 (Current: ~93).
  - **IMR:** Reduce to **28** by 2019 (Current: ~25).
  - **Under-5 Mortality:** Reduce to **23** per 1,000 by 2025.
- **NCD Premature Mortality:** Reduce mortality from cancer, diabetes, and heart disease by **25%** by 2025.

Health System Performance

- **Public Facility Utilization:** Increase usage of public health facilities by **50%** from 2017 levels.
- **Immunization:** Achieve **90% full immunization** coverage of newborns by 1 year of age.
- **Screening:** Achieve "80% control status" for known hypertensive and diabetic individuals at the household level.

Strategic Features and Pillars

- **Shift to "Wellness":** Moving beyond curative care to establish **1.5 lakh Health and Wellness Centres (now Ayushman Arogya Mandirs)**.
- **Financial Protection:**
  - Goal to reduce **Out-of-Pocket Expenditure (OOPE)** by providing free essential drugs and diagnostics in all public facilities.
  - Increasing government health expenditure to **2.5% of GDP**.
- **Digital Health Ecosystem:** Establishment of the **National Digital Health Mission (NDHM)** to create a digital health ID (ABHA) for every citizen.
- **Integrated Medicine (AYUSH):** Promoting the "cross-referral" model where patients can access Yoga, Ayurveda, or Unani alongside modern medicine in the same facility.
- **Regulatory Reform:**
  - Setting up the **National Medical Commission (NMC)** to replace the MCI and reform medical education.
  - Capping prices of essential drugs and medical devices through the **NPPA**.
- **Human Resource for Health:**
  - Developing a cadre of **Mid-Level Care Providers** (B.Sc. Community Health) to manage primary wellness centers.
  - Scaling up nursing and paramedic schools in underserved districts.

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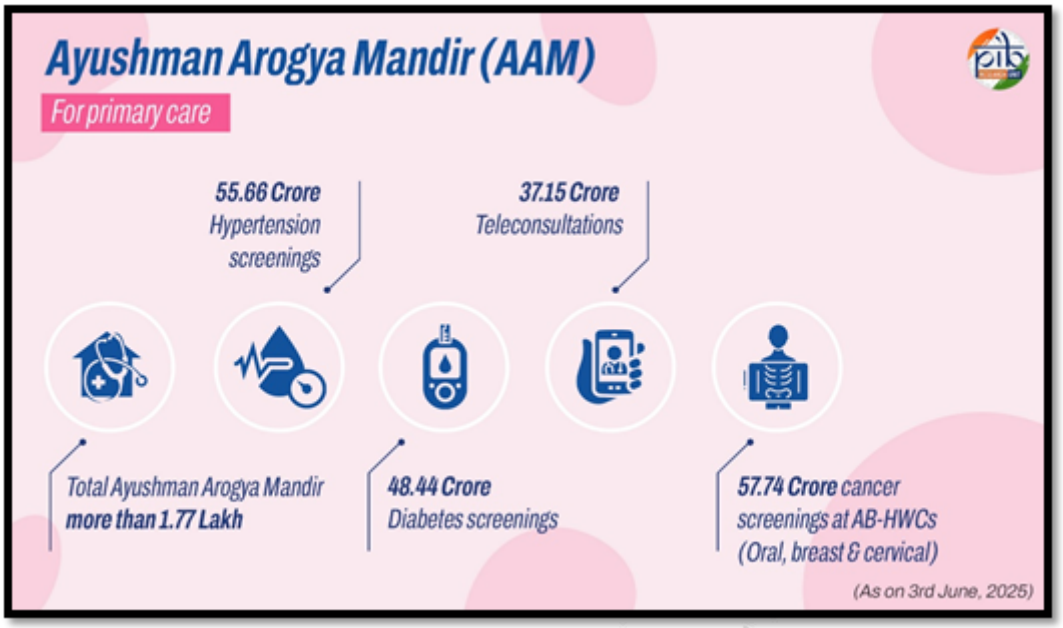

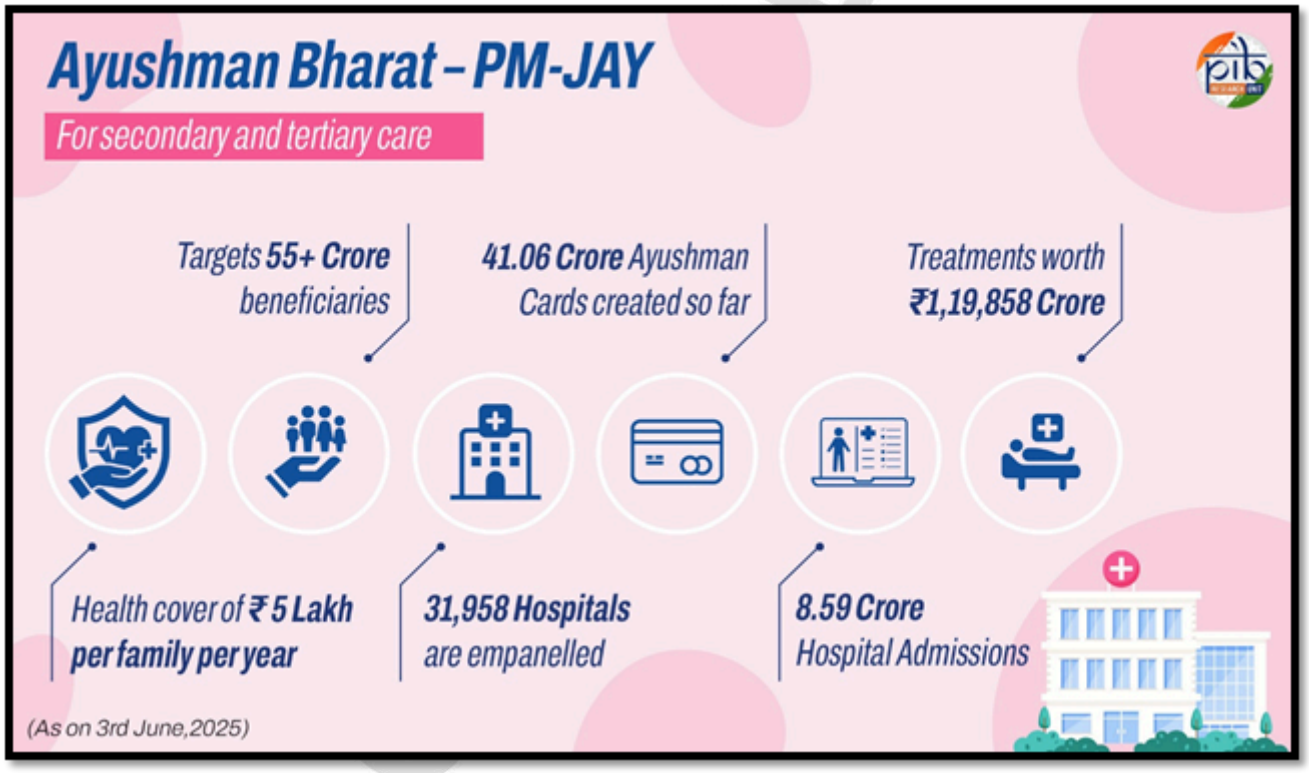

Measures to Address Issues Facing the Public Healthcare System

Accessibility

1. **Strengthen Primary Healthcare:** Achieve 100% saturation of AAMs to provide primary care within 30 minutes of every household.
2. **Ensure Equitable Access:** Deploy **mobile health units** and **e-Sanjeevani telemedicine** in underserved tribal and rural areas.
3. **Hybrid Telemedicine Models:** Integrating **e-Sanjeevani** with physical OPDs to create a "hub-and-spoke" model for specialist rural access.
4. **Unified Health Interface (UHI):** Scaling the National Digital Health Mission to enable seamless interoperability between private and public providers.

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5. **Mental Health Integration:** Expand **District Mental Health Programme (DMHP)**; increase budget and trained mental health professionals.

Affordability

1. **Increase Public Health Expenditure:** Raise spending to at least **2.5% of GDP by 2025**, with 2/3rd allocations for primary and preventive care. (**National Health Policy 2017**)
2. **Hybrid Public-Private Health Fund**: Pool **CSR, health cess, and private capital** to bridge funding gaps and expand infrastructure in **Tier-2/3 cities**.
3. **Value-Based Healthcare**: Shift from **fee-for-service** to **outcome-based payments**. E.g- linking **PM-JAY reimbursements** to patient recovery.
4. **Capping Private Treatment Costs:** Implementing a "National Commission for Healthcare Cost Management" to regulate and cap high-cost private procedures.
5. Expanding **Jan Aushadhi Kendras** to every panchayat to provide medicines at low discounts.
6. **Tax Reforms on Medical Devices:** Reducing or exempting GST on essential medical equipment and diagnostics to lower treatment costs.

Quality

1. **Health-in-All-Policies (HiAP)**: Integrate health considerations across sectors like **urban planning, agriculture, and environment** to address **social determinants** (air, nutrition, sanitation).
2. **Public Health Management Cadre (PHMC)**: Create a **specialized administrative cadre** to separate **clinical care from health governance**, improving efficiency.
3. **Infrastructure Modernization:** Upgrade PHCs/CHCs with **essential diagnostics, electricity, water, digital health tools**.
4. **Promote Preventive Healthcare:** Scale awareness on **NCD screenings, immunizations**, nutrition, and sanitation; focus on behaviour change. Eg. **Eat Right India**
5. **Build Local Resilience:** Develop **block-level hospitals**, decentralize health planning, and train local health workers. Eg. Tamil Nadu's **Makkalai Thedi Maruthuvam**
6. Enforce **prescription: Only antibiotic use** and regulate pharmaceutical waste to combat **antimicrobial resistance (AMR)**.

Infrastructure and technological measures

1. **Decentralized MedTech Clusters**: Develop regional hubs for **indigenous manufacturing** of high-end medical devices (MRI, ventilators, robotics).
2. **Precision Medicine**: Integrate **genomic sequencing** (leveraging Genome India) for **personalized diagnosis and prevention**.
3. **Digital & AI Integration**: Use **data analytics and AI** for better diagnostics, surveillance, and planning.
4. **One Health Framework**: Integrate **human, animal, and environmental health systems** for early detection of pandemics and zoonotic diseases. Strengthen **AI-driven bio-surveillance**

Best practices

1. **Kerala:** Decentralized Public Health System with strong PHCs, and proactive pandemic response.
2. **Tamil Nadu - Makkalai Thedi Maruthuvam:** Doorstep delivery of healthcare for elderly and NCD patients.
3. **Rajasthan's "Right to Health" Act: The first state** to provide a legal guarantee for emergency treatment without prepayment in both public and private facilities.

A robust public healthcare system is key to achieving **SDG 3 (Good Health & Well-being)** and **SDG 10 (Reducing Inequality)** by ensuring access to affordable, quality care for all.

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AYUSH

1. **AYUSH** is an acronym for **Ayurveda, Yoga and Naturopathy, Unani, Siddha, Sowa-Rigpa, and Homoeopathy**.

Significance of AYUSH

1. **Public Health Access:** Integration into **Ayushman Arogya Mandirs** ensures primary care reaches the last mile.
2. **Combating Lifestyle Diseases:** Effectively manages NCDs like diabetes and hypertension through holistic lifestyle modifications.
3. **Rural Livelihoods** through cultivation of medicinal plants. **Eg.** Farmers earn 3x more from Ashwagandha than traditional cereal crops (Agri-Ministry 2026).
4. **Export Potential: Eg.** India exports **$1.54 billion** worth of AYUSH products to over 150 countries annually.
5. **Employment Generation:** The sector has the potential to create nearly **3 million jobs** by 2027.
6. **Geriatric Health Management:** Offers non-invasive therapies for India's aging population to improve quality of life.
7. **Economic Growth Engine:** The AYUSH industry is projected to reach **$200 billion by 2030**.
8. **Medical Value Travel (MVT):** India is the global wellness hub, attracting millions for traditional treatments.
9. **Soft Power Projection:** Yoga and Ayurveda serve as cultural ambassadors, enhancing India's global standing. **Eg. WHO Global Traditional Medicine Centre**.

Steps Taken

1. **National AYUSH Mission (NAM)**
   a. Creating awareness about the benefits of Ayush
   b. Increasing accessibility to Ayush services
   c. Promoting the cultivation of medicinal plants
   d. Strengthening educational institutions for Ayush
2. **Ayush Mark and Ayush Premium Mark certification** programs for Ayush products.
3. **Champion Sector Services Scheme (CSSS)**: To develop skilled human resources in the AYUSH sector and promote medical value travel services.
4. Establishment of **National Commission for Indian System of Medicine (NCISM)** and **National Commission for Homeopathy (NCH)** to improve education and practice standards.
5. **Ayurgyan Scheme:** Implementing a central scheme for capacity building, research, and **Ayurveda Biology** integrated research.
6. **Regional MVT Hubs:** Integration of AYUSH into **5 Regional Medical Value Tourism Hubs** for international patients.
7. **Ayush Grid**: Modernizes traditional medicine via **MAISP** for services, **Nivesh Saarthi** for investment, and **NAMASTE** for standardized global health reporting.
8. **Global Standardization (ICD-11):** Integration into WHO's ICD-11 enables systematic global reporting and clinical validation.
9. **Insurance Integration:** Collaboration with IRDAI has enabled over 27 insurers to cover Ayush treatments, making holistic care affordable and accessible.

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Major Challenges

1. **Validation Gap:** Many therapies lack rigorous, double-blind clinical evidence, hindering global scientific acceptance.
2. **"Mixopathy" Controversy:** Conflict over AYUSH practitioners performing surgeries or prescribing allopathic drugs.
3. **Heavy Metal Toxicity:** Lead and mercury in some formulations causes international safety alerts and bans. **Eg.** Repeated US/EU trade alerts regarding **Bhasma safety.**
4. **Standardization Gap:** Lack of uniform protocols for diagnosis and drug formulations across different practitioners.
5. **Non-Integration into Mainstream:** Traditional medicine is often used as "supplementary" rather than integral to the primary health mix.
6. **Biopiracy Threats: Eg.** Ongoing patent disputes over modified Curcumin.
7. **Inadequate R&D Funding:** Private sector investment in traditional medicine research remains negligible compared to standard pharmaceuticals.

Way Forward

1. Require **peer-reviewed clinical trials** for all new AYUSH formulations to build global trust.
2. **Integrative Curricula:** Introduce "Ayurveda Biology" and basic AYUSH concepts in allopathy medical education (and vice versa).
3. **Blockchain for Purity:** Use digital ledgers to track herbs from "Farm-to-Pharmacy," ensuring 100% organic authenticity.
4. **Scale Bharat-VISTAAR:** Leverage AI to predict disease trends and optimize herb cultivation for farmers.
5. **One Nation One Standard:** Finalize a unified pharmacopoeia for all systems to eliminate labeling and safety confusion.

Regulation of Medical Education

Key Data Points

1. **Total Medical Colleges: 822** (increased from 387 in 2014).
2. **Total MBBS Seats:** Over **1,29,000** seats available for the 2026 academic cycle.
3. **Postgraduate (PG) Seats:** Roughly **81,000- 85,000** seats (including MD/MS and DNB).
4. **AIIMS Expansion:** Total of **23 AIIMS** are now operational across India.
5. **Doctor-Population Ratio: Reached 1:834 (including AYUSH), surpassing the WHO norm of 1:1,000.**

Medical Education Architecture

1. **Regulatory Body:** The **National Medical Commission (NMC)**, established under the **NMC Act, 2019**, replaced the Medical Council of India (MCI). It oversees four autonomous boards
   a. **Undergraduate Medical Education Board (UGMEB)**
   b. **Postgraduate Medical Education Board (PGMEB)**
   c. **Medical Assessment and Rating Board (MARB)**
   d. **Ethics and Medical Registration Board (EMRB)**
2. **Pradhan Mantri Swasthya Suraksha Yojana (PMSSY)** - 22 new AIIMS established. 157 new medical colleges approved.
3. **Entrance/Licensing:**
   a. **NEET-UG/PG:** UG entry-level exams.
   b. **NExT (National Exit Test):** Phased in as a common final-year licentiate exam and PG entrance test.

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Major issues

1. **Systemic Integrity Failure:** NEET-UG 2024 paper leak and grace marks scandal exposed National Testing Agency's systemic vulnerability.
2. **Ghost Faculty Malpractice:** Private colleges often "hire" doctors only for inspection days to meet NMC norms.
3. **Inadequate Research Output:** Undergraduate education remains rote-learning heavy with minimal focus on "Physician-Scientist" pathways. **Eg.** India contributes <5% of global high-impact medical research papers
4. **Urban-Rural Divide:** Expanding seats produces doctors; but 80% of **Community Health Centres** specialist posts remain vacant in rural areas.
5. **PG Seats Bottleneck:** India produces ~1.3 lakh MBBS graduates/year but has only 72,627 PG seats - forcing a specialisation crisis.
6. **Private College Fee Crisis:** Private medical college fees (Rs 12-40 lakh/year) make MBBS unaffordable for middle-class families.
7. **Faculty Shortage:** Rapid seat expansion is outpacing the supply of qualified medical teachers.
8. **Gender Safety in Medical Colleges:** The 2024 Kolkata RG Kar Medical College rape-murder of a postgraduate trainee exposed systemic safety failures.
9. **Rural Retention:** Service bonds and compulsory rural postings are criticised as coercive and counterproductive without structural incentives.
10. India's **medical curriculum** has not adequately integrated AI, digital health, telemedicine, and precision medicine.
11. **Foreign Medical Graduates** face low pass rates (~20%) in screening exams like FMGE/NExT.

Way Forward

1. **Link private colleges to district hospitals** to serve the underserved areas of the country. (NITI Aayog)
2. **Outcome-Based Accreditation:** Link college ratings to the performance of their students in the NExT exam.
3. **Rural Incentives:** Offer "PG Seat Reservations" or "Salary Bonuses" for students completing 3 years of rural service.
4. **Faculty Development:** Create a **"National Faculty Bank"** to allow mobile, high-quality teaching across various states.
5. **Integrative Learning:** Introduce AI and **Digital Health** modules in the MBBS curriculum
6. **Inter-Disciplinary Research:** Set up "Innovation Labs" in medical colleges to encourage bio-engineering and medical startups.
7. **Focus on Allied Health:** Strengthen the **National Commission for Allied and Healthcare Professions** to reduce the doctor's load.

Mental Health

Mental health is defined as a state of well-being in which an individual realizes their own abilities, can cope with the normal stresses of life, can work productively, and is able to make a contribution to their community (**WHO**).

The **Mental Healthcare Act (2017)** defines it as a **"substantial disorder of thinking, mood, perception, orientation or memory."**

According to **Sigmund Freud**, mental health is the capacity of an individual "to work and to love".

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In the Sukdeb Saha case, the SC recognised **mental health as a** part of **right to life under Article 21**.

> Mental health in India is no longer a **"silent pandemic,"** but a mainstream public health priority. **Economic Survey 2025-26** - digital addiction and youth suicide as critical economic risks.

1. Roughly **10.6% of Indian adults** (approx. 150 million) suffer from mental morbidity.
2. According to a **NIMHANS study**, mental health disorders are more prevalent among women (20%) than men (10%).
3. **Economic Loss:** Untreated mental health conditions are estimated to cost India **$1.03 trillion** between 2012-2030 (**WHO**).
4. **Suicide Burden:** India accounts for nearly **one-third of global suicides**. It is the leading cause of death for ages 15- 29 (**NCRB 2025**).
5. **Digital Addiction:** Over **40% of urban youth** report "high distress" linked to screen time and social media algorithms (**2026 Trend Report**).
6. **Urban-Rural Divide:** Morbidity is significantly higher in urban metros (**13.5%**) compared to rural areas (**6.9%**).

Major Factors Behind the Crisis

1. **Digital Addiction & Screen Fatigue: Eg. Economic Survey 2025-26** flagged **digital burnout** as a primary driver of adolescent clinical depression.
2. **Stigma and Social Exclusion:** Fear of being labeled "mad" prevents early intervention, allowing mild cases to become chronic.
3. **The "Hustle Culture":** Societal glorification of overwork and constant productivity leads to stress and workplace burnout. **Eg.** "Karoshi" (death from overwork) phenomenon in IT hubs.
4. **Academic Hyper-Competition:** Intense pressure from coaching ecosystems and high-stakes exams (NEET/JEE). Eg. rising suicide cases in Kota
5. **Transition to nuclear families** has removed traditional community-based emotional buffers and support systems. **Eg.** Loneliness among the urban elderly has doubled since 2015 (NSO).
6. High correlation between **rising drug addiction and mental disorders. Eg.** Punjab's "integrated clinics" report that 60% of addicts suffer from co-occurring depressive disorders.
7. **Gender-Based Violence:** Victims of domestic abuse face high risks of PTSD and long-term depressive illness.
8. **Financial Instability:** Debt-cycles and lack of job security in the informal sector create persistent survival-related anxiety. **Eg.** Farmer suicides in the Vidarbha region
9. **Social Comparison:** Platforms like Instagram and TikTok create an "altered reality" fueling inadequacy and body dysmorphia.

Challenges in addressing mental health crisis

1. **Huge Treatment Gap:** The **treatment gap** for common mental disorders is estimated to be between **70%-90%**. **National Mental Health Survey (NMHS)**
2. **Severe Shortage of Mental Health Professionals:** India has only **0.75 psychiatrists per 100,000 people**, far below the **WHO norm of 3 per 100,000**.
3. **Stigma and Social Discrimination:** Mental illness is often linked with **superstition, fear, and shame**, leading to **social exclusion** and delayed help-seeking.

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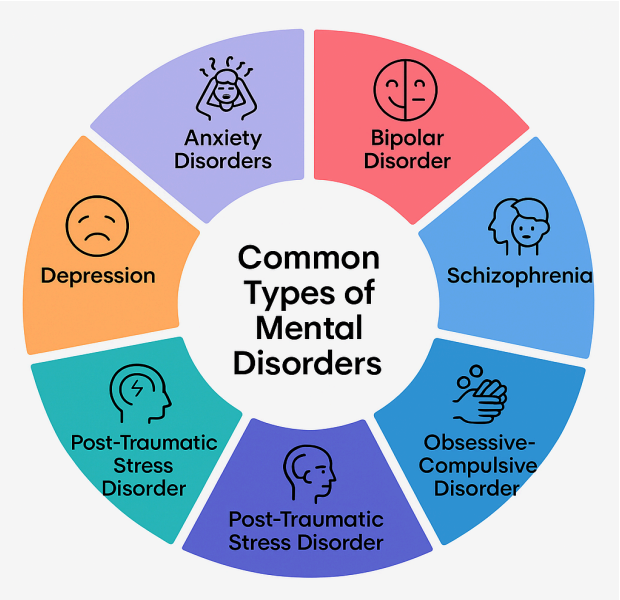
4. **Urban-Rural Divide**: Lack of trained staff, awareness, and infrastructure in rural India widens the **mental health service gap**.
5. Mental health services are **not fully integrated** into **primary health centres (PHCs)** despite the **Mental Healthcare Act, 2017** mandating universal access.
6. **Inadequate Public Spending:** India spends **less than 1.5% of the total health budget** on mental health (MoHFW, 2024-25).
7. **Suicide as a Public Health Crisis**
   a. India accounts for **36.6% of global female suicides** and **24.3% of male suicides** (Lancet Psychiatry).
   b. **NCRB 2025**: Over **1.7 lakh suicides** reported in India, with financial distress, unemployment, and family issues as top causes.



8. **Weak Implementation of Mental Healthcare Act, 2017: Many states have not fully implemented** provisions like State Mental Health Authorities, Mental Health Review Boards, and free treatment
9. **Low Mental Health Literacy:** Lack of awareness about **early symptoms**, available treatment, and rights leads to **late diagnosis and poor outcomes**.

Steps taken

1. **National Mental Health Programme (NMHP):**
   a. **District Mental Health Programme (DMHP):** focuses on community mental health services at the primary healthcare level, covering 716 districts.
   b. Includes counselling, OPD, suicide prevention, and 10-bedded inpatient facilities.
2. **Mental Healthcare Act, 2017** replaced the 1987 law, ensuring the right to mental healthcare and decriminalising suicide.
3. **National Health Policy (NHP), 2017** recognized mental health as a priority and integrated it into primary healthcare.
4. **2022 - National Suicide Prevention Strategy (NSSP)** launched to reduce suicide mortality by **10% by 2030**.
5. **NIMHANS and iGOT-Diksha Collaboration for Psycho-social Support and Training**
6. **Ayushman Bharat** - mental health services are now an integral part of Comprehensive Primary Health Care in over 1.75 lakh Ayushman Arogya Mandirs
7. **Kiran**: A 24/7 toll-free helpline to offer support to those dealing with mental health issues
8. **Manodarpan**: Its components include **Advisory Guidelines** for students, teachers and faculty of School systems and Universities along with families; National level database and directory of counselors; Toll-free helpline; Handbook on Psychosocial Support etc.

State-Level initiatives

1. **Tamil Nadu's "Natpudan Ungalukkaga":** A dedicated mental health helpline for students and suicide prevention during board exams.
2. **Karnataka's "Mano Chaitanya":** Providing free psychiatric consultations and medicine every Tuesday at the Taluk level ("Super Tuesdays").
3. **Meghalaya's Mental Health Policy:** The first state to have its own holistic policy focusing on "social determinants" and community involvement.
4. **Delhi's "Happiness Curriculum":** Mindfulness and emotional well-being classes in government schools to build resilience from childhood.

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Global Level Steps

1. **WHO Comprehensive Mental Health Action Plan (2013-2030):** The global roadmap for strengthening governance and community care.
2. **UN Convention on Rights of Persons with Disabilities (UNCRPD):** India is a signatory; it mandates dignity and non-discrimination for those with mental illness.
3. **Mental Health Gap Action Programme (mhGAP):** WHO initiative to train non-specialist health workers in low-resource settings.
4. **PARO Declaration (2024):** South-East Asia's commitment to transitioning from long-stay psychiatric hospitals to community networks.

Way Forward

1. Mandate **time-bound constitution** of State Mental Health Authorities (SMHAs) and Mental Health Review Boards (MHRBs) in all states. Ensure budget earmarking as per **Section 18**.
2. Raise mental health allocation to **≥2% of health budget**, with ring-fenced funds under **National Health Mission (NHM)**
3. **Bottom-Up Approach:** Transition from hospital-based care to **"Community Mental Health Networks."** Train ASHAs, CHOs and ANMs in **WHO's mhGAP-IG framework**..
4. Mandate **SEL (Social Emotional Learning)**, stress management modules, and school counsellors in NEP implementation under **NCERT guidelines**.
5. **Real-time Data Monitoring:** Develop **National Mental Health Dashboard** tracking suicide rates, treatment gap, service delivery, etc., to inform policy decisions.
6. **Focus on High-Risk Groups:** Targeted schemes for **LGBTQIA+ youth, prisoners, disaster survivors**, and **postpartum women** through intersectional and trauma-informed care models
7. **Learn from the HIV-AIDS Model: Evidence-based interventions, community participation**, decentralized care, and stigma reduction
8. **Workplace Regulation:** Enforce the "Mental Health at Work" guidelines to curb toxic overwork and promote "Right to Disconnect."
9. **Digital Detox Policies:** Introduce "Screen-time Management" education and regulate the addictive algorithms of social media apps.

"There is no health without mental health." - WHO. Mental health must be central to India's public health agenda, not peripheral.

Digital Health

Digital Health refers to the use of **information and communication technologies (ICT)** to deliver healthcare services, manage health data, and improve the efficiency of the health ecosystem.

Key Policies Shaping Digital Healthcare

1. National Health Policy (NHP) 2017: Promotes digital technology, electronic health records, and telemedicine to bridge service gaps and improve healthcare efficiency and accessibility.

2. National Health Mission (NHM): NHM is a program that works to achieve the goals of the NHP. Provides financial and technical support to states, focusing on improving healthcare infrastructure and accessibility for underserved and marginalized rural populations.

3. Health Data Management Policy: A critical component of ABDM, this policy outlines data privacy, security, and governance standards for digital health records.

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4. National Digital Health Mission (NDHM): NDHM will implement the following digital systems across the country:

- **Unique Health ID (UHID)** to identify and authenticate individuals based on past health records.
- **Digi Doctor:** A repository of doctors with individual details like name, institution, qualification, specialization and years of experience.
- **Health Facility Register (HFR):** A repository of Health Facilities across the country.
- **Personal Health Records (PHR):** A PHR is an electronic record of an individual which would contain health-related information of that individual.
- **Electronic Medical Records (EMR):** An app that contains the medical and treatment history of a patient.

5. **PM-ABHIM:** The largest pan-India infrastructure scheme strengthening laboratories, emergency responses, and medical research through integrated digital databases and modern facilities.

Initiative	Highlights
Ayushman Bharat Digital Mission (ABDM)	• **Health ID**: • **Healthcare Professionals Registry (HPR)** • **Health Facility Registry (HFR)** • **Unified Health Interface (UHI):** An open network facilitating digital health services. • Includes **ABHA** (78+ crore accounts), **HPR** (Health Professionals Registry), and **HFR** (Facility Registry). Integrates AI research with IIT Kanpur.
Aarogya Setu	• National Health App powered by ABDM. • Supports ABHA registration, digital prescriptions, lab reports, and appointment booking via e-Sanjeevani.
e-Sanjeevani	Free government **telemedicine platform**; over **20 crore consultations** by 2024. • **e-Sanjeevani OPD**: Facilitating doctor-to-patient consultations remotely. • **e-Sanjeevani AB-HWC**: Connecting Health and Wellness Centers (HWCs) with specialist doctors for better healthcare accessibility in remote areas.
U-Win Portal	Digitizes vaccination records for **pregnant women and children (0-17 years)** under the **Universal Immunization Programme**. Key features include **self-registration**, **QR-based e-vaccination certificates**, **ABHA & Child ABHA ID creation**, and **multilingual access** in 11 languages.
CoWIN Platform	Used for **COVID-19 vaccine registration and tracking**
Tele-MANAS	Launched in 2022; 53 cells across 36 states/UTs have handled over 17.6 lakh mental health counseling call
National Health Stack (NHS)	Digital infrastructure blueprint for healthcare policy implementation and health insurance.

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Initiative	Highlights
e-Hospital	SaaS-based cloud solution connecting patients and doctors; includes the **Online Registration System (ORS)**.
e-BloodBank	Facilitates the implementation of a complete blood bank management system.

Key Benefits of Digital Health in India

1. **Enhanced Access to Specialized Care:** Platforms like **e-Sanjeevani** have facilitated over **28.2 crore consultations**, bringing specialist advice to remote areas.
2. **Continuity through Interoperability:** The **ABDM framework** allows any doctor across India to access a consistent medical history.
3. **Efficiency and Cost Reduction:** The **ABDM "Scan & Share"** QR system has reduced hospital registration times from **2 hours to 4-5 minutes**.
4. **Strengthened Disease Surveillance:** Digital databases enable real-time tracking of non-communicable diseases like diabetes and hypertension.
5. **Empowered Patient Ownership (ABHA):** With **ABHA IDs**, citizens now hold their own longitudinal health records, reducing dependence on physical files and repeat testing.
6. **Precision Immunization Tracking:** The **U-Win portal** tracks **beneficiaries** to ensure zero missed doses for mothers and children.
7. **Multilingual Interfaces:** Key platforms like **U-Win** and **e-Sanjeevani** are now available in **11+ regional languages**

Major Issues and Limitations

1. **Private Sector Participation Gap:** Despite the **Digital Health Incentive Scheme (DHIS)**, many mid-sized private providers remain hesitant to integrate due to perceived high costs.
2. **The Digital Divide:** While mobile penetration is high, "digital literacy" remains low.
3. **Cybersecurity Vulnerabilities:** High-profile incidents like the **AIIMS ransomware attack** highlight the critical need for more robust encryption.
4. **Interoperability Friction:** legacy systems in many older hospitals still struggle to communicate with the modern ABDM "Health Stack."
5. **Regulatory Lag:** The rapid rollout of AI and machine learning in health has moved faster than the implementation of comprehensive, legally binding digital health laws.
6. **Physical Infrastructure Constraints:** In "shadow zones" of rural India, unreliable electricity and inconsistent 4G/5G speeds frequently lead to dropped telemedicine calls.
7. **Informed Consent Ambiguity**, especially regarding how their data is used for third-party research.
8. **Data Entry Burden** crisis for frontline health workers, who must often maintain both physical registers and digital U-Win/ABDM records.

Way Forward

1. **Adopt Open Standards for Interoperability:** Implement the **full-STAC** (Standards, Technologies, Architecture, and Content) model to enable seamless data exchange.
2. **Expand Digital Infrastructure in Rural Areas**
3. Enforce the **Digital Personal Data Protection Act** with emphasis on consent-based data sharing.
4. **Enhance Digital Literacy among Healthcare Workers:** Integrate digital health modules in MBBS and nursing education.

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5. **Promote Public-Private Partnerships (PPPs):** Engage private players to co-develop platforms, innovations, and scale-up services.
6. **Scale AI-based Diagnostics and Monitoring:** Use AI for imaging, early diagnosis, and triaging-especially in oncology and cardiology.
7. Align national frameworks with the **WHO Global Initiative on Digital Health**. Ensure clear governance structures and regulatory oversight.
8. **Build Strong Monitoring and Evaluation Mechanisms:** Establish KPIs and dashboards for real-time tracking of digital health implementation.

"Digital health is a great equalizer and has the potential to make quality healthcare affordable and accessible for all." - **PM Narendra Modi**

ASHA (Accredited Social Health Activist) Workers

It is a community health worker instituted under the **National Rural Health Mission (NRHM)** (2005), now part of the **National Health Mission**.

ASHAs serve as an interface between the community and the public health system, especially in rural areas. There are **over 10 lakh ASHA workers** across India

Success story - In **Bihar**, ASHAs were instrumental in boosting institutional deliveries from 39% (2005) to over 80% (2025).

COVID-19 Role: ASHAs were frontline workers, conducting door-to-door surveys, contact tracing, vaccination drives.

> **2022**: ASHA workers were awarded the **WHO Global Health Leaders Award** for their crucial role, especially during COVID-19.

Key Functions

1. Facilitate access to health services like immunization, institutional delivery, contraception, and basic sanitation.
2. Promote hygiene, nutrition, and preventive healthcare practices.
3. Act as DOTS (Directly Observed Treatment, Short-course) providers for TB.
4. Provide first contact healthcare and refer serious cases to health facilities.
5. Maintain health records and assist in surveys (Eg. NFHS, COVID tracking).

Importance of ASHA Workers

1. **Last-Mile Connect:** Serve as the critical link between community and public health institutions.
2. **Maternal Safety:** Connection with an ASHA increases the likelihood of safer institutional delivery by **1.6 times**.(PLOS Global Public Health study)
3. **Low-Cost, High-Impact Model:** Reduced maternal and infant mortality through community-level interventions.
4. **Behavioral Change Agents (Community Health Educators):** Influence social norms on hygiene, reproductive health, and nutrition.
5. **Crisis Response Role:** Frontline response during health emergencies such as COVID-19, dengue, and malaria outbreaks.
6. **Frontline Disease Surveillance Agents**: Handled COVID-19 contact tracing, TB tracking, and dengue control.

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7. **Enablers of National Health Programs**: Critical to Ayushman Bharat, NHM, and PM-JAY grassroots implementation.
8. **Agents of Women's Empowerment**: Contribute to women-led local governance under **Mission Shakti**.
9. **Global Recognition**: WHO's Global Health Leaders Award (2022) for public health leadership.

Government Measures

1. **Monthly Fixed Incentives:** ASHAs receive **₹2,000 monthly for routine tasks,** supplemented by performance-based incentives linked to various National Health Programmes.
2. **State-Level Top-ups:** Individual States and Union Territories provide additional financial incentives beyond the central government..
3. **Team Based Incentives (TBI):** Under Ayushman Arogya Mandirs, ASHAs can earn up to ₹1,000 monthly in team incentives based on monitored performance indicators.
4. **Non-Monetary Benefits:** ASHAs are entitled to essential resources including uniforms, identity cards, cycles, mobile phones, drug kits, and dedicated rest rooms.
5. **Retirement Recognition:** Workers with ten years of service receive a ₹20,000 cash award and a citation as an acknowledgment of their contribution.
6. **Social Security Package:** The government funds annual premiums for PMJJBY and PMSBY, providing life and accident insurance coverage of up to ₹2 lakh.
7. **Pradhan Mantri Shram Yogi Maan Dhan (PM-SYM):** Pension benefit of ₹3,000/- pm after age of 60 years (50% contribution of premium by GOI and 50% by beneficiaries).
8. **Health Insurance Expansion:** Healthcare coverage of ₹5 lakh per year under Ayushman Bharat (AB-PMJAY) has been extended to all ASHA workers recently.

Challenges Faced by ASHA Workers:

1. **Low and Irregular Remuneration**: Most earn ₹2,000-₹6,000/month (performance-based); delays in payments are common.
2. **Lack of Formal Employment Status**: Classified as "**volunteers,"** not regular government employees-no social security, pension, or insurance.
3. **Overburdened with Multiple Roles**: Expected to handle maternal care, NCD screening, surveys, mental health outreach, and more.
4. **Inadequate Training and Upgradation**: Many ASHAs lack updated knowledge for new health challenges (digital health, telemedicine).
5. **Exposure to Health Risks Without Safety Gear**: Face infectious diseases (Eg. COVID-19, TB) without proper PPE or support. Burnout and mental stress, especially post-COVID.
6. **Gender-Based Challenges and Harassment:** Face resistance, harassment, and lack of respect in patriarchal rural settings. No formal grievance redressal mechanism in many states.
7. **Community Stigma**: This risk resulted in stigma and discrimination from the community, labelling them as high-risk COVID-19 contacts.

Suggestions:

1. **Regularize Payments with Fixed Honorarium**: Ensure a minimum fixed salary, with performance incentives on top. Use DBT (Direct Benefit Transfer) to avoid delays.
2. **Recognize ASHAs as Formal Health Workers**: Grant official employee status under NHM or create a semi-formal cadre with job security, insurance, and pension.
3. **Standardize Workload and Roles**: Clearly define responsibilities; avoid ad hoc assignments (election, survey duties). Introduce a task-based workload monitoring system.

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4. **Training & Upskilling**: Regular capacity-building on NCDs, mental health, **digital tools, emergency care. Use models like ECHO and blended learning for remote areas.**
5. **Occupational Safety and Health Support**: Supply PPE, transport allowances, and mental health counseling services. Mandate risk allowances during epidemics or disaster situations.
6. **Institutionalize Grievance Redressal Mechanisms**: Create block-level helpdesks and toll-free complaint lines. Ensure representation in local health committees.
7. **Promote Community Respect and Recognition**: Include ASHAs in Panchayat-level planning and health events.

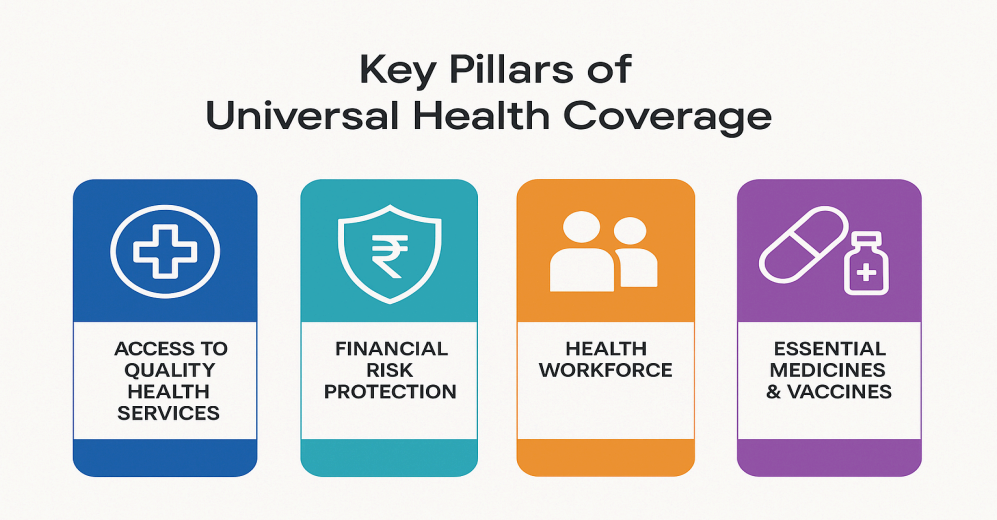
The success of the **National Health Mission** hinges on ASHA workers. Strengthening their role aligns with India's commitment to **accessible, affordable, and equitable healthcare for all**.

ASHAs are the grassroots enablers of **Ayushman Bharat**, and fulfilling their needs is key to realizing **'Swasth Bharat, Samriddh Bharat'** - a healthy and prosperous India.

Universal Health Coverage (UHC)

As per **WHO,** "UHC is about ensuring that people have access to needed health services when and where they need them, without financial hardship."

Enshrined in **National Health Policy 2017**: aims for "assurance-based healthcare," strengthening primary care, and reducing out-of-pocket expenditure.



Importance of UHC:

- **Ensures Right to Health for All** - UHC operationalizes **Article 21** (Right to Life) & **Directive Principles** (Art. 47).
- **Reduces Out-of-Pocket Expenditure (OOPE) -** UHC ensures financial risk protection and shields families from medical bankruptcy.
- **Enhances Equity and Social Justice - Eg.** Ayushman Bharat has enabled 25 crore poor citizens to access secondary and tertiary care (NHA 2025).
- **Improves Health Outcomes and Productivity**
- Healthier population = higher **demographic dividend** and **economic productivity**.
- Countries with UHC (Eg. Thailand, Sri Lanka) show better disease control and workforce efficiency.
- **Supports National and Global Goals** - Aligns with **National Health Policy 2017** & fulfills **SDG 3.8**: Achieve universal health coverage.
- **Mitigates Future Health Crises** - COVID-19 exposed India's healthcare gaps; UHC can build resilience through preventive and community-based care.

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Non-Communicable Diseases (NCDs)

An estimated 6.1 million Indians die from NCDs each year; approximately one in four Indians faces the risk of dying from an NCD before reaching the age of 70

India is projected to **lose $4.58 trillion by 2030** due to **NCDs and mental health disorders**.

Global:
Leading Cause of Death: NCDs account for approximately 74% of all global deaths, with over 43 million deaths in 2021.
Premature Mortality: Every 2 seconds, someone under the age of 70 dies from an NCD.
Disproportionate Impact: More than three-quarters of NCD deaths occur in low and middle-income countries.

India:

- NCDs are responsible for approximately **67.6% of all deaths** in India. (**37.9% in 1990**)
- **Premature Mortality:** Over **25% of NCD-related deaths** occur between **ages 30 and 70**
- **Cardiovascular Diseases (CVDs)**: CVDs remain the leading cause of death, contributing to around **28% of total deaths**.
- Hypertension affects roughly **30% of adults**, posing significant health risks for CVDs.
- **Diabetes**: India has an estimated **212 million people with diabetes**, accounting for about **26% of the global diabetic population**.
- **Cancer**: India is witnessing a significant rise in cancer cases, with projections indicating an increase from **1.39 million cases in 2020 to 1.57 million by 2025**.
- As per the **(NFHS)-5 (2019-21),** overall, **24% of Indian women and 23% of Indian men** are overweight or obese

Risk Factors

Risk Factor	Details
Triple Burden of Malnutrition	• Undernutrition, micronutrient deficiencies, and obesity.
Unhealthy Diet	• High intake of fats, salt, and sugar; low consumption of fruits and vegetables. • **NFHS-5:** Only **8.6% of women** and **7.7% of men** consume fruits daily.
Physical Inactivity	• Only **1 in 3 adults** meet WHO guidelines for physical activity. • Sedentary urban lifestyles contribute to obesity and diabetes.
Tobacco Use	• **28.6% of adults** use tobacco in some form (NFHS-5). India is the **second-largest consumer** of tobacco globally.
Harmful Use of Alcohol	• **18.7% of men** reported alcohol consumption (NFHS-5). Rising consumption among youth and women.
Air Pollution	• Caused **1.67 million deaths** in India (WHO, 2019). • Strongly linked to cardiovascular and respiratory NCDs.
Raised Blood Pressure	• **1 in 4 adults** affected (India Hypertension Control Initiative). • Leads to strokes, heart attacks, kidney failure. Often undiagnosed.

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Risk Factor	Details
Raised Blood Glucose (Diabetes)	• **212 million people** in India have diabetes (2024), highest in the world. • Often detected late; leads to vision loss, renal failure, CVDs.
Stress and Mental Health	• Chronic stress contributes to hypertension, obesity, and diabetes. • Mental disorders often co-exist with NCDs, reducing quality of life.
Genetic and Epigenetic Factors	• South Asians at higher risk of diabetes/CVD at lower BMI. • Intergenerational malnutrition (Eg. maternal undernutrition) increases adult NCD risk.

Steps Taken:

Government Initiative	Key Features
National Programme for Prevention and Control of Non-Communicable Diseases	• Covers Diabetes, Hypertension, Cancers (Oral, Breast, Cervical), Chronic Kidney Disease, STEMI, COPD, NAFLD, and Stroke. • 770 District NCD Clinics, 479 Day Care Centres, 6,410 CHC Clinics, and 233 Cardiac Care Units. • Universal screening initiative targeting all individuals **over 30 years of age**.
Ayushman arogya mandirs	• Provide **comprehensive primary healthcare**, including **NCD screening** and **wellness activities**.
India Hypertension Control Initiative (IHCI)	• Joint initiative of **ICMR, MoHFW, WHO**. • Promotes **standardized protocols** and regular follow-up.
PMJAY (Pradhan Mantri Jan Arogya Yojana)	• Secondary & tertiary care for **NCD-related hospitalizations** (Eg. cardiac stents, surgeries).
National Multisectoral Action Plan (NMAP)	• Promotes **whole-of-government** and **whole-of-society** approach. • Involves **inter-ministerial coordination** (transport, food, education).
Eat Right India (FSSAI)	• Encourages "Aaj Se Thoda Kam" (less salt, sugar, fat) and repurposes used cooking oil via the RUCO initiative.
Fit India & Khelo India	• National movements promoting daily fitness and world-class sports infrastructure to combat sedentary lifestyles.
National Tobacco Control Programme (NTCP)	• Focus on **awareness, cessation centres**, and enforcement of **COTPA (2003)**.
Taxation & Legislative Measures	• **Sin taxes** on tobacco, alcohol, sugary drinks. • **Front-of-pack labeling** norms by FSSAI.

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Global Measures	Key Features
WHO Global Action Plan for NCDs (2013-2030)	• Aims to reduce **premature NCD deaths by 25%** by 2030. • Sets **9 voluntary global targets** (Eg. tobacco use, salt intake, blood pressure control). • Promotes **"best buys"**: cost-effective policies like tobacco taxation, alcohol regulation, promoting physical activity.
Sustainable Development Goal (SDG) 3.4	• Target 3.4 aims to **reduce NCD mortality by one-third by 2030**. • Linked to SDG 3.8 on **Universal Health Coverage** • Emphasizes prevention, mental health, and wellness.
WHO Framework Convention on Tobacco Control (FCTC)	• First **legally binding international treaty** on health. • Adopted by 182 countries including India. • Focuses on **advertising bans, tobacco taxes, smoke-free laws, packaging regulations**.
WHO Best Buys for NCDs	• **Cost-effective interventions.** • 16 highly effective interventions, such as taxing sugar-sweetened beverages and reducing salt intake, yielding high health returns.
Global Coordination Mechanism on NCDs (GCM/NCD)	• Multi-stakeholder platform led by WHO • Facilitates **policy dialogues**, technical cooperation, and knowledge sharing.

Way Forward

1. **Integrate Health in All Policies (HiAP)**: A multi-sectoral approach ensures that health determinants are addressed comprehensively.
2. **Strengthen NCD Management via Primary Healthcare:** Eg. Kerala's Aardram Mission integrates NCD screening at the primary level.
3. **Adopt Life-Course Approach**: Begin interventions early in life, including maternal nutrition, adolescent wellness, and elderly care.
4. **Behavior Change Communication (BCC)** to promote physical activity, balanced diets, and reduce tobacco/alcohol use. Eg. Tamil Nadu's "My Health, My Responsibility" campaign.
5. **Leverage Digital Health Interventions (DHIs)**: Deploy low-cost DHIs (Eg. telemedicine, mHealth, chatbots).
6. **Use Fiscal Tools to Modify Risk Behaviors:** Implement higher taxes on tobacco, salt, and sugar to curb lifestyle-related risk factors and generate health financing.
7. **Promote Community Participation**: Strengthen Village Health Sanitation and Nutrition Committees (VHSNCs) to involve communities in health planning.

The battle against NCDs is not only medical but also **social and economic**-requiring a **whole-of-society and whole-of-government** approach

Antimicrobial resistance

1. Antimicrobial resistance (AMR) happens when **microorganisms like bacteria and viruses evolve to survive the drugs specifically designed to kill them.**

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2. This transformation renders standard treatments **ineffective**, leading to persistent infections and an increased risk of disease spread..
3. As per **WHO's, Global Antibiotic Resistance Surveillance Report 2025,** approximately 1 in 6 bacterial infections worldwide was caused by antibiotic-resistant bacteria.
4. In 2023, India, with one of the highest **AMR rates**, had 1 in 3 bacterial infections resistant to common antibiotics, with **ICU infections** by **E. coli**, **K. pneumoniae**, and **S. aureus** highly resistant.
5. **Mortality Projection:** An estimated **~39 million deaths** globally from antibiotic-resistant infections by **2050**. Of these, **2 million annual deaths** are projected in **India alone**.
6. **Economic Cost: $100 trillion** cumulative loss in global economic output due to AMR by **2050**.
7. AMR is now recognized as a **"Global Health Security Threat"** (WHO, 2022).

Reasons for the Spread of Antimicrobial Resistance (AMR)

1. **Irrational Use of Antibiotics in Humans**
   a. **Overprescription and misuse** of antibiotics for viral infections like cold and flu.
   b. **Self-medication and over-the-counter (OTC)** antibiotic sales without prescription.
   c. Inadequate **completion of antibiotic courses**, allowing resistant bacteria to survive.
2. **Overuse in the Livestock and Poultry** Industry as growth promoters. Eg. Use of colistin (a last-resort antibiotic) in poultry
3. **Inadequate sanitation, sterilization, and hygiene in hospitals** leads to resistant "superbugs." Eg. Hospital-acquired infections (HAIs) like MRSA and NDM-1
4. **Weak Regulation and Enforcement:** Poor implementation of the Schedule H1 Drugs Rule, Inadequate control over pharmaceutical waste disposal.
5. **Lack of Public Awareness and Health Literacy** about rational antibiotic use
6. Weak implementation of the **National Action Plan on AMR (2017)**, and absence of a unified **One Health surveillance system** linking human, animal, and environmental data.
7. **Pharmaceutical Industry Practices**: Inadequate R&D investment and dependency on old-generation antibiotics

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Implications of the Spread of Antimicrobial Resistance (AMR)

Public Health Implications

1. **Increased Mortality and Morbidity:** AMR infections cause nearly **5 million deaths annually** worldwide (WHO, 2024).
2. Common infections like **pneumonia, tuberculosis, urinary tract infections, and sepsis** are becoming untreatable.
3. **Rise of "superbugs"** such as **NDM-1**, **MRSA**, and **XDR-TB** makes routine surgeries risky.

Economic Implications

1. **Higher Treatment Costs:** Resistant infections require expensive, second- or third-line drugs.
2. **Global GDP loss:** The World Bank estimates AMR could cause a **3.8% GDP loss by 2050**, pushing **28 million people into poverty**.
3. **Healthcare burden:** India spends an additional **₹1.4 lakh crore annually** managing AMR-linked complications (ICMR, 2023).

Social Implications

1. Increased **inequality in healthcare access**: Poor patients cannot afford newer antibiotics
2. Erosion of **public trust in medical systems** due to rising treatment failures.

Healthcare System Implications

1. **Overburdened hospitals** with longer patient stays and recurring infections.
2. **Reduced surgical safety**: C-sections, transplants, and cancer therapies depend on effective antibiotics for infection prevention.
3. Risk of **nosocomial (hospital-acquired) infections** becoming endemic.

Agricultural and Environmental Implications

1. AMR reduces the effectiveness of veterinary treatments, impacting **livestock health and food security**.
2. **Contamination of soil and water** with antibiotic residues harms biodiversity.

Global Health and Security Implications

1. Cross-border spread through **travel, trade, and migration** undermines disease control efforts.
2. **Threat to Sustainable Development Goals (SDGs):** Especially SDG 3 (Good Health), SDG 2 (Zero Hunger), and SDG 6 (Clean Water).
3. Undermines **global pandemic preparedness**: Infections may no longer respond to standard treatments.

Measures Taken to Address AMR

Measure	Description
National Action Plan(NAP) on AMR (2017-2021)	• Framework to tackle AMR based on the **One Health Approach**. • Focuses on awareness, surveillance, infection prevention, antimicrobial supervision, and R&D.
NAP-AMR 2.0	• The updated 2025- 29 plan provides a budgeted, coordinated roadmap across twenty ministries to systematically contain antimicrobial resistance nationwide.
Drugs and Cosmetics Rules, 1945	It combats **AMR** by ensuring **antibiotic quality** via **Schedule M (Good Manufacturing Practices)** and promoting **rational use** through **prescription-only rules** under **Schedules H and H1**.

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Measure	Description
National AMR Surveillance Network (NARS-Net)	• Coordinated by **ICMR**, it collects and monitors AMR trends across hospitals and labs in India. • Data supports policy and clinical interventions.
FSSAI Initiatives	• Issued guidelines to curb **antibiotic residues in meat, milk, and poultry**. • Bans non-therapeutic antibiotic use in food animals.
Red Line Campaign	• Public awareness campaign to highlight the **Red Line on antibiotic packs**, warning against self-medication.
Over-the-counter(OTC) Sales Ban	• States like Kerala and Gujarat have prohibited antibiotic sales without prescriptions to curb dangerous and widespread self-medication.

Global Measures

Measure	Description
1. WHO Global Action Plan on AMR (2015)	• 5 strategic objectives including **improving awareness**, **reducing infection incidence**, and **optimizing antimicrobial use**.
2. Tripartite & Quadripartite Collaboration	• Coordinated efforts by **WHO, FAO, UNEP, and WOAH** under the **One Health** umbrella to address human, animal, and environmental health.
3. Global Antimicrobial Resistance Surveillance System (GLASS)	• WHO-led initiative for standardized AMR data collection and comparison globally.
4. Fleming Fund (UK)	• Supports low- and middle-income countries, including India, in building **lab capacity and surveillance infrastructure**.
7. ReAct Network and CDC Initiatives	• Independent global networks promoting **antibiotic stewardship**, advocacy, and research.• CDC provides global technical assistance on infection control and resistance monitoring.

Way Forward

1. Implement **National Action Plan on AMR (2017-2025)** effectively.
2. Establish a **One Health surveillance network** linking human, animal, and environmental data.
3. Enforce **antibiotic stewardship programs** across hospitals.
4. Incentivize **R&D for new antibiotics and diagnostics** through global partnerships (Eg. GARDP, CARB-X).
5. Implement **stringent waste disposal protocols** for pharmaceutical industries and healthcare facilities.
6. Conduct nationwide campaigns like the **Red Line Campaign** to educate the public on the dangers of antibiotic misuse.
7. Align national policies with the **revised WHO Global Action Plan on AMR**, emphasizing a One Health approach.

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"The rise of antimicrobial resistance is a global crisis that threatens a century of progress in health and medicine." - **World Health Organization**

One Health Approach

One Health is an integrated, unifying approach that aims to sustainably balance and optimize the health of people, animals, and ecosystems.

Key Components

1. **Human Health:** Public health surveillance, pandemic preparedness, and clinical care.
2. **Animal Health:** Livestock management, veterinary services, and wildlife disease monitoring.
3. **Environmental Health:** Ecosystem biodiversity, water/soil quality, and climate impact assessment.
4. **Integrated Surveillance:** Shared databases for early detection of "spillover" events across species.

Need for One Health in India

1. **Zoonotic Disease Burden:** India is a **global hotspot.** Over **60% of emerging infectious diseases in India** are zoonotic (Eg. Nipah virus, avian influenza, rabies, brucellosis)
2. **Antimicrobial Resistance (AMR):** Excessive antibiotic use in poultry and livestock creates drug-resistant "superbugs" infecting humans. **Eg. "Delhi Superbug" (NDM-1)**
3. **Livestock Economy Protection: Eg.** The **Lumpy Skin Disease** outbreak (2022-23) led to millions of cattle deaths and milk shortages.
4. **High Human-Wildlife Interface: Eg.** Kyasanur Forest Disease (**Monkey Fever**) in Karnataka.
5. **Climate Change Impacts:** Warming temperatures expand the geographical range of disease vectors like mosquitoes and ticks.
6. **Food Safety and Security: Eg.** Regular **Salmonella and E.coli** outbreaks in urban food chains require farm-to-fork surveillance.
7. **Pandemic Preparedness:** Shifting from "reactive" crisis management to an "anticipatory" model to prevent future COVID-like events.
8. **Biodiversity Conservation:** Protecting wildlife health is essential for maintaining ecological balance and preventing species extinction. **Eg. vulture population collapse** due to Diclofenac

Major Challenges in Adopting One Health

1. **Siloed Governance Models: Eg. Lack of unified databases** prevents real-time tracking of diseases across health, Agriculture, and Environment ministries
2. **Inadequate Veterinary Infrastructure: Eg.** Most rural veterinary clinics lack **BSL-2/3 labs** for rapid pathogen identification.
3. **Weak Wildlife Surveillance: Eg.** Forest departments lack the **epidemiological training** to detect early animal die-offs.
4. **Data Interoperability Issues: Eg. ABHA (Human)** and animal tagging systems are not yet digitally linked .
5. **Legal and Statutory Gaps:** There is no single "One Health Act" to mandate cross-ministerial cooperation.
6. **Public Awareness Deficiency: Eg. Poor hygiene in wet markets** persists because consumers are unaware of zoonotic risks.
7. **AMR Regulation Challenges:** Controlling over-the-counter antibiotic sales for livestock is difficult in a fragmented retail market. **Eg.** Widespread use of **Colistin** in poultry

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Steps Taken by Government

1. **National One Health Mission (NOHM):** Launched with a cross-ministerial effort involving **13 departments** to coordinate pandemic preparedness.
2. **National Institute for One Health (Nagpur):** Established as the anchor institution for research, training, and policy integration.
3. **National Joint Outbreak Response Team (NJORT):** A multi-disciplinary team comprising human, animal, and wildlife experts for rapid investigations.
4. **BSL-3/4 Lab Network:** A national grid of high-security labs; a new **BSL-4 facility** in Gujarat was foundation-laid in Jan 2026.
5. **One Health Governance Framework:** Released in Dec 2025 to provide states/UTs with a roadmap for setting up **State One Health Cells**.
6. **Integrated Disease Surveillance Program (IDSP):** Upgraded to include animal health indicators for a more holistic "Integrated Surveillance."
7. **National One Health Hackathon (IDEA ONE):** Launched to engage youth in solving inter-sectoral health challenges via technology.

Global Initiatives

1. **The Quadripartite Alliance:** Collaborative framework between **FAO, UNEP, WHO, and WOAH**.
2. **One Health Joint Plan of Action (2022-2026):** A global roadmap for mainstreaming One Health at national levels.
3. **One Health Summit (Lyon, April 2026):** Global leaders reaffirmed commitment to "Prevention at Source" under the French G7 Presidency.
4. **PREZODE Initiative:** A global program to prevent zoonotic disease emergence through socio-ecological research.

Way Forward

1. Establish a **National One Health Commission** to coordinate MoHFW, MoEFCC, and Animal Husbandry
2. **Statutory Mandate:** Enact a formal legal framework to institutionalize inter-sectoral coordination with clear leadership. (**EU Animal Health Act**)
3. Develop a **National One Health Digital Platform** for real-time data fusion and risk assessment.
4. Integrate **One Health modules** into Medical, Veterinary, and Environmental Science courses.
5. Ensure every state establishes a **One Health Cell** with dedicated financial and human resources.
6. **Empower Grassroots Workers:** Train ASHA workers and Veterinary assistants to work together on community surveillance.
7. Strictly enforce the **National Action Plan on AMR** with a focus on environmental discharge.
8. **Wildlife Sentinel Sites:** Establish dedicated monitoring zones in protected areas to detect "sentinel" animal die-offs.
9. **Public-Private Partnership:** Engage the private biotech sector for rapid development of "One Health Diagnostics."
10. **Social Science Inclusion:** Incorporate behavioral scientists to understand the human-animal relationship in various cultures.
11. **Sustainable Funding:** Create a dedicated **One Health Fund** to ensure long-term resilience against "Triple Threats" (Pandemics, Climate, AMR).

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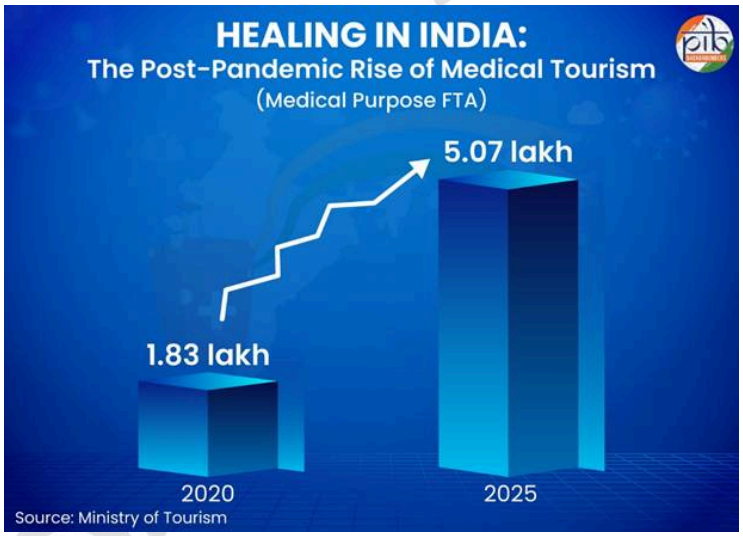
Medical tourism

India has solidified its position as the **"Pharmacy and Hospital of the World,"** with the Medical Value Travel (MVT) sector evolving into a strategic pillar of India's soft power and economic growth.

Key Data

1. The medical tourism market is about USD 8.7 billion in 2025, with projections of USD 16.2 billion by 2030.
2. **Global Ranking:** India ranks **10th** on the Medical Tourism Index (MTI)
3. India hosts over **40 JCI-accredited** and **600+ NABH-accredited** hospitals.
4. MVT accounts for roughly **5-7%** of India's total tourism revenue (**Economic Survey 2026**).



Key Driving Factors

1. **Cost Competitiveness: Eg.** A heart bypass costing $140,000 in the US is available for ~$5,000 in India.
2. **No Waiting Periods:** Immediate access to specialized care, unlike the 6-12 month wait times in many Western public systems.
3. **Skilled Medical Pool:** India possesses a vast cadre of English-speaking, globally trained doctors and surgeons.
4. **Technological Sophistication:** Widespread adoption of robotic surgeries, AI-diagnostics, and high-end radiological equipment (BSL-3/4 labs).
5. **AYUSH Integration:** Combining modern allopathy with traditional wellness (Ayurveda, Yoga) for holistic post-operative recovery.
6. **Visa Liberalization:** Streamlined e-Medical and e-Medical Attendant Visas for citizens of over **167 countries**. **Eg.** E-visa processing time reduced to under **48–72 hours** for medical cases.
7. **Hospitality Standards:** Hospitals provide concierge services, dedicated international wings, and multilingual staff (Arabic, French, etc.).
8. **Pharmaceutical Support:** Domestic production of low-cost biologics and generic medicines keeps the total package price low.

Major Benefits

1. **Foreign Exchange Earnings: Eg.** MVT revenue is growing at a **CAGR of 12-15%** as of 2026.
2. **Employment Generation:** Creates jobs for doctors, nurses, interpreters, and workers in hospitality and transport.
3. **Infrastructure Spillovers:** High-end infrastructure built for medical tourists also benefits the local domestic population.
4. **Reverse Brain Drain:** World-class research and high-paying roles encourage Indian doctors abroad to return home.
5. **Promotion of Soft Power:** Healing foreign citizens builds global goodwill and strengthens bilateral ties with the Global South.
6. **Growth of Ancillary Industries: Eg.** Dedicated "Medical Wellness Resorts" in **Kerala**
7. **Technological Progress:** High demand for advanced care incentivizes hospitals to adopt the latest medical innovations.

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8. **Cultural Exchange:** Traditional Indian wellness systems gain global legitimacy through direct patient experience.
9. **Standardization of Care: Eg.** Mandatory **ISO 22525** standards for medical wellness services.

Key Challenges

1. **Regional Concentration:** MVT services are heavily clustered in South and West India (Chennai, Mumbai, Delhi).
2. **Unregulated Intermediaries:** Exploitative unlicensed agents often mislead patients regarding costs and doctor credentials.
3. **Insurance Discrepancies: Eg.** Many African insurance providers do not yet have tie-ups with Indian hospital chains.
4. **Language Barriers:** Shortage of trained interpreters for languages like Arabic, French, or Russian in Tier-2 cities.
5. **Rising Global Competition: Eg. Thailand** dominates the cosmetic surgery market, while Singapore leads in high-end oncology.
6. **Quality Perception:** Lingering perceptions of poor general hygiene and antibiotic resistance (AMR) in Indian environments.
7. **Legal Recourse Gaps:** Lack of a clear, fast-track legal mechanism for foreign patients to address medical negligence.

Government Initiatives

1. **Heal in India Campaign:** A flagship initiative to position India as a global hub for integrated healthcare.
2. **Regional Medical Hubs (Budget 2026):** Proposal for **five integrated healthcare complexes** (Medical + Research + AYUSH) in partnership with states.
3. **Ayush Visa:** A dedicated visa category for foreigners seeking traditional Indian medical treatments.
4. **MVT Facilitation Centres:** Centralized units at airports and hubs to assist patients with immigration, customs, and transport.
5. **NABH Digital Health Certification:** Incentivizing hospitals to adopt digital records for seamless international insurance claims.
6. **National Medical & Wellness Tourism Board:** An apex body to coordinate between the Ministries of Health, Tourism, and Ayush.

Way Forward

1. **Accreditation for Facilitators: Eg. "Star Rating" system** for MVT facilitators.
2. **Global Insurance Portability:** Actively collaborate with international Third Party Administrators (TPAs) to enable "cashless" treatment.
3. **Decentralized MVT**: **Eg.** Developing **Special Health Zones (SHZ)** around greenfield airports.
4. **Unified National Branding: Eg.** Replicate the **"Incredible India"** success specifically for the healthcare sector.
5. **Language Skill Augmentation:** Upskill guides and nursing staff in foreign languages via the **National Institute of Hospitality**.
6. **Dedicated Medical Corridors:** Create high-speed, congestion-free road/rail links between major airports and medical hubs.
7. **Tele-Health Continuity: Eg.** Integrating **e-Sanjeevani** for international follow-ups.
8. **Ethical Oversight: Eg.** Real-time digital tracking of **organ donor-recipient matches** to prevent commercialization.
9. **Standardized Costing: Eg.** A central **MVT Price Portal** to ensure price transparency for global patients.

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Education in India

"Education is not only the foundation upon which our civilization has been built, but it is also the architect of humanity's future."**~Prime Minister Shri Narendra Modi**

- Education is the manifestation of perfection already in man - **Swami Vivekananda**
- **Confucius** - "If you think in terms of a year, plant a seed; if in terms of ten years, plant trees; if in term of 100 years, teach the people"
- "Education is the most powerful weapon you can use to change the world" - **Nelson Mandela**.

Early Childhood Care and Education (ECCE)

ECCE encompasses the holistic development of a child from **birth to eight years**.

It includes protective care, adequate nutrition, healthcare, and "play-based" early learning to foster physical, cognitive, and socio-emotional growth (**UNESCO/Ministry of Education**).

Importance of ECCE

1. **Cognitive Foundation: Eg.** 90% of cumulative brain development occurs before the age of 6 (**NCERT/NEP**).
2. **Economic Mobility: Eg.** Every $1 invested in ECCE yields a return of **$7 to $12** (**Heckman Curve**).
3. **School Readiness:** Reduces primary school dropout rates by easing the transition into formal 5+3+3+4 schooling.
4. **Nutritional Security:** Integrated delivery of supplementary nutrition during formative years prevents stunting and wasting. **Eg. Saksham Anganwadi** 2.0
5. **SEL (Social-Emotional Learning)** through collaborative storytelling and games - Develops empathy, self-regulation, and conflict-resolution skills.
6. **Language Acquisition:** The 3- 8 age window is the "sensitive period" for effortless multilingual and mother-tongue mastery.
7. **Health Prophylaxis: Eg. Rashtriya Bal Swasthya Karyakram (RBSK)** screenings at AWCs detect "4Ds" (Defects, Deficiencies, Diseases, Delays).
8. **Foundational Literacy & Numeracy (FLN):** Pre-primary exposure is the single biggest predictor of achieving Grade 3 literacy targets under **NIPUN Bharat**.
9. **Demographic Dividend:** Maximizing the potential of India's 15.8 crore children (0- 6) to drive future national productivity.

Major Challenges

1. **Enrolment Skew:** Nearly **25% of 5-year-olds** skip pre-primary and enter Class 1 directly, lacking readiness.
2. **Institutional Fragmentation: Split responsibility between the Ministry of Education and MWCD often leads to coordination "silos."**
3. **Pedagogical Gap:** Predominance of "rote learning" and "schoolification" (worksheets) instead of developmentally appropriate play.
4. **Teacher Capacity:** Anganwadi workers are overburdened with administrative tasks, leaving little time for teaching.

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5. **Funding Disparity:** India spends significantly less on pre-primary per child compared to primary or secondary education.
6. **Infrastructure Deficit:** Many AWCs operate from rented, cramped spaces without child-friendly toilets or play areas.
7. **Lack of Regulatory Standards:** Unregulated growth of private preschools has led to wide variations in safety and curriculum quality.
8. **Low Parental Awareness:** Parents often prioritize "writing and numbers" over "play and social skills" in early years.
9. **Urban-Rural Divide: Eg.** Rural children have 50% less access to structured pre-schooling compared to urban peers. (NSO)
10. **Language Barrier:** Scarcity of high-quality play-materials and picture books in regional tribal dialects.

Flagship Steps in India

1. **NEP 2020 (5+3+3+4):** Formalizes the 3- 8 age group as the "Foundational Stage," & aims to universalise ECCE by 2030.
2. **Aadharshila (2024):** A 48-week national curriculum for 3- 6 year olds, focusing on cognitive and psychomotor abilities.
3. **NIPUN Bharat Mission:** Targets universal Foundational Literacy and Numeracy for every child by the end of Grade 3.
4. **Saksham Anganwadi 2.0:** Upgrading AWCs with smart classrooms, clean energy, and modern pedagogical kits like **Jaadui Pitara**.
5. **Poshan Abhiyaan:** National Nutrition Mission ensuring health and diet are the bedrock of cognitive development.
6. **Vidya Pravesh:** A three-month "play-based" preparation module for children entering Grade 1 to ensure readiness.

State-Level Best Practices

1. **Odisha's "Shishu Vatikas":** a mandatory, play-based early childhood education program for children aged 5-6 in all government primary schools
2. **Kerala's "Varnakkoodu":** Upgrading Anganwadis into child-friendly "designer spaces" with themes, gardens, and localized play-kits.
3. **Madhya Pradesh's "Bal Choupal":** Direct community engagement where parents are taught how to play "learning games" at home.

Global Best Practices

1. **Finland (Play-centricity):** No formal academic instruction until age 7; focus remains entirely on play and social interaction.
2. **Reggio Emilia (Italy):** The "Environment as the third teacher" approach, using child-led exploration and arts.
3. **South Korea (Nuri Curriculum):** A unified national curriculum for all children aged 3- 5.

Way Forward

1. **Universal Access:** Saturation of **Balavatikas** (Preparatory classes) in every primary school by 2030 as per NEP.
2. **Cadre Creation:** Establish a specialized "ECCE Teacher Cadre" with diplomas in child psychology and pedagogy.
3. Increase **public spend** to at least **1.5% of GDP** specifically for foundational education.

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4. **Convergence:** Merge MWCD and Education Ministry data on a single **National One Health/Education** dashboard.
5. **Empower Parents:** Use EdTech to send daily play-activities to parents, making them "co-educators."
6. **District Planning:** Empower Panchayats to monitor AWC infrastructure under the **Village Health Sanitation and Nutrition Committee**.

School Education

Key Data Points

1. **Public Spending:** Education expenditure stands at ~**4.5% of GDP** in 2026, nearing the **6%** NEP target (**Economic Survey 2026**).
2. **Gross enrollment ration**
   a. **Enrollment in pre-primary** rose to **83.3%** in 2024 compared to **76%** in 2018 (**ASER 2024**).
   b. **Universal Enrollment (6- 14 years):** currently at **98.1%** vs **97.2%** in 2018 (**ASER 2024**).
   c. **Secondary GER:** rose to **78.5%** in 2024-25 from **50.1%** a decade ago (**UDISE+**).
3. **Secondary Dropout Rate:** Declined to **8.2%** in 2024-25 from **12.6%** in 2021-22 (**UDISE+**).
4. **Foundational Literacy Recovery: 23.4%** of Std III children can read Std II text, showing recovery from the **16.3%** pandemic low in 2022 (**ASER 2024**).
5. **"Small School" Trend: 52.1%** of govt primary schools now have <60 students, up from **44%** in 2022 (**ASER 2024**).
6. India has over **1.01 crore** school teachers, a **6.7%** rise over 2022-23 (**UDISE+**).
7. **Female Teacher Representation @ 54.2%**, up from **52.3%** in 2022 (**UDISE+**).
8. **Pupil-Teacher Ratio (PTR):** Improved to **21** at the Secondary level (NEP norm- 1:30) (**UDISE+**).
9. **64.7%** of schools have computer access today vs **47%** in 2021 (**UDISE+**).
10. **Internet Penetration: 63.5%** of schools are now connected, (**34%** in 2021) (**UDISE+**).
11. **Safe Drinking Water:** Reached near-universal levels at **99.3%** compared to **95%** in 2018.
12. **Girls' Toilets:** Functional access rose to **97.3%** in 2024-25 from **66.4%** in 2018 (**ASER**).
13. **90%** of adolescents (14- 16) have a smartphone at home, though only **37%** own one personally (**ASER 2024**).

Major Issues in School Education

Accessibility

1. **Out of school children**: over 1.4 million children are out of school due to poverty
2. **Adolescent Retention Bottlenecks: Eg.** Financial constraints account for **44% of dropouts** among senior school adolescents.
3. **Digital Inequity (The Rural Gap): Eg. only 18.47% of rural schools had internet access**, compared to **47.29% in urban areas**
4. **Gendered Dropout Drivers: over 40%** adolescent girls are disproportionately pushed out for domestic labor, marriage or due to menstrual hygiene gaps.
5. **Institutional Silos:** Disconnect between Anganwadi (Health) and Primary Schools (Education) delays early foundational learning intervention.

Quality

1. **Endemic Learning Poverty: Eg.** Only ~23% of Grade 3 students meet foundational reading benchmarks (ASER 2024).

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2. **Vocational Stigma: Eg.** Less than 1% of school-going adolescents have formal institutional vocational training.
3. **Teacher Non-Teaching Load:** Burdensome administrative tasks (surveys, data entry) reduce active "Time on Task" in classrooms.
4. **Rote-Learning Inertia:** Assessments still prioritize memorization over the NEP-mandated "critical thinking" and "applied knowledge" frameworks.
5. **Teacher Vacancy Crisis: Eg.** Over **1 lakh schools** across India continue to operate with merely a single teacher.

Infrastructure

1. **Lack of high-quality, scientifically accurate textbooks and teachers** in regional dialects hampers comprehension at foundational levels.
2. **Infrastructure "Maintenance" Gap: Eg.** 67,000 schools still report "non-functional" toilets despite having the structures.
3. **Multigrade Classroom Fatigue:** Small schools force one teacher to handle multiple grades simultaneously, diluting instructional quality.

Major Steps Taken

1. **NEP 2020 Structural Shift:** Redesigning the 10+2 system into a **5+3+3+4** pedagogical model that integrates three years of pre-schooling into the formal education continuum.
2. **NIPUN Bharat Mission:** Targeting universal **Foundational Literacy and Numeracy (FLN)** for Grade 3 by 2026-27 through "Lakshyas" (goals) like reading 60 words per minute.
3. **PM-SHRI (Schools for Rising India):** Upgrading **14,500+ schools** as "exemplar" institutions featuring smart classrooms, ICT labs, and eco-friendly infrastructure.
4. **PARAKH (Assessment Centre):** Establishment of a national center to standardize board exams and introduce the **Holistic Progress Card (HPC)** for 360-degree evaluation.
5. **PM POSHAN :** Extension of nutritional support to include **Balvatikas** (pre-primary) and the introduction of breakfast in several pilot states to drive enrollment.
6. **National Credit Framework (NCrF):** A unified framework allowing students to earn and store credits for both academic and vocational skills in a "Credit Bank."

Way Forward

1. **The "School Hub" Model:** Merge resource-starved small schools into "clusters" to share high-end labs, libraries, and specialized teachers.
2. Legally extend the **Right to Education Act** (currently 6- 14) to cover the entire 3- 18 age group (Foundational to Secondary).
3. **Digital Equity:** Provide individual tablets with pre-loaded e-content to rural students to bridge the personal-device gap.
4. **Outcome-Based Funding:** Link state grants to improvements in **National Achievement Survey (NAS)** scores rather than just enrollment numbers.
5. Adopt the **"Business Blasters"** and "Happiness Curriculum" models nationally to foster entrepreneurship and emotional resilience. **(Delhi Model)**
6. Replicate the **"KITE-Victers"** model for high-quality, state-wide digital broadcasting of core lessons. **(Kerala Model)**
7. **Education Management Cadre:** Establish a separate cadre for administrative tasks, allowing teachers to focus exclusively on classroom instruction.
8. **Biannual Admission Cycles:** Fully implement the proposal for **two-cycle admissions** to allow flexibility for students who miss the June cycle.

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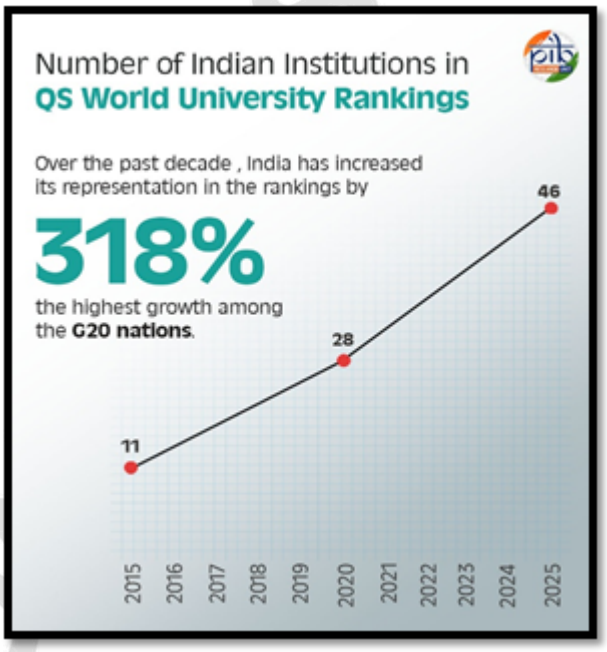
Higher Education in India

With over **4.6 crore students**, India now manages the world's second-largest higher education system.

Key Data Points

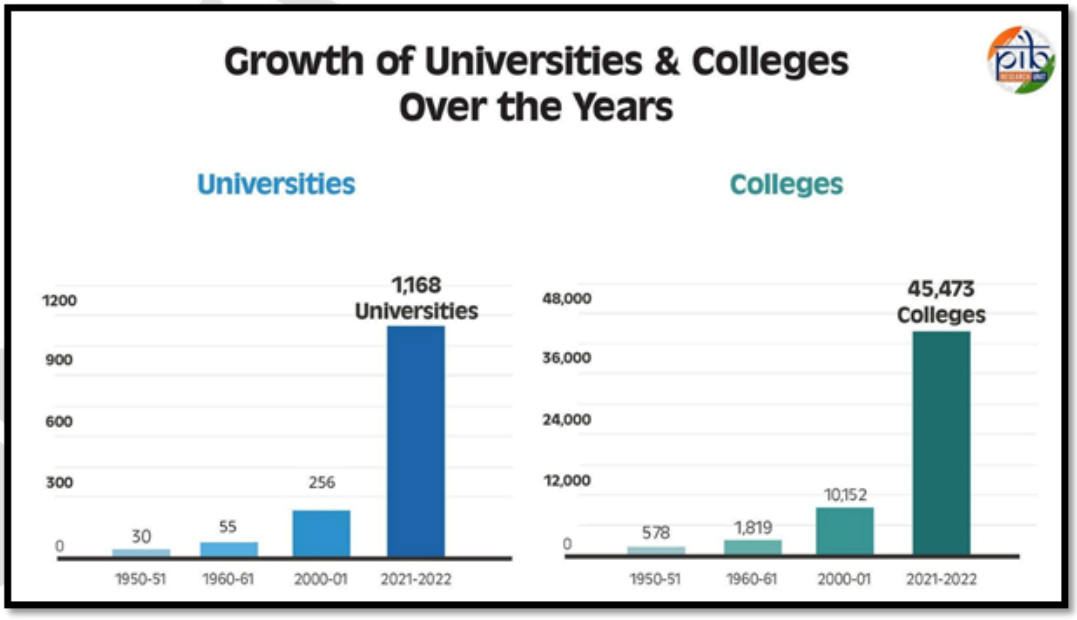
1. In 2021, **India allocated 1.57% of its GDP to higher education**
2. **Student Enrollment:** Total enrollment is estimated at **4.65 crore** in 2026, a significant jump from **3.42 crore** 2014-15 (**AISHE/NITI**).
3. **Gender Parity Index (GPI):** Standing at **1.01**, indicating slightly higher female participation than male participation for the first time in history.
4. Total **Ph.D. enrollment** increased by **81.2%** over the last decade, with female Ph.D. scholars doubling in the same period (**AISHE**).
5. India now hosts over **1,168 Universities** and **45,473 Colleges**, up from 760 universities in 2014-15 (**AISHE**).
6. R&D expenditure remains stagnant at **0.64% of GDP**, significantly lower than China's 2.4% or the USA's 3.4% (**Economic Survey 2026**).
7. **Internationalization:** India ranks **26th** as a global destination despite being 3rd in outbound students (**NITI Aayog**).



Regulatory Architecture

Apex Regulatory Layer

1. **University Grants Commission:** funding and standard-setting body for universities.
2. **All India Council for Technical Education (AICTE):** Regulates technical education.
3. **National Council for Teacher Education (NCTE):** regulates teacher training institutes (B.Ed, D.El.Ed).
4. **National Research Foundation (NRF):** An apex body created to seed, grow, and facilitate research across all HEIs.



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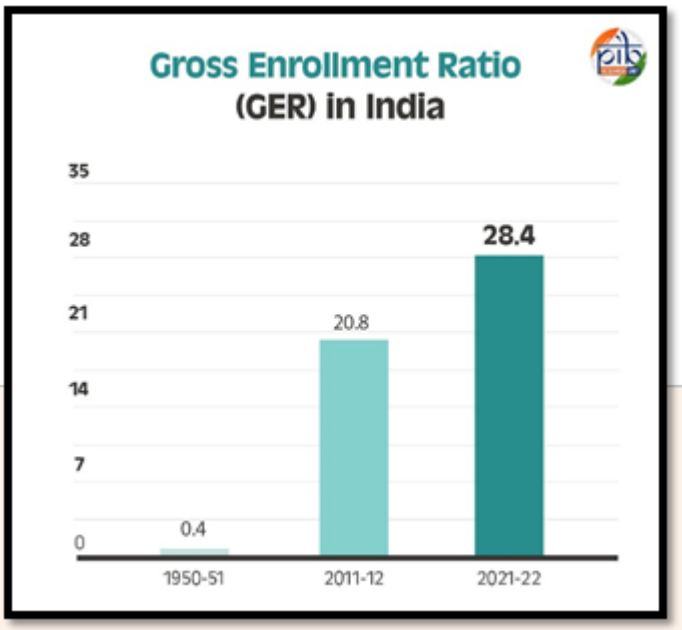
Accreditation Institutions:

1. **NAAC (National Assessment and Accreditation Council): assess and accredit Higher Educational Institutions (HEIs) in India.**
2. **National Board of Accreditation:** assesses the quality of technical and professional programs.



Professional Standard-Setting Bodies:

1. **National Medical Commission (NMC)**
2. **Bar Council of India (BCI)**
3. **Institute of Chartered Accountants of India (ICAI)**

Viksit Bharat Shiksha Adhishthan Bill, 2025

- The Bill establishes the Viksit Bharat Shiksha Adhishthan (the Commission) as the **apex regulatory body for higher education.**
- The Commission will have following **three Councils:**
- **Regulatory Council** which will function as the common regulator for higher education
- **Accreditation Council** to oversee the system of accreditation
- **Standards Council** to determine academic standards.
- The **functions** of the Commission include:
- Providing **strategic direction** for higher education and research
- **Developing a roadmap** for transforming higher educational institutions (HEIs) into large multi-disciplinary education and research institutions
- **Suggesting schemes** for improving the quality of education.
- **Appeals** against the decisions of the Commission and the Councils will be lie before the **central government**
- This body **replaces the University Grants Commission (UGC), All India Council for Technical Education (AICTE), and National Council for Teacher Education (NCTE).**
- The Bill **exempts legal and medical education** from its purview.

Major Issues in Higher Education

1. GER in higher education is **28.4% (AISHE 2021-22)**, still far from NEP 2020's target of **50% by 2035**.
2. **Quality vs Quantity Imbalance:** Rapid expansion led to **massive variations in academic quality**, especially in private colleges and tier-2/3 universities.
3. **Employability Gap: Eg.** TeamLease 2026 report notes 75% of HEIs are not "industry-ready" for hiring.
4. **Fragmented Regulatory Structure:** Multiple overlapping bodies like UGC, AICTE, NCTE create confusion and **bureaucratic delays**
5. **Faculty Vacancy Crisis:** Chronic shortages of permanent faculty in state and central universities hamper research and teaching.
6. **Ranking-Centric Academic Culture:** Institutions prioritize "superficial" publication counts for NIRF/global rankings over deep pedagogical innovation.
7. **Limited Research Funding:** 0.64% GDP spend on R&D is among the lowest in the G20. India contributes **<5%** of global research publications

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8. **Regulatory Overlap & Red Tape:** Multiple bodies (UGC/AICTE/State Boards) often issue conflicting guidelines, delaying institutional autonomy.
9. **Low Accreditation Coverage: Eg.** Over 30% of Indian HEIs remain unaccredited as of early 2026.
10. **Digital Divide in HEIs:** Rural colleges lack the high-speed connectivity required for advanced AI and cloud-based learning.
11. **Brain Drain (Outbound Mobility)** due to perceived lack of quality at home. **Eg.** India is the 3rd largest source of international students globally (NITI Aayog 2025).
12. **Stagnant Curricula: Eg.** Lack of mandatory "AI Ethics" or "Quantum Basics" in general B.Sc./B.Com streams.
13. **Poor Industry-Academia Linkage: Eg.** Only 4% of higher education focuses on formal skill-training.
14. While enrollment is equal, **women remain underrepresented in core STEM** and overrepresented in Arts. **Eg.** <20% of enrollment in top-tier engineering technology streams.
15. **The "Degree Factory" Model:** Proliferation of private universities that prioritize profit and volume over rigorous academic rigor.
16. **Crisis of Academic Integrity:** Rising cases of plagiarism and AI-generated academic fraud in thesis submissions.
17. **Examination Integrity Crisis**: Eg. **NEET-UG 2024 paper leak**
18. **Rising student suicide: Over 35% of recorded suicides in India occur in the 15-24 age group,** with academic pressure being a leading cause.

Major Steps Taken

1. **HECI Bill (2025):** Establishing a single-point regulator to streamline approvals and ensure "Light but Tight" oversight.
2. **Academic Bank of Credits (ABC):** A digital repository where students can store credits, enabling **multiple entry-exit** options.
3. **PMRF 2.0 (Budget 2025-26):** Expansion of Prime Minister's Research Fellowship to award **10,000 new fellowships** for doctoral research.
4. **APAAR (Digital Spine):** The "One Nation, One ID" for students, linking all academic records, skills, and transfers in one portal.
5. **AI CoE for Education:** Allocation of **₹500 crore** to set up Centers of Excellence in Artificial Intelligence at premier institutes.
6. **International Branch Campuses:** Allowing top-100 global universities (like Deakin/Wollongong) to set up campuses in GIFT City and beyond.
7. **Anusandhan NRF:** Functionalization of the National Research Foundation to bridge the gap between private R&D and public academia.

Way Forward

1. **Graded Institutional Autonomy**: Shift to **maturity-based self-governance** with "**light but tight**" regulation.
2. **Triple Helix Model (Govt- Academia- Industry)**: Create **regional innovation clusters** aligned with local economic needs (Eg. agritech, textiles).
3. **Pedagogy over Rankings:** Reform evaluation metrics to reward teaching quality and student mentorship rather than just paper volume.
4. **Apprenticeship-Embedded Degrees**: Integrate **30- 40% credits** through industry internships under **National Credit Framework (NCrF)**.
5. **Professor of Practice (PoP)**: Allow **industry experts** to teach without PhD requirement.

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6. **Phygital Learning Ecosystem**: Combine **physical + digital education** & enable **hub-and-spoke model**, allowing rural students to access courses from top institutions.
7. **Internationalisation at Home**: Promote **twinning programs, joint degrees, and collaborations with foreign universities**.
8. Adopt the **"Dual Education System"** where students spend 50% of their time in the workplace. **(Germany)**
9. **Regional Research Hubs:** Incentivize premier IITs/IIMs to mentor regional state universities through "Hub-and-Spoke" models.
10. **Equity in R&D:** Direct NRF funds specifically toward "Aspirational District" universities to balance research equity.
11. **Standardized Exit Exam:** Implement a national-level exit test for professional courses to ensure a minimum quality floor.
12. **Digital Resilience:** Invest in satellite-based internet for remote colleges to ensure "AI-readiness" for all students.

Internationalisation of Higher Education in India

Around **17 foreign universities** (mainly from **UK & Australia**) approved under **UGC Regulations 2023**.

Policy Framework

- **NEP 2020 Vision**
  - Focus on **Access, Equity, Quality, Affordability, Accountability**.
  - Allows **Top 100 global universities** to set up campuses in India.
  - Promotes **student mobility, credit transfer, and global standards**.
- **UGC Regulations 2023**
  - Permits **Top 500 QS-ranked universities** to establish campuses.
  - Ensures the same **curriculum, standards, and degree equivalence** as parent campus.
  - Grants **operational autonomy** (faculty hiring, fees flexibility).

Drivers for Foreign Universities

- **High Demand** - Young population, **GER < 30%**, rising middle class.
- **Supportive Policies** - NEP 2020 + enabling UGC framework.
- **Declining Overseas Students** - Stricter visa norms in **UK, US, Canada**.
- **Revenue Diversification** - New market amid funding constraints abroad.
- **Strategic Partnerships** - Research collaboration and talent pipeline (Eg. **UKIERI**).

Need for Internationalisation

- **Addressing Brain Drain** - India has a **1:28 inbound- outbound student ratio**; over **16 lakh Indians renounced citizenship since 2011** → need for **brain circulation**.
- **Enhancing Global Competitiveness** - Heavy dependence on **US, UK, Australia**; weak domestic **research ecosystem**.
- **Boosting Research & Soft Power** - Only **47,000 foreign students (2022)**, but with support, it has potential to reach **7.89- 11 lakh by 2047**.
- **Reducing Forex Outflow** - Overseas education spending projected at **₹6.2 lakh crore by 2025** (~**2% of GDP**).
- **Supporting Development Goals** - Builds a **high-skilled workforce** for economic growth and innovation.
- **Skilled Workforce** - Supports **AI, Data Science, Finance** aligned with **Digital India & Make in India**.
- **Affordability** - Foreign degrees at **~50% lower cost** than studying abroad.

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Key Challenges

- **Limited Scholarships** - **41% institutions** cite lack of financial aid as a major barrier.
- **Weak Global Perception** - No Indian university in **Top 50 (QS Asia Rankings 2026)**.
- **Regulatory & Visa Barriers** - Complex approvals; absence of **single-window system**.
- **Infrastructure Gaps** - Lack of **international-standard campuses and facilities**.
- **Difficulty in Attracting Foreign Faculty** - Due to **rigid rules, low pay, and tenure issues**.
- **Limited Global Courses** - Few **joint degrees / interdisciplinary programs**.
- **Weak Branding** - Lack of **global outreach and alumni networks**.
- **Autonomy Challenges** - Limited autonomy in **fees, curriculum, faculty** and **complex UGC approvals**.

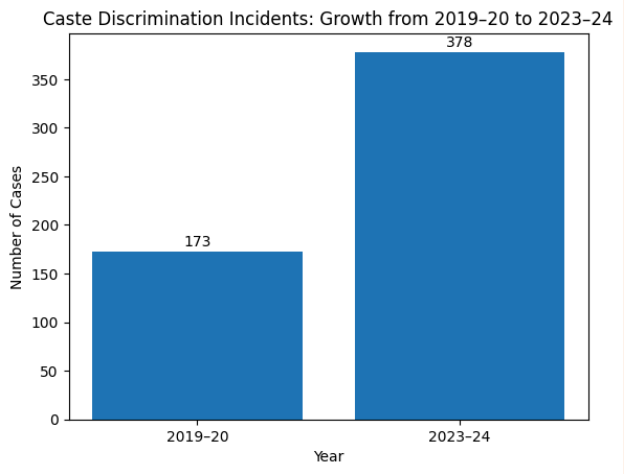
Key Recommendations

- **Introduce fast-track single-window clearances and ensure transparent policies on autonomy, taxation, and fund repatriation.**
- **Scholarships & Fellowships** - Launch **Vishwa Bandhu Scholarships & Fellowships** to attract global talent.
- **National Research Fund** - Create **Bharat Vidya Kosh (USD 10 billion)** with govt + diaspora funding.
- **Regulatory & Visa Reforms** - **Single-window clearance**, fast-track visas, flexible hiring for foreign faculty.
- **Ranking Reforms** - Expand **NIRF** to include **globalisation and partnerships metrics**.
- **Branding & Outreach** - Create **Bharat ki AAN (Alumni Ambassador Network)**.
- **Institutional Readiness** - Improve **student support systems and cultural integration**.
- **Strategic Academic Partnerships** - Enable **joint degrees, credit transfers, and collaborative research** on India-specific themes.
- **Robust Monitoring & Evaluation** - Define success using **research output, employability, and community impact metrics**.

UGC (Promotion of Equity in Higher Education Institutions) Regulations, 2026:

As per UGC, there is **118.4% rise in reported caste-based discrimination cases** in last 5 years

Key Provisions



- **Broad Coverage of Discrimination** - Extends protection to **OBCs**, along with **SCs, STs**.
- **Expanded Definition** - Covers **explicit and implicit discrimination** based on **caste, religion, gender, disability, etc.**, including violations of **human dignity and equality in education**.
- **Equal Opportunity Centres (EOCs)** - Mandatory in all institutions to ensure **equity, inclusion, and grievance redressal**.
  - **Equity Committee** with representation from **SC/ST/OBC/PwD/women**.
  - **Time-bound process**: Complaint review within **24 hours**, report in **15 days**, action within **7 days**.
  - Additional mechanisms: **Equity Squads, 24×7 helpline, Equity Ambassadors**.
- **Reporting & Accountability** -

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  - **Bi-annual reports** by EOCs and **annual institutional reports** to UGC.
  - **Institution head** made directly responsible for compliance.
- **National Monitoring Mechanism** - UGC to set up a **national committee** (with statutory bodies + civil society) for oversight and review.
- **Strict Penalties** - Non-compliance may lead to **loss of UGC recognition, debarment from schemes, or ban on academic programmes**.

Significance

- **Strengthens Anti-Discrimination Framework** - Addresses widespread caste discrimination
- **Inclusive Social Justice Approach** - Inclusion of **OBCs** broadens equity framework.
- **From Advisory to Enforceable** - Introduction of **penalties** ensures real accountability.

Concerns

- The SC has issued an interim stay on the regulations for excluding general category students, potential **division of society** and **undermining campus unity**.
- **No Safeguards Against False Complaints** - Removal of penalty provisions may lead to **misuse**.
- **Due Process Issues** - **Strict timelines** may compromise **fair hearing and natural justice**.
- **Administrative Burden** - Smaller institutions may struggle with **resources and compliance requirements**.

R&D in India:

Status of R&D in India

- **Low R&D Spending** - GERD at only **~0.64% of GDP**, with limited private sector role.
- Ranked **38th in Global Innovation Index (2025)** (up from 48th in 2020).
- India ranks **6th globally in patent filings**; patent-to-GDP ratio rose from **144 (2013) to 381 (2023)**.

Key Challenges

- **Funding Deficit** - Much lower R&D spending than **US (~3.5%), China (~2.4%), South Korea (~4.8%)**.
- Lack of **performance-based and outcome-linked funding models**.
- **Bureaucratic Bottlenecks** - Delays in **procurement, approvals, and compliance** hinder research speed.
- **Lab-to-Market Gap** - Weak conversion of **research into patents, products, and commercialization**.
- **Siloed Ecosystem** - Weak **University- Industry- Government collaboration** and poor interdisciplinary research.
- **Human Capital Issues** - Low **researcher density (~260 per million)** vs **>4000 in US/UK**; persistent **brain drain**.
- **Low Research Impact** - Weak **citation impact (CNCI)** and limited presence in **high-impact journals**.
- **Lack of academic autonomy:** The **Academic Freedom Index (AFI)** categorizes India's academic status as **"completely restricted"**.

Steps taken

- **The Research, Development, and Innovation (RDI) Scheme-** ₹1 lakh crore initiative aimed at boosting private sector participation in high-impact R&D.

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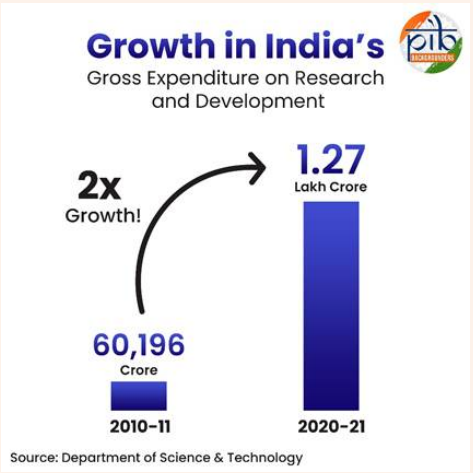

- **Star Faculty Repatriation Scheme** - Targets global Indian-origin **"star researchers"** with **lab grants, autonomy, and streamlined processes** in institutions like IITs and national labs.
- **Ramanujan Fellowship (DST)** - Attracts **mid-career Indian-origin scientists abroad** with **flexible institutional choice, funding, and research autonomy**.
- **Ramalingaswamy Re-entry Fellowship (DBT)** - Encourages long-term return in **biotech/biomedical fields** with **salary, HRA, and research grants**.
- **Prime Minister's Research Fellowship (PMRF)**
- **INSPIRE Faculty Award (AORC)** - Supports **early-career researchers** with **Assistant Professor-level funding and lab grants**
- **Biotechnology Research Clusters (DBT)** - Expansion of **world-class research hubs** (NCR, Mohali, Bengaluru) with **advanced labs and industry linkages**.
- **VAIBHAV Fellowship Program** - Promotes **diaspora collaboration and knowledge exchange**

Measures to Strengthen R&D

- **ROPE Framework** - Focus on **Removing Obstacles and Promoting Enablers** to eliminate administrative bottlenecks. (**NITI Aayog Report on Ease of Doing R&D in India**)
- **Increase Funding** - Raise GERD to **1.5 % of GDP**; promote **private investment, tax incentives, CSR funding**.
- **Strengthen Institutions** - Fast-track **ANRF** to support research beyond elite institutions.
- **Promote Commercialisation** - Establish **Technology Transfer Offices (TTOs)**; strengthen **industry-academia linkages**.
- **Mission-Driven Research** - Focus on **AI, Quantum Computing, Green Hydrogen, Semiconductors**.
- **Human Capital Development** - Introduce **merit-based career systems** and support **early-stage researchers**.
- **Encourage Interdisciplinary Research** - Break **academic silos** and fund cross-domain innovation.
- **Lab-to-Market Focus** - Shift from **basic research** to **mission-mode R&D** for commercialization and real-world applications.
- Encourage **private sector participation** and use of **CSR funds** for startups and innovation.

Digital Education/E-Learning

Digital Education refers to the use of **technology (internet, computers, mobile apps, TV, radio, etc.)** to deliver **educational content, tools, and experiences remotely or in blended settings**.

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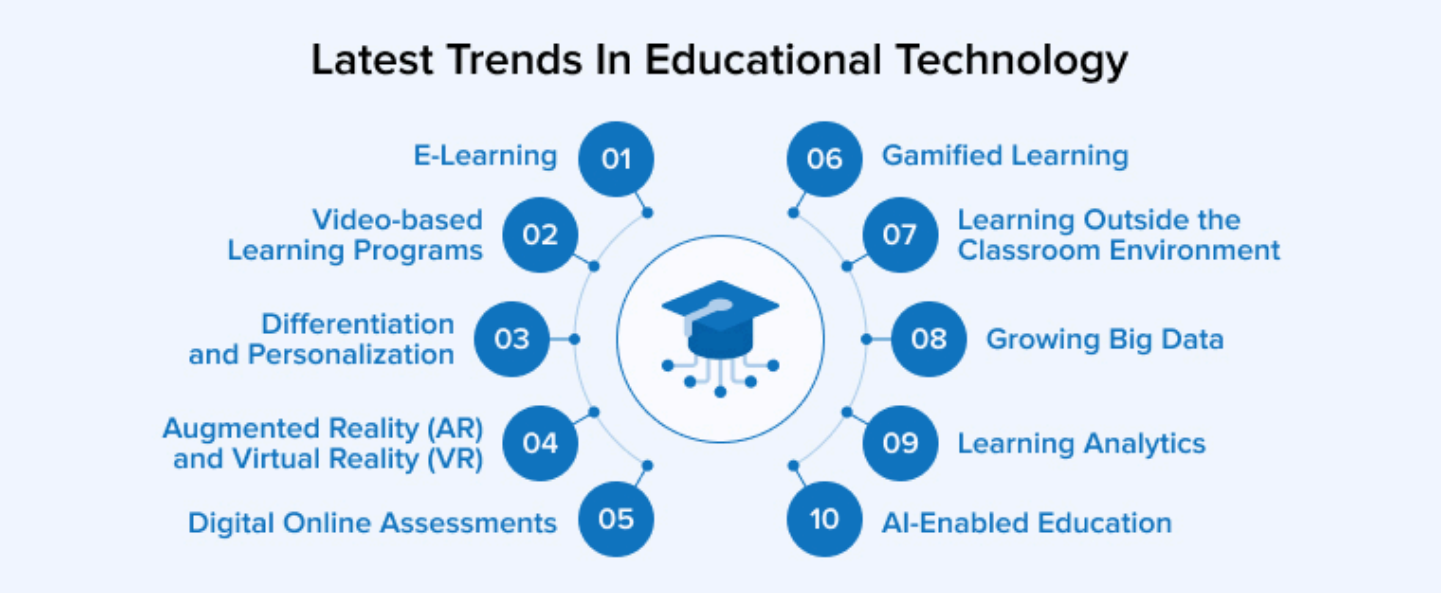

Aspect	Significance
1. Democratization of Learning	Bridges urban-rural and socio-economic gaps by enabling access to quality content across regions.
2. Continuity of Education	Maintained learning during crises like **COVID-19 lockdowns**, ensuring minimal disruption.
3. Flexibility & Personalization	Allows **self-paced learning** through apps and online platforms (Eg. SWAYAM, DIKSHA, BYJU's).
4. Scalability of Content	One-time digital content can be reused at scale, reducing marginal cost for large populations.
5. Promotion of Foundational Literacy & Digital Skills	Supports **FLN goals** and prepares learners for a **digital economy** (as per NEP 2020).
6. Inclusive Learning Models	Beneficial for learners with disabilities and out-of-school children through audio-visual aids.
7. Teacher Empowerment	Platforms like **NISHTHA and DIKSHA** offer training, lesson plans, and performance tracking tools.
8. Alignment with NEP 2020	Advances NEP's vision of **tech-integrated, equitable, and holistic education**.
9. Low-Cost Mass Outreach	Initiatives like **PM eVidya** and **Swayam Prabha DTH channels** reach learners without the internet.
10. Support for Life-long Learning	Enables access to **upskilling, reskilling, and continuing education** across age groups.

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Challenges in Online Education in India

Challenge	Explanation
1. Digital Divide	Only **33% of rural households** have internet access (NSSO, 2022), leading to **exclusion of marginalized groups**.
2. Device Inaccessibility	**Over 60% of students** in government schools lacked smartphone or laptop access during COVID-19 .
3. Language and Content Gap	Most content is in **Hindi or English**; limited regional language and tribal dialect resources.
4. Low Teacher Digital Literacy	Many teachers are unfamiliar with online platforms, limiting **interactive pedagogy**.
5. Poor Infrastructure in Schools	35% of schools lack reliable electricity and digital tools (UDISE+, 2021-22).
6. Weak Monitoring and Assessment	Lack of **real-time feedback**, learning analytics, and **adaptive assessments** results in low engagement.
7. Cybersecurity and Safety Risks	Children face risks of **cyberbullying, data privacy violations**, and screen addiction.
8. Gendered Digital Access	Girls are **less likely** to have access to personal devices or uninterrupted time to study at home (GSMA 2022).
9. One-way Content Delivery	Many platforms offer **non-interactive video content**, limiting student engagement and doubt resolution.
10. Exclusion of Children with Disabilities	Lack of accessible interfaces, assistive tech, and sign language content leads to further marginalization.

Government Initiatives for Online Education in India

Initiative	Key Features
DIKSHA (Digital Infrastructure for Knowledge Sharing)	National portal with QR-linked e-content, e-textbooks, teacher training resources; supports 30+ languages.
SWAYAM (Study Webs of Active Learning for Young Aspiring Minds)	Free online courses by IITs, IIMs, IGNOU; includes credit transfer for UGC/AICTE-approved programs.
e-Pathshala	NCERT platform offering e-books, audio-video resources, and interactive content for K-12 students.
NISHTHA	Online teacher training modules via DIKSHA; covers pedagogy, foundational literacy, ICT skills.

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Initiative	Key Features
PM e-Vidya	Umbrella platform integrating DIKSHA, SWAYAM, Swayam Prabha (TV), radio, and podcasts.
Swayam Prabha	34 DTH channels broadcasting curriculum-aligned lessons 24x7 to reach learners without internet.
IGNOU eGyanKosh	Repository of digital course materials and videos for open and distance learning programs.

Focus Area	Way Forward
Bridging the Digital Divide	Provide free/subsidized devices (smartphones/tablets) and affordable data to students in rural and low-income areas.
Strengthening Infrastructure	Ensure electricity, internet, and smart classrooms in all government schools; use solar-powered tech in remote regions.
Multilingual & Inclusive Content	Develop region-specific, bilingual e-content and make platforms compliant with disability accessibility standards (as per RPwD Act).
Teacher Capacity Building	Expand NISHTHA and DIKSHA-based digital training to include pedagogy for online, blended, and tech-integrated learning.
Promote Blended Learning	Implement **hybrid models** combining in-person teaching with digital tools for flexibility and resilience (aligned with NEP 2020).
Cyber Safety & Digital Literacy	Integrate digital hygiene and safety modules in school curriculum to protect children online.
Public-Private Partnerships	Leverage ed-tech startups and CSR initiatives to co-create scalable, low-cost content and platforms.

Global Best Practices

- **Khan Academy (USA)**: Personalized learning paths with adaptive assessments.
- **Estonia**: Fully digitized school curriculum from early grades.

New Education Policy, 2020

1. The National Education Policy (NEP), 2020 is the **first education policy of the 21st century** and replaces the 34 year-old National Policy on Education (NPE), 1986.
2. Built on the foundational pillars of **access, equity, quality, affordability, and accountability**, this policy is aligned to the 2030 Agenda for Sustainable Development.
3. It aims to **transform India into a global knowledge superpower** by making both school and college education more holistic, flexible, multidisciplinary.

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Domain	Provision	Details
School Education	New Pedagogical Structure	Replaces 10+2 system with **5+3+3+4** covering ages 3-18 (Foundational to Secondary).
	Early Childhood Education (ECCE)	Universal ECCE by 2030 via **Anganwadis** and pre-primary schools.
	Foundational Literacy & Numeracy (FLN)	Launch of **'NIPUN Bharat'** mission to achieve FLN in all Class 3 students by 2026-27.
	Mother Tongue as Medium	Preference for teaching till Class 5 (or 8) in **mother tongue/regional language**.
	Holistic Progress Card	Shift to **competency-based assessments** and 360° multi-dimensional report cards.
Higher Education	Multidisciplinary Education	All HEIs to become **multidisciplinary institutions** by 2040.
	Flexible Degree Options	**Multiple entry-exit** with certificate/diploma/degree; supported by **Academic Bank of Credits (ABC)**.
	National Research Foundation (NRF)	New body to fund and coordinate **quality research across disciplines**.
	Higher Education Commission of India (HECI)	Single regulator replacing UGC, AICTE, NCTE with 4 verticals: Regulation, Grants, Accreditation, Standards.
	Increased GER Target	Aim to raise **Gross Enrollment Ratio to 50% by 2035**.
Digital & Ed-Tech	National Digital Education Architecture (NDEAR)	Digital infrastructure for education delivery and governance.
	Virtual Labs & e-Content	Expansion of **DIKSHA, SWAYAM**; creation of **National Digital University**.
	Use of AI & Adaptive Learning	Promotes use of **AI-based personalized learning platforms**.
Teacher Development	New Norms for Teacher Education	By 2030, teachers to hold a **4-year integrated B.Ed. degree** as minimum qualification.
	Continuous Professional Development (CPD)	Training via platforms like **NISHTHA** with incentives for regular upskilling.
Equity & Inclusion	Gender Inclusion Fund	Dedicated fund to support **girls and transgender children**.

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Domain	Provision	Details
	Focus on Disadvantaged Groups	Inclusive policy for **SC/ST, OBC, disabled, rural/urban poor** and Aspirational Districts.
	Special Education Zones (SEZs)	Focused attention on **educationally backward regions**.
Internationalization	Foreign Universities	Top 100 world universities allowed to **establish campuses in India**.
	Globalization of HEIs	Push for **Study in India**, international student & faculty exchange.

Three language formula

The Three-Language Formula promotes multilingual education in schools by encouraging students to learn three languages, including at least two Indian languages.

Evolution

1. **Article 351 of the Constitution**: Directs the Union Government to promote the spread and development of the Hindi language.
2. **Kothari Commission (1964-66)**: Proposed the **Three-Language Formula** to promote national integration while preserving linguistic diversity.
3. **National Education Policy (NEP) 1968**: Recommended the use of **regional languages** at primary and secondary levels, and encouraged their adoption at the university level.
4. **Programme of Action (1992)**: Emphasized that the **mother tongue or regional language** should be the medium of instruction at the pre-school level.
5. **Right to Education Act, 2009**: Suggested that, wherever possible, **mother tongue** should be the medium of instruction at the elementary level.
6. **National Education Policy (NEP) 2020**: Advocates the use of **home language/mother tongue/local/regional language** as the medium of instruction at least up to Grade 5, preferably up to Grade 8 and beyond.

Steps taken

1. **ASMITA Initiative**: Aims to produce **22,000 books** in **22 scheduled languages** over five years.
2. **Bhashini**: An **AI-powered language translation system** that allows people to speak in their own language while communicating with speakers of other Indian languages.
3. **Bahubhasha Shabdakosha**: Aims to create a **multilingual dictionary repository** for Indian languages.

Significance of the Three-Language Formula

1. **Cognitive and Educational Benefits** - Learning multiple languages enhances critical thinking and improves retention when early education is in the mother tongue (UNESCO).
2. **Cultural Sensitivity and Awareness** - Exposure to diverse languages promotes metalinguistic skills and fosters respect for different cultures.
3. **Global and Local Balance** - English provides access to global knowledge and opportunities, while regional languages maintain emotional and cultural identity.

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4. **Preservation of Linguistic Heritage** - Encourages the survival of local and indigenous languages amid globalization.
5. **Constitutional Compliance and Inclusivity** - Fulfills Article 350A by ensuring primary education in the mother tongue for linguistic minorities.
6. **Strengthening Federalism** - Allows states the flexibility to choose languages, preventing central linguistic imposition.
7. **National Integration** - Promotes unity through interregional language learning without compromising diversity.

Challenges in Implementation

1. **Cognitive Overload and Learning Interference** - Adding a third language increases student burden, causing confusion across languages and reducing mastery of core subjects.
2. **Lack of Consensus Among States - Tamil Nadu has consistently opposed** the TLF, insisting on a **two-language policy** (Tamil and English), citing cultural imposition.
3. **Asymmetrical Application Across Regions** - Northern states often neglect the teaching of southern or eastern Indian languages. Eg. UP and Bihar prefer to offer Sanskrit and Urdu
4. **Shortage of Trained Teachers and Resources** - In many states, there is an **acute lack of teachers** for the third language, especially for non-Hindi or classical languages.
5. **Lack of Infrastructure and Resources** - Many government schools, especially in rural areas, lack **textbooks, digital aids, and multi-language materials**.
6. The formula is a **guideline, not a law**, leading to its **inconsistent and discretionary implementation** by states.

Way Forward

1. **Flexibility with Inclusiveness**: Allow states to adapt the formula in accordance with their linguistic realities while preserving the spirit of multilingualism.
2. **Invest in Teacher Training and Materials**: Ensure proper recruitment and training of language teachers for effective implementation.

Aligning with UNESCO's Multilingual Education Guidance, which advocates for mother tongue-based multilingual education, can ensure linguistic justice and educational equity in a diverse nation.

Skill Development in India

The "India Skills Report 2025" indicates that the national employability rate in India is projected to reach 54.81%. This represents a notable increase from 51.25% in 2024.

"Education which does not help the common mass of people to equip themselves for the struggle for life... is it worth the name?" - **Swami Vivekananda.**

Data -

1. **Pradhan Mantri Kaushal Vikas Yojana** (PMKVY 4.0 ongoing (2022-2026)) - **1.48 crore candidates trained** since 2015.
2. **PM Vishwakarma Yojana** - Support to artisans of 18 traditional trades - includes training, credit, marketing, ID, and toolkit.
3. **Skill Shortage:** In 2025, 80% of employers reported difficulty in finding skilled professionals, a figure higher than the global average of 74%.
4. **~29 million skill gap projected** in sectors like construction, healthcare, retail, and logistics by 2030 (FICCI-EY Report).

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5. India needs **30 lakh digitally skilled professionals** by 2026 (NASSCOM 2025).
6. Only **30% of rural youth** have access to formal skilling programs.
7. Over **1 lakh trainer vacancies** in public and private ITIs (MSDE data).

Need for Skill Development

1. India has over **68% population in the working-age group**, with a median age of ~29. Without appropriate **skilling and upskilling**, this dividend risks becoming a **demographic disaster**.
2. Only **4.7% of the workforce in India** has formal training, compared to 52% in the US and 80% in South Korea (NSDC Report, 2024).
3. Over **45% of Indian graduates are unemployable** due to lack of industry-relevant skills (India Skills Report 2025).
4. Skilling enhances productivity, innovation, and entrepreneurship-key for achieving **$5 trillion economy target**.
5. Every 1% increase in skilled workforce can raise GDP by ~0.3% (World Bank estimation, 2022).
6. Enhancing Global Mobility - Demand for skilled Indian workers is rising in the Gulf, **Europe, and ASEAN**, especially in healthcare, IT, and construction.
7. Enabling Gender and Social Inclusion - **Digital and soft skill training** increases women's workforce participation and remote work opportunities.
8. **Preparing for Industry 4.0**
   a. Sectors like **AI, robotics, data analytics, renewable energy, and EVs** require future-ready talent.
   b. Reports by **NASSCOM (2025)** highlight demand for **30 lakh digital jobs** by 2026

Major Schemes

Skill India Mission (2015)

1. Aims to provide **skill, re-skill, and up-skill training** to the **working-age population**.
2. Focus on **employability, inclusivity, and industry relevance**.
3. Implemented through key schemes: **PMKVY, JSS, NAPS, CTS (ITIs)**.

Pradhan Mantri Kaushal Vikas Yojana (PMKVY 4.0)

1. **Flagship short-term skilling scheme** aligned with **NSQF and industry needs**.
2. **27.24 lakh candidates trained** across **38 sectors, 36 States, 737 districts**.
3. Introduced **69 courses** and **154 future-skill roles** (AI, Industry 4.0, green jobs).
4. Implemented through **16,900+ institutions**, including **6,800+ skill hubs**.

Jan Shikshan Sansthan (JSS)

1. Focus on **non-formal, community-based skilling** for **non-literates, neo-literates, dropouts**. **36.48 lakh beneficiaries trained (since 2018)**.
2. Introduced **32 new NSQF-aligned courses**. Products marketed via **UdyamKart portal** for livelihood support.

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National Apprenticeship Promotion Scheme (NAPS 2.0)

1. Promotes **"earn while you learn" apprenticeship model**.
2. Govt provides **25% stipend (up to ₹1,500/month)** via DBT.
3. **54.41 lakh+ apprentices engaged since 2016**.
4. Introduced **Certificate of Proficiency (CoP)** (67,711 issued).

Craftsmen Training Scheme (CTS) - ITIs

1. Long-term **vocational training system** (since 1950).
2. Training in **169 courses across 14,688 ITIs** (Govt + Private).
3. **Enrolment increased** from **12.51 lakh (2022- 23)** to **14.70 lakh (2025- 26)**.

ITI Strengthening - PM- SETU (2025)

1. **₹60,000 crore scheme** to upgrade **1,000 ITIs** (hub-and-spoke model).
2. **SPVs with industry partners** for co-management. Focus on **industry-aligned courses and emerging technologies**.
3. Upgrade of **5 National Skill Training Institutes** as Centres of Excellence.

Major Issues

1. **Fragmented implementation (In-silos approach)** of schemes like PMKVY, PM-NAPS, and JSS.
2. **Demand-supply mismatch** across sectors and regions and limited integration between skill and higher education.
3. **Low Placement Rate:** out of **31.55 million** enrolled candidates, only **18%** of **PMKVY-trained** individuals have secured employment.
4. **Inadequate Quality Monitoring**
   a. Weak tracking of outcomes (placement, retention, wage growth) under schemes like PMKVY.
   b. Certifications often lack **industry recognition**, affecting employability.
5. **Slow Adoption of Future Skills** - Curriculum in most institutions lags behind **emerging skill needs** (AI, robotics, sustainability, cloud computing).
6. **Limited Apprenticeship Penetration** - Only ~0.1% of the workforce is engaged in formal apprenticeships, due to low awareness and regulatory burdens for employers.
7. **Industry-Academia Skill Mismatch** - Limited involvement of industry in designing course content in most training programs.
8. **Outdated Infrastructure and Trainer Deficit**
   a. Over **1 lakh vacant trainer positions** in ITIs and private training institutes (MSDE).
   b. Many centers lack modern labs, tools, and digital content delivery mechanisms.
9. **Low Awareness and Societal Perception** - Skill-based and vocational training is still viewed as **inferior to academic education**, especially among middle-class families.
10. **Rural-Urban Skilling Divide** - Skilling infrastructure is concentrated in urban areas, while **70% of youth reside in rural India**.
11. **Gender Disparities** - Women's participation in formal skill programs is low due to mobility constraints, safety concerns, and societal norms.

Way Forward

1. Enhance **skill mapping** through **Sector Skill Councils (SSCs)** to align training programs with evolving job market needs.
2. **Unified Governance Framework** - Create a **National Skill Regulator** or strengthen NSDC with statutory authority to streamline schemes, certification, and impact tracking.

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3. Adopt **Result-Based Financing (RBF)** and **Skill Impact Bonds** to link funds with placement, wage gain, and retention outcomes.
4. Align ITIs with local demand via Industry-Academia Boards.(**Sharda Prasad Committee, 2016**)
5. Strengthen **NSQF** credit alignment with UGC/AICTE for easy transition from vocational to higher education.
6. **Robust Monitoring -** Institutionalize **Third-Party Assessments**, publish skilling report cards, and link data with **NCS Portal**.

"Skilling is building a better India. If we have to move India towards development, then skill development should be our mission." **- PM Narendra Modi**

Skill development is key to achieving **SDG 4 (Quality Education)** and **SDG 8 (Decent Work and Economic Growth)**.

Role of AI

AI in Healthcare

1. **Early Cancer Screening -** AI analyzes non-invasive thermal or radiological images to detect malignancies years before physical symptoms appear.
2. **Tuberculosis (TB) Detection -** Automated X-ray analysis identifies TB markers, reducing dependency on scarce radiologists in remote districts.
3. **Clinical Decision Support (CDSS) -** Integrated into telemedicine, AI assists doctors with differential diagnoses and suggests evidence-based treatment protocols.
4. **Maternal & Child Health Bots - Eg.** Madhya Pradesh's **Suman Sakhi** WhatsApp bot provides maternal and newborn health guidance to families.
5. **Precision Oncology -** AI analyzes genomic data to tailor chemotherapy and immunotherapy specifically to a patient's genetic profile.
6. **Predictive Outbreak Surveillance - Eg.** The **Media Disease Surveillance (MDS)** tool assists the NCDC in early epidemic detection and response.
7. **AI-Driven Mental Health Support -** Chatbots provide cognitive behavioral therapy (CBT) and emotional support. **Eg. Wysa**, an Indian AI-bot
8. **Drug Discovery Acceleration-** AI simulates molecular interactions, cutting the time to identify potential drug candidates from years to weeks.
9. **Robot-Assisted Surgery -** AI-stabilized robotic arms allow surgeons to perform complex procedures with sub-millimeter precision.

AI in Education

1. **Personalized Learning Paths - Eg. DIKSHA platform** uses AI to suggest personalized video content based on student performance data.
2. **Multilingual Textbook Translation - Eg. Bhashini** translated NCERT Grade 1-2 textbooks into 22 Indian languages to support mother-tongue instruction.
3. **Inclusive Support for SLDs -** AI tools assist children with Specific Learning Disabilities (SLDs) through speech-to-text and phonetic exercises.
4. **Automated Essay Feedback -** Natural Language Processing (NLP) tools provide real-time feedback on grammar, structure, and clarity to students.
5. **Teacher Administrative Relief -** AI automates attendance, worksheet generation, and basic grading, allowing teachers to focus on mentorship.

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6. **Early Warning for Dropouts -** Analytics flag students at risk of dropping out based on attendance and performance anomalies.
7. **Virtual Labs for STEM -** AI-simulated laboratories allow rural students to conduct complex science experiments without physical equipment.

Key Constraints

1. **Data Silos** - Lack of data integration limits real-time response.
2. **Algorithmic Bias** - AI models trained on **foreign datasets** may not reflect India's socio-environmental diversity.
3. **High Energy Consumption** - AI systems require **energy-intensive computing**
4. **Digital Divide** - Limited **internet access and digital literacy** excludes vulnerable populations.
5. **Privacy Risks** - Integration of **health + education data** raises concerns of **surveillance and discrimination**.
6. **Skill Gap** - Shortage of **interdisciplinary experts (education + health + AI)**.

Measures to Strengthen AI Deployment

1. **Unified Data Architecture** - Create **interoperable, real-time data systems** across sectors.
2. **Explainable AI (XAI)** - Ensure **transparent and interpretable AI models** for clinical trust.
3. **Inclusive Communication** - Use **vernacular, offline channels (SMS, radio)** for last-mile delivery
4. **Capacity Building** - Develop **cross-disciplinary workforce** trained in AI, education & health.
5. **Privacy-Preserving Systems** - **strong data governance frameworks**.
6. **Green AI Regulations** - Promote **energy-efficient AI and renewable-powered data centres**.

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Chapter 3

Issues Related to Poverty and Hunger

Poverty And Hunger Issues

PYQs

[2026] Malnutrition in India is not merely a public health concern; it is also a challenge of social equity, human development and effective welfare governance. Discuss (10)

[2025] Inequality in the ownership pattern of resources is one of the major causes of poverty. Discuss in the context of 'paradox of poverty'. (15)

[2024] Poverty and malnutrition create a vicious cycle, adversely affecting human capital formation. What steps can be taken to break the cycle? (10)

[2020] "The incidence and intensity of poverty are more important in determining poverty based on income alone". In this context analyze the latest United Nations Multidimensional Poverty Index Report. (15)

[2019] There is a growing divergence in the relationship between poverty and hunger in India. The shrinking of social expenditure by the government is forcing the poor to spend more on Non- Food essential items squeezing their food - budget.- Elucidate. (10)

[2018] How far do you agree with the view that the focus on lack of availability of food as the main cause of hunger takes the attention away from ineffective human development policies in India? (15)

"The war against hunger is truly mankind's war of liberation." - John F. Kennedy

Definitions of Key Concepts-

1. **Poverty**: It is a "pronounced deprivation in well-being" which includes low incomes and the inability to acquire basic goods and services necessary for survival with dignity. - **World Bank**
    a. **Absolute Poverty:** Inability to meet basic minimum needs of survival, measured against a defined poverty line. The **WB International Poverty Line (IPL)** is **$2.15/day (2017 PPP)** for extreme poverty.
    b. **Relative Poverty:** Income/consumption substantially below the average standard of a society.
    c. **Multidimensional Poverty:** Measured beyond income, includes deprivations in **health, education, and standard of living**
2. **Hunger:** A physiological state where a person cannot afford or access enough food to meet basic daily energy requirements.
    a. **Chronic Hunger** - sustained inability to consume sufficient calories
    b. **Acute Hunger** - short-term severe food shortage (during famines, disasters)
    c. **Seasonal Hunger** - periodic, linked to agricultural cycles
3. **Hidden Hunger:** A form of malnutrition where the diet is deficient in essential **micronutrients** (vitamins and minerals) despite consuming adequate calories.
4. **Food Security:** As per the FAO, it exists when all people, at all times, have physical, social, and economic access to sufficient, safe, and nutritious food.
    a. **FOUR PILLARS of Food Security:**

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b. **Availability** - sufficient quantity of food
c. **Accessibility** - physical and economic access
d. **Utilisation** - proper biological use (clean water, sanitation, healthcare)
e. **Stability** - consistent access over time

5. **Nutritional security**: Nutrition security means consistent access, availability, and affordability of foods and beverages that promote well-being, prevent disease, and, if needed, treat disease. - **FAO**

Key Data Points

Poverty Dimension

- **M**ultidimensional poverty dropped from 24.8% to **~11.28%** in the last decade. (NITI Aayog)
- **24.85 crore people exited multidimensional poverty between 2013-14 and 2022-23** in India. (NITI Aayog)

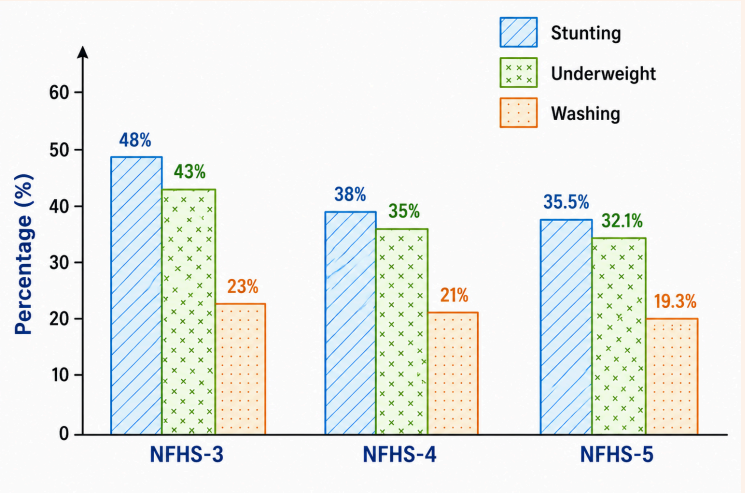
- India has virtually eliminated extreme poverty (below **$2.15/day**) at 2.3%. (World Bank)
- **Standard of Living: 95.4%** of Indian households now reside in pucca or semi-pucca houses. (NFHS-5)
- **Health Coverage:** Around **41%** of households have at least one member covered under a health insurance scheme (NFHS-5).

Hunger Dimension

- **Undernourishment:** at approximately **13.7%**. (FAO SOFI Report 2025)
- **Moderate to severe food insecurity** affects nearly **16%** of the population (FAO 2025).
- **India among top 3 countries globally** with highest hidden hunger burden (affects 70%+ population (WHO/UNICEF).)
- India ranked **102 out of 123** countries in the 2025 Global Hunger Index (**Serious** category)

Malnutrition Dimension

- **Child Stunting** (low height-for-age)**: 35.5%** of children under five. (NFHS-5) (**38.4%** (NFHS-4))
- **Child Wasting** (low weight-for-height)**:** India has one of the highest wasting rates globally at **19.3%** (NFHS-5). (**21%** (NFHS-4))
- **Underweight** prevalence has reduced from 35.8% (NFHS-4) to 32.1% (NFHS-5)
- **Anaemia: 67%** of children and **57%** of women aged 15-49 years (NFHS-5).



- **Double burden of malnutrition - 24% women, 23% men overweight/obese** (NFHS-5)
- **81.35 crore beneficiaries** receive subsidised foodgrain under National Food Security Act

Kerala has become the 1st **state** to eradicate extreme poverty in India. (Extreme Poverty Eradication Programme (EPEP))

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Committees on Poverty Estimation

1. **WORKING GROUP (1962) - Planning Commission - Per capita consumption expenditure of ₹20 & 25/month at 1960-61 prices** (rural & urban) - based on PV Sukhatme's nutritional norms
2. **DR. V.M. DANDEKAR & N. RATH STUDY (1971) - First systematic estimation** of poverty.
   a. **Calorie norm:** 2,250 calories per capita per day (rural and urban)
   b. Used **NSSO Consumer Expenditure Survey** data
   c. Estimated **~40% population below poverty line** in early 1970s
3. **ALAGH COMMITTEE**
   a. **Calorie requirements: 2,400 calories/day rural, 2,100 calories/day urban**
   b. **Poverty line based on minimum nutritional requirements** translated to monetary expenditure
4. **LAKDAWALA COMMITTEE (1993)**
   a. **State-specific poverty lines** for rural and urban areas
   b. **CPI-IW** (Consumer Price Index for Industrial Workers) for urban poverty line
   c. **CPI-AL** (CPI Agricultural Labourers) for rural poverty line
   d. Continued **calorie-based approach** (2,400 rural, 2,100 urban)
5. **TENDULKAR COMMITTEE (2009)**
   a. **Moved from calorie norms to expenditure-based poverty line**
   b. Included **education, health, transport** in poverty calculation (broader cost of living)
   c. **Poverty line: ₹816 rural, ₹1,000 urban per capita per month** (2011-12 prices)
   d. **MMRP (Modified Mixed Reference Period)** - different recall periods for different items
   e. **Estimated 21.9% poverty (2011-12)**
6. **RANGARAJAN COMMITTEE (2014)**
   a. **Reverted to calorie + protein + fat norms** + non-food expenditure
   b. **Higher poverty line: ₹972 rural, ₹1,407 urban** per capita per month (2011-12 prices)
   c. **Estimated 29.5% poverty (2011-12)** - higher than Tendulkar's 21.9%
7. **NITI AAYOG MULTIDIMENSIONAL POVERTY INDEX (2021, 2023)**
   a. **Adopted OPHI-UNDP MPI methodology**
   b. **12 indicators** across 3 dimensions: **Health, Education, Standard of Living**
   c. **Uses NFHS data**

Issues with Current Estimation Methods

1. **No Updated Official Poverty Line -** India hasn't updated official poverty line post **Rangarajan Committee (2014)**
2. **Outdated Consumption Data - HCES 2011-12 was the last comprehensive consumption survey. HCES 2022-23 published in February 2024** broke gap but reliability questioned
3. **Methodological Disputes - Calorie-based (Lakdawala) vs Expenditure-based (Tendulkar) vs Multidimensional (MPI)** - different methods give vastly different estimates.
4. **One size fits for all** approach (**single national line**) is inadequate due to **state-wise Variations** inflation, prices, lifestyles
5. **Multidimensional vs. Monetary Conflict -** Declining MPI scores often mask stagnant real wages and high inflation for the bottom 20%.
6. **Invisible Out-of-Pocket Expenditure -** Poverty lines fail to fully capture the impact of health shocks on family budgets. **Eg.** Health costs push **55 million** Indians into poverty annually
7. **The "Near-Poor" Vulnerability - Eg.** Climate shocks or job loss can move **24% of "vulnerable"** Indians into destitution

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8. **Calorie Bias in Nutrition** - Traditional estimation focuses on "calories" rather than "nutrition," ignoring the quality and diversity of diet.
9. **Digital Exclusion in Mapping** - Heavy reliance on Aadhaar-linked datasets for identifying the poor can exclude those without digital access.
10. **Gender Blindness** - Treating the "household" as a unit ignores intra-household inequality where women eat less and last. **Eg.** High female anaemia (**57%**) in non-BPL families
11. **Urban Poverty Underestimation** - High costs of urban housing and transport are not adequately reflected in national poverty baskets.
12. Poverty lines are static and don't account for **Climate-Induced Transitory Poverty** caused by heatwaves or floods.

Government Schemes

Food Security & Nutrition

1. **PM Garib Kalyan Anna Yojana (PMGKAY)**
   a. Provides **5kg of free foodgrains** per month to eligible individuals.
   b. Covers **81.35 crore** beneficiaries (67% of the population.)
2. **One Nation One Ration Card (ONORC)**
   a. Intra-state and inter-state **portability** of ration cards, benefiting migrant workforce
   b. Enabled over **195 crore** portable transactions
3. **PM POSHAN (formerly Mid-Day Meal)**
   a. Provides hot cooked meals to school children
   b. Serves approximately **11.8 crore** children across 11.2 lakh schools

Employment & Livelihoods

1. **VB-G RAM G (formerly MGNREGS)**
   a. Guarantees 125 days of manual work for rural poor
   b. Generated **192.54 crore person-days** of work in FY 2025-26
2. **DAY-NRLM (Ajeevika)**
   a. Mobilizes rural poor women into **Self-Help Groups (SHGs)**, facilitating bank linkages and micro-entrepreneurship.
   b. Mobilized over **10.29 crore women** into 9.1 million SHGs
3. **PM-SVANidhi**
   a. Provides collateral-free working capital loans to **street vendors** to formalize their small businesses and improve credit history.
   b. Benefited over **63 lakh** street vendors with total disbursements over ₹11,000 crore.
4. **Pradhan Mantri Kaushal Vikas Yojana (PMKVY)**
   a. **Aims to enable Indian youth to take up industry-relevant skill training. PMKVY 4.0 - Focus Areas**
   b. **Industry 4.0**
   c. **Web 3.0, AR/VR**
   d. **Climate Change, Circular Economy, Green Economy**
5. **National Urban Livelihood Mission (NULM)**
   a. **Skill up-gradation and employment creation.**
   b. **Entrepreneurship development.**
   c. **Provision of shelters for the urban homeless.**
   d. **Support for urban street vendors.**

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Housing & Infrastructure

1. **PMAY-Gramin (Pradhan Mantri Awaas Yojana) -** More than **3.02 crore** rural houses completed (2.99 crore)
2. **Jal Jeevan Mission (Har Ghar Jal)**
   a. Aims to provide **Functional Household Tap Connections (FHTC)** to every rural home (55 liters per capita per day).
   b. Reached **81.57%** (15.79 crore) of rural households by January 2026
3. **Swachh Bharat Mission (SBM) 2.0**
   a. Moves beyond toilet construction to **ODF Plus**, focusing on solid and liquid waste management in villages.
   b. Over **5.68 lakh villages** declared ODF Plus as of February 2026

Social Security & Income Support

1. **PM-KISAN -** Direct cash transfer of **₹6,000/year** to farmers to help with agricultural inputs and domestic needs. Disbursed over **₹3 lakh crore** to over 11 crore farmers since inception.
2. **PM-JANMAN -** A saturation-focused mission for **PVTGs** across 11 critical sectors like housing and water. Completed **2.5 lakh houses** and 8,199 village water projects.
3. **National Social Assistance Programme-** monthly **pensions** to the elderly, widows, and persons with disabilities from BPL households. (supports over **3 crore** vulnerable citizens)

Health & Life Insurance

1. **Ayushman Bharat PM-JAY**
   a. Covers hospital costs up to **₹5 lakh/family** per year; recently expanded to include all **Senior Citizens (70+)** regardless of income.
   b. Authorized over **11.69 crore** hospital admissions worth ₹1.73 lakh crore.
2. **PM-Jeevan Jyoti & Suraksha Bima (PMJJBY/PMSBY)**
   a. Low-cost Life and Accident insurance (premiums as low as ₹1/day) designed for the unorganized sector.
   b. Combined enrollment crossed **50 crore** individuals by 2026

Financial Inclusion & Skilling

1. **PM Jan Dhan Yojana (PMJDY)**
   a. The bedrock of the **JAM Trinity**, ensures every adult has a bank account to receive government benefits directly.
   b. Over **57 crore** accounts opened with deposits exceeding ₹2.3 lakh crore.
2. **PM-Mudra Yojana**
   a. Provides loans up to ₹10 lakh for non-corporate, non-farm small/micro-enterprises.
   b. Over **45 crore** loans sanctioned with nearly **70%** of women beneficiaries.

Reasons for Poverty in India

Structural

1. **Colonial Legacy:** deindustrialization of the economy and exploitation of agricultural resources. Eg. India's GDP share fell from 24.4% in 1700 to 4.2% in 1950
2. **Low Agricultural Productivity:** Fragmented landholdings, Lack of modern technology access, Insufficient irrigation facilities, and dependency on monsoon rains.
3. **Institutional Inertia:** Post-independence "Hindu Rate of Growth" (slow growth) delayed large-scale poverty exit for decades.
4. **Inadequate Infrastructure:** India's infrastructure gap is estimated at **$1.4 trillion,** hindering economic growth.

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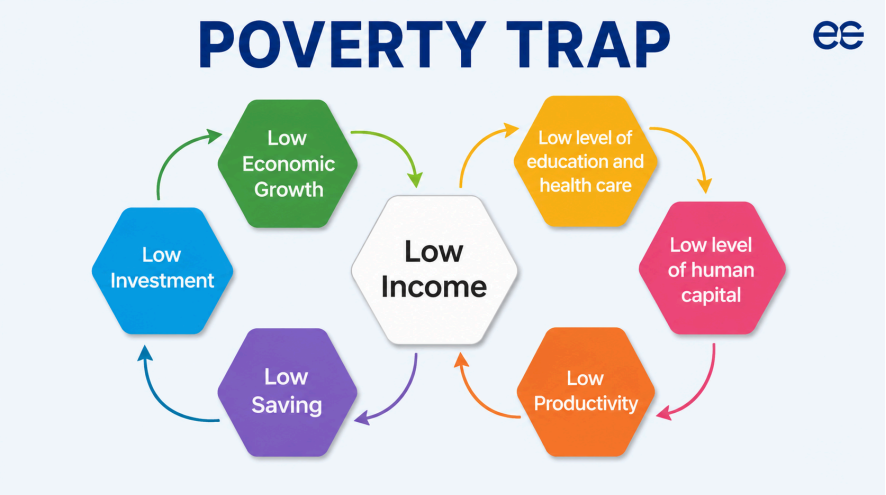
Political & administrative issues

1. **Governance and Policy Failures** due to high centralization, corruption, and overlapping. **Eg.** 30% of NREGA payments are delayed beyond the 15-day limit.
2. **Failure of land reforms - Eg.** 10% of landowners in India control over 55% of the land.
3. **Inclusion/Exclusion Errors:** Faulty targeting leads to "ghost beneficiaries" while the needy are left out.

Economic

1. **Polycrisis - multiple crises** of slow economic growth, increased fragility, climate risks, and heightened uncertainty have come together at the **same time. (WB)**
2. **Informal Economy:** Over **90% of the workforce is still** employed in the informal sector with **low wages and no social protection**.
3. **Agricultural Dependence:** Around **42% workforce** dependent on agriculture contributing only **16-18% of GDP**, reflecting **low productivity**. (**disguised unemployment**)
4. **Income Inequality:** Top **10% hold over 77% of national wealth** (Oxfam India 2024).
5. **Regional Disparities:** Concentration of poverty in the "BIMARU" belt. Bihar's poverty rate is **>25%**, while Kerala's is **<1%** (NITI Aayog 2025).



6. **Capital Deficiency** for MSMEs hampers entrepreneurship. Only **10% of MSMEs** have access to formal credit

Unemployment and Underemployment

1. **Youth Unemployment:** Above **15%** among 15-29 age group (PLFS 2023-24).
2. **Skill Mismatch:** Inadequate alignment between **education, skilling, and labor market demand**.

Low Human Capital Development

1. **Education:** Learning poverty - over **55% of children at grade 5** unable to read basic text (ASER 2024).
2. **Health: Malnutrition (Stunting 35%, Wasting 32%)** reduces productivity and earning potential.
3. **Health Shocks:** Lack of universal primary health coverage causes high out-of-pocket expenditure. Medical costs push **~50 million Indians** back into poverty every year

Gender Inequality

1. **Female Labor Force Participation:** Only **41%** - limits household income potential. (Global avg - 47%)
2. **Gender Wage Gap:** Women earn **~30% less** than men for similar work.
3. **Social Norms:** Restrict economic participation and mobility of women in rural areas.

Social Exclusion and Caste Barriers

1. **Caste-linked Poverty: SCs, STs, and OBCs** continue to have **higher poverty ratios** due to discrimination and landlessness.
2. **Tribal Deprivation:** Over **40% of India's poor belong to SC/ST communities** (NITI Aayog, MPI 2023).

Climate Vulnerability

Extreme weather events disproportionately push the rural poor into debt traps. Eg. heatwaves reducing agricultural productivity

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Way Forward

1. **Women-Centric Economic Empowerment**
   a. Boost **female labor force participation** through **childcare infrastructure, flexible work, and credit support** under **DAY-NRLM** and **Stand-Up India**.
   b. Promote **women-led SHGs** in sectors like food processing, handicrafts, and renewable energy.
2. **Multidimensional Poverty Reduction Approach**
   a. Focus on **data-driven local planning** and **convergent implementation** across ministries.
   b. Move from **poverty alleviation to vulnerability prevention** through **social insurance and resilience-building.**
3. **Climate-Resilient Livelihoods** - Renewable energy SHGs, climate-smart agriculture, MGNREGA assets for adaptation.
4. **Agricultural Reform** - Strengthen Farmer Producer Organizations, market linkages and expand MSP for diversified crops.
5. Explore targeted **"Basic Income Support"** for the bottom 10% to provide a survival floor. Eg. conditional cash transfer model of **Brazil's Bolsa Família**.
6. **Kudumbashree Model:** Scale Kerala's women-led SHG model nationally to build grassroots economic resilience.
7. **Gender-Responsive Budgeting:** Mandate that 30% of all rural development funds be managed directly by Women's SHG Federations.
8. **Universal Health Coverage (UHC):** Expand Ayushman Bharat to cover **out-patient (OPD)** costs, not just hospitalization.
9. **MSME Credit Guarantee:** Provide interest subvention specifically for "Micro" units (Udyami Bharat scheme expansion).
10. **Launch a national Urban MGNREGA to address the rising "Urban Poor" phenomenon.**

Global Models

- **Grameen Bank Model (Bangladesh)** - Microcredit to poor women
- **Sustainable Livelihood Approach (DFID, UK)** - Builds five capitals (human, natural, social, physical, financial).

State-Level Models in India

- **Chhattisgarh PDS Reform** - Universal food access and digitization; reduced leakages.
- **Pudhu Vaazhvu Project (Tamil Nadu)** - Community-based poverty alleviation and livelihood diversification.

The feminization of Poverty in India:

The "feminization of poverty" means that women have a higher incidence of poverty than men, that their poverty is more severe than that of men, and that poverty among women is on the increase.

- **Gender Inequality Index (GII):** India ranked **102nd** in the UNDP GII 2025
- **Labor Participation:** As of Dec 2025, **Female LFPR** stood at **35.3%**, (male rate of ~79%).
- **Wage Gap:** Average female earnings in self-employment are roughly **₹6,374**, compared to **₹17,914** for males-a gap of nearly 64% (PLFS 2025).

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- Women spend **289 minutes/day** on unpaid domestic services (men spend only **88 minutes)** (Time Use Survey 2024-25)

1. **High Vulnerable Employment among Women - 79.6%** of working women are in **vulnerable employment** vs **70.1%** of men. (Source: World Bank Gender Data Portal, 2023). (data need to update 78.2% and 69%)
2. **Overrepresentation in Informal Sector - Over 90%** of employed women work in the **informal sector**, with no job security or social benefits.
3. **Low Land Ownership -** Women own only **13.96%** of operational land holdings in India, limiting economic independence. (Source: Women and Men in India Report 2023, MoSPI)
4. **High Poverty among Female-Headed Households -** Households headed by women show a **higher incidence of poverty** compared to male-headed ones.
5. **Major Share in Agricultural Labor -** Women constitute about **47%** of agricultural laborers in India but earn significantly less than male counterparts. (33% women agricultural laborers)

Major Causes

1. **Dual Burden of Labor - Eg.** Women spend 3x more time on unpaid care than men (Time Use Survey 2025).
2. **Asset Ownership Deficit - Eg.** Only **13.9%** of operational landholdings in India are owned by women (Agri-Census).
3. **Occupational Segregation -** Women are concentrated in "pink-collar" or low-end agricultural jobs. **Eg.** Over **75% of rural women** workers are engaged in agricultural labor (PLFS 2025).
4. **The "Motherhood Penalty" -** Career breaks for childbirth and lack of affordable childcare often lead to permanent exit from the workforce.
5. **Credit Inaccessibility** due to lack of formal credit history or land titles. **Eg.** Female-led MSMEs face a **$158 billion** credit gap in India (World Bank/IFC report).
6. **Digital Gender Divide -** Limited access to smartphones and data prevents women from leveraging the digital gig economy.
7. **Vulnerability of Female-Headed Households - Eg.** Poverty incidence in urban female-headed households is **1.5% higher** than male-headed ones (NSSO).
8. **Nutritional Deprivation -**"Eating last and least" results in chronic health issues that lower economic productivity. **Eg. 57%** of women are anaemic (NFHS-5).
9. **Climate Change Disproportionality -** As primary water/fuel collectors, climate disasters increase women's "time poverty" and health risks.

Impact

1. **Intergenerational Cycle -** Poor mothers are more likely to have stunted children, perpetuating poverty across generations.
2. **Feminization of Agriculture -** As men migrate for urban jobs, women are left with low-productivity, unmechanized farm work. (**~42%** of the agricultural labor force).
3. **Compromised Agency** due to financial dependence. **Eg.** Only **18%** of women have a major say in large household purchases (NFHS-5).
4. **Health Deterioration -** Poverty forces women to neglect personal health in favor of family needs, increasing mortality.
5. **Rise in Informalization -** Destitution forces women into exploitative home-based industries (beedi rolling, garment) with no rights.

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6. **Educational Dropout for Girls** - Poor families prioritize sons' education, while girls are kept back for domestic chores.
7. **Increased Domestic Violence** - Economic stress in poor households often manifests as physical abuse against women.
8. **Child Marriage Prevalence** - Poverty treats daughters as "burdens," leading to early marriage to reduce household expenses.

Way Forward

1. **Adopt "Gender Budgeting":** Mandate 30% allocation of all rural/urban development funds for women-specific assets (Dasgupta Committee).
2. **Lakhpati Didi Scale-up:** Expand the goal to **6 crore women** to ensure a minimum annual income of ₹1 lakh per household.
3. **Kudumbashree Model:** Replicate Kerala's model of thrift and credit societies to build grassroots financial resilience.
4. **Universal Creche Facilities:** Enforce the **Maternity Benefit Act** and launch "Palna" (National Creche Scheme) at the Panchayat level.
5. **Namo Drone Didi:** Scale technology adoption by SHGs for precision agriculture to reduce manual labor.
6. **Skilling for Green Jobs:** Train women in renewable energy and EV-component assembly .
7. **Workplace Safety:** Implement the "ILO C190" standards to eliminate violence and harassment in the informal workplace.

Hunger

India faces a "paradox of plenty," where it remains a food-surplus nation, yet struggles with systemic **Nutritional Insecurity** and **Hidden Hunger**.

Key Data Points

1. **Global Hunger Index:** India ranked **102nd out of 123** countries; categorized as "Serious."
2. **Prevalence of Undernourishment:** Approximately **13.7%** of the population (FAO SOFI 2025).
3. **NFHS-5 Malnutrition Core:** Stunting (**35.5%**), Wasting (**19.3%**), and Underweight (**32.1%**) in children under five.
4. **Anaemia Crisis:** Affects **67%** of children and **57%** of women (15-49 years) (NFHS-5).

Major Causes of Hunger & Malnutrition

Social Causes

1. **Gender Inequality in Food:** Women often eat last and least, leading to maternal and child malnutrition.
2. **Lack of Breastfeeding Awareness** hinders infant immunity and growth.
3. **Early Marriage & Pregnancy:** Adolescent mothers are more likely to have low-birth-weight, malnourished infants.
4. **Marginalized groups** face structural barriers to diverse diets and quality healthcare. **Eg.** Stunting rates among ST children are 6% higher than the national average.
5. **Sanitation and Hygiene Gaps:** Poor WASH (Water, Sanitation, Hygiene) leads to enteric infections, preventing nutrient absorption.

Administrative Causes

1. **PDS Inclusion/Exclusion Errors: Eg.** Use of 2011 Census data for 2026 allocations excludes many new urban poor.

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2. **Anganwadi Infrastructure Deficit:** Many centers lack basic tools like growth-monitoring charts and functional kitchens.
3. **Storage and Logistics Loss** due to inadequate cold-chain infrastructure. **Eg.** India loses ~₹92,000 crore worth of produce annually due to post-harvest mismanagement.
4. **Data Silos in Monitoring:** Lack of real-time nutrition tracking prevents timely intervention in high-burden districts.

Economic Causes

1. **Stagnant rural incomes** limit the ability to purchase high-value protein sources. **Eg.** Poor households spend 60% of income on cereals, neglecting milk and eggs.
2. **High out of pocket health expenditure - Eg.** Health shocks push 55 million Indians back into poverty annually (Economic Survey).
3. **Micronutrient Inflation: Eg.** "Thalinomics" 2026 shows a 15% rise in the cost of a nutritious vegetarian thali.(1% drop in price)
4. **Agricultural Mono-cropping:** Focus on rice-wheat cycles has reduced the availability of traditional, nutrient-dense millets.

Technological Causes

1. **Limited Bio-fortification:** Lack of high-yield, bio-fortified seeds prevents farmers from growing naturally nutritious crops.
2. **Digitization Barriers:** Biometric failures at PDS shops (PoS machines) lead to denial of food.

Relationship Between Poverty and Hunger

1. **Cyclical Nature:** Poverty limits food access, and hunger causes malnutrition, which reduces cognitive/physical productivity, entrenching poverty.
2. **Hidden Hunger Paradox:** Even as monetary poverty falls, families shift to cheap, calorie-dense "junk" foods, remaining malnourished.
3. **Low Productivity:** A hungry child grows into an adult with lower earning capacity, ensuring the poverty cycle continues.
4. **Intergenerational Trap:** Malnourished mothers give birth to stunted children, passing poverty to the next generation via biology.
5. **Resource Divergence:** Families must choose between food and "capability" investments like education, leading to long-term destitution.
6. **Vulnerability to Shocks:** Poor households lack buffers; a single crop failure leads to immediate, acute hunger.
7. **Gendered Poverty:** Female poverty is higher, leading to "maternal depletion" and high child mortality in poor families.
8. **Sanitation Link:** Poverty forces people into unhygienic environments, where diarrhea wastes consumed nutrients (Poverty-Hunger-Disease triad).

Impact of poverty and hunger

Dimension	Impact
Health	Leads to **malnutrition, stunted growth, high infant and maternal mortality**, and poor immunity. **Eg.** NFHS-5 (2019-21) shows **35.5% Indian children are stunted**.
Education	Causes **low enrollment and high dropout rates**, especially among girl children, due to pressure to work or care for siblings.

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Dimension	Impact
Productivity	Malnourished and uneducated populations suffer from **low human capital** and poor physical and cognitive productivity.
Social Inequality	Reinforces **caste, gender, and regional disparities**, as marginalized groups face intergenerational poverty.
Child Labour	Traps children in **informal, hazardous work**, depriving them of education and childhood. **Eg.** As per Census 2011, 10.1 million child laborers in India.
Crime & Exploitation	Poverty increases vulnerability to **trafficking, bonded labor, petty crimes**, and even violent extremism.
Political Instability	Hunger and deprivation erode **trust in governance**, leading to unrest and democratic disengagement.
Economic Growth	Creates a **demand gap** and limits domestic consumption, weakening market expansion and inclusive GDP growth.
Public Health Burden	Increases dependence on **public healthcare systems** with high costs and low outcomes.
Global Standing	Persisting hunger lowers India's ranking in **Global Hunger Index** and affects international image. **Eg.** India ranked **102th out of 125** in GHI 2025.

Government Initiatives

1. **PM Garib Kalyan Anna Yojana (PMGKAY):** Free 5kg foodgrains to 81 crore people.
2. **POSHAN Abhiyaan 2.0:** Mission-mode approach to improve nutritional outcomes for children/mothers.
3. **One Nation One Ration Card:** Portability of food security for migrant workers.
4. **Anemia Mukt Bharat:** Strategy to reduce anemia via iron-folic acid supplementation.
5. **Eat Right India:** FSSAI initiative to promote safe, healthy, and sustainable food.
6. **Pradhan Mantri Matru Vandana Yojana (PMMVY):** Direct cash transfer for pregnant women.
7. **Mission Shakti:** Integrated women empowerment program including nutritional security.
8. **PM-JANMAN:** Targeted food and health saturation for PVTG tribal groups.
9. **Food Fortification Resource Centre (FFRC):** Scaling up fortification in staple foods.

Way Forward

1. **Shift to "Nutri-Cereals":** Incentivize millets (Shree Anna) to replace rice-wheat in PDS (Committee Rec: **Ashok Dalwai**).
2. **Universalize PDS:** Move away from targeted universal PDS in high-burden districts to eliminate exclusion.
3. **Kudumbashree Model:** Use women's SHGs to run community kitchens (Janakeeya Hotels) as seen in **Kerala**.
4. Pilot **"Conditional Cash Transfers"** for diverse food, similar to **Brazil's Bolsa Família**.
5. **Nutri-Gardens:** Promote "Poshan Vatika" in every school and Anganwadi for fresh micronutrients.
6. **Village-level Cold Rooms:** Invest in decentralized solar cold storage to reduce perishable waste.

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7. **Mandatory Bio-fortification:** Integrate bio-fortified seeds into the National Seed Mission.
8. **Agri-Nutrition Linkage:** Align agricultural production patterns with local nutritional requirements (**MS Swaminathan Rec**).

India's vision of 'Sabka Saath, Sabka Vikas' demands inclusive nutrition as the foundation for sustainable human development.

As Gandhi said, 'To a hungry man, God appears as bread'-nutrition must precede all other rights and reforms.

Achieving SDG-2 'Zero Hunger' requires a shift from calorie sufficiency to holistic nutritional equity, especially for vulnerable populations.

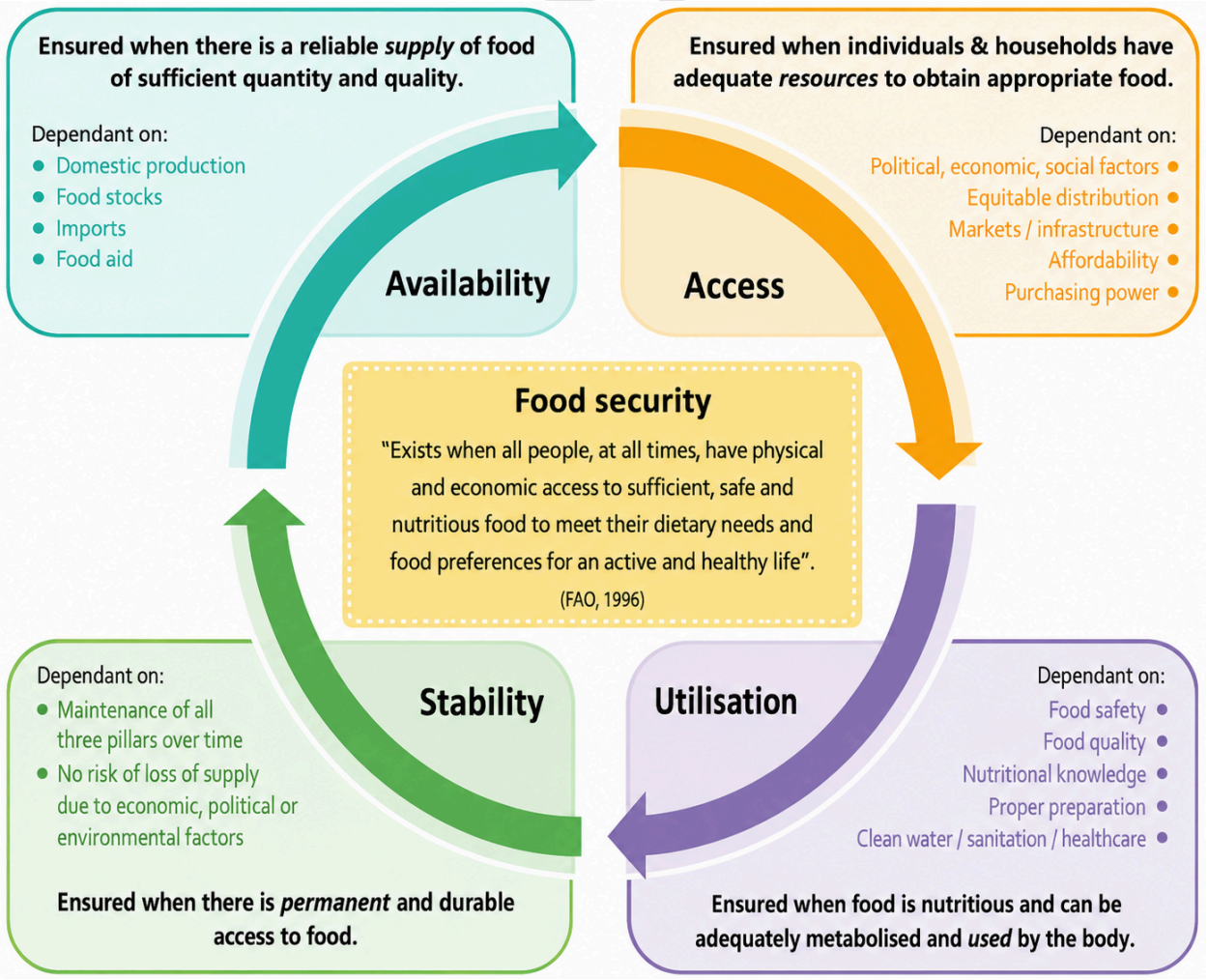
National Food Security Act

The **National Food Security Act (NFSA)** was enacted in **2013** to provide **legal entitlement to subsidized food grains** to nearly two-thirds of India's population.

Objective: "To provide for food and nutritional security in the human life cycle approach, by ensuring access to adequate quantities of quality food at affordable prices."

Key Features and Provisions

1. **Legal Entitlement:** Covers up to **75% of rural** and **50% of urban** populations (81 crore people
2. Beneficiaries are divided into **Antyodaya Anna Yojana (AAY)** (**35kg per family/month**) and **Priority Households (PHH)** (**5kg per person/month**)



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3. **Maternity Benefits:** PW&LM are entitled to a cash maternity benefit of at least **₹6,000** (Pradhan Mantri Matru Vandana Yojana).
4. **Nutritional Standards:** Mandates free meals through **Anganwadis** and **PM-POSHAN** (Mid-Day Meals) in schools.
5. The eldest woman (18+ years) is designated as the **"Head of the Household"** for the purpose of issuing ration cards.
6. **Grievance Redressal:** Statutory requirement for District Grievance Redressal Officers (DGRO) and State Food Commissions.
7. **Transparency:** Mandates **Social Audits** and the setting up of Vigilance Committees at the local level.

Major Achievements of NFSA

1. Successfully integrated PMGKAY to provide **100% free foodgrains** to all 81.35 crore beneficiaries since 2023.
2. **One Nation One Ration Card** recorded over **200 crore portability transactions** for migrants.
3. Achieved **99.9% ration card digitization** and **99.8% Aadhaar seeding**
4. **Rolled out fortified rice (Iron, Folic Acid, B12) across all NFSA districts** to combat anemia.
5. Proven as the world's largest safety net during global supply chain disruptions (COVID).

Major Issues

1. **Outdated Coverage Cap -** NFSA coverage is still pegged to the 2011 Census. Experts estimate **over 100 million people** are legally eligible but currently without ration cards.
2. **High Fiscal Burden -** food subsidy bill is over **₹2.1 lakh crore** annually.
3. **Biometric Failures (ABBA) -** Elderly or manual laborers with worn fingerprints often face denial of service due to authentication failures.
4. **Micronutrient "Hidden Hunger" -** The heavy focus on rice and wheat neglects essential proteins like pulses and oils in the PDS basket.
5. **Storage and Spoilage - Eg.** India loses approx. **₹92,000 crore** of produce annually due to post-harvest mismanagement and pests.
6. **Inclusion/Exclusion Errors - Eg.** Aadhaar cleaning removed **5.8 crore fake cards**, yet many "invisible poor" remain unlisted.
7. **Nutrition Utilization Gap -** Food security is provided, but poor water/sanitation (WASH) limits nutrient absorption.

Way Forward

1. **Diversify the Basket:** Include millets (**Shree Anna**), pulses, and edible oils to move from calorie security to **nutritional security**.
2. **Universalize PDS in Distressed Areas:** Follow the **Chhattisgarh Model** to universalize PDS in the 112 Aspirational Districts.
3. **Adopt "Smart" Technology:** Implement **Iris-scanners** and **OTP-based overrides** to prevent exclusion due to fingerprint failures.
4. **Modernize Storage:** Rapidly scale the "World's Largest Grain Storage Plan" in the cooperative sector to eliminate spoilage.
5. **Strengthen Social Audits:** Mandate **Panchayat-led social audits** every six months to identify "ghost" beneficiaries locally.
6. **Convergence with WASH:** Link NFSA with the **Jal Jeevan Mission** to ensure clean water, maximizing the biological utilization of food.

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7. **Promote DBT-Cash:** Gradually shift to **Direct Benefit Transfer** in urban areas to give beneficiaries choice and reduce logistical costs.
8. **Decentralized Procurement:** Encourage states to procure locally to reduce the massive carbon footprint and costs of FCI logistics. (**Shanta Kumar committee**)
9. **Implement Shanta Kumar committee recommendations:** Rationalize FCI's role and move toward a more efficient, private-sector-integrated supply chains

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